

The attached Summary Plan Description (SPD) and Summaries of Material Modifications (SMM's) are copies of those that applied to the named plan as of July 13, 2026. The originals of the SMM's may not have been signed by an authorized representative of the Plan Administrator of this plan. For a signed copy, contact the Plan Administrator.

These copies are provided for the convenience of plan participants and covered persons and are not intended to replace the actual plan documents on file with the plan administrator, or the SPD and SMM's that were distributed to the plan participants in accordance with any applicable law (including ERISA, if applicable). Any discrepancies between these copies, and the actual SPD or SMM's will be resolved in favor of the original documents as maintained by the plan administrator. These documents may be amended by the plan administrator at any time, and such amendments will prevail over these documents.

**HIGHLAND COMPUTER FORMS, INC.
EMPLOYEE BENEFIT PLAN
SUMMARY PLAN DESCRIPTION**

SUMMARY OF MATERIAL MODIFICATIONS NO. 4

This Summary of Material Modifications is a description of important changes which have been made to the Plan. You should read these changes carefully and keep this document with your copy of the Summary Plan Description. If you have any questions about these changes, you should contact the Plan Administrator for more information.

The Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description (herein referred to as “SPD”) is hereby amended and modified as set forth below. Such amendment is effective as of September 1, 2026.

Section 11.2 entitled “**MEDICAL BENEFIT EXCLUSIONS AND LIMITATIONS**,” as set forth in Article XI of the Plan and as otherwise amended, is amended by adding thereto the following new subsection:

“AK. Charges for intraoperative neuromonitoring (IOM).”

Highland Computer Forms, Inc. hereby adopts the above amendment to the Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description effective as of September 1, 2026.

**HIGHLAND COMPUTER FORMS, INC.
EMPLOYEE BENEFIT PLAN
SUMMARY PLAN DESCRIPTION**

SUMMARY OF MATERIAL MODIFICATIONS NO. 3

This Summary of Material Modifications is a description of important changes which have been made to the Plan. You should read these changes carefully and keep this document with your copy of the Summary Plan Description. If you have any questions about these changes, you should contact the Plan Administrator for more information.

The Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description (herein referred to as “SPD”) is hereby amended and modified as set forth below. Such amendments are effective as of January 1, 2026.

1) Section 2.3 entitled “**OUT-OF-POCKET LIMITS**” as set forth in Article II of the Plan, is amended in its entirety as follows:

“2.3 OUT-OF-POCKET LIMITS

Calendar Year Out-of-Pocket Limits
(includes all Copayments)

<u>Per Individual</u>	\$10,600.00
<u>Per Family</u>	\$21,200.00”

2) Subsections AH and AI previously added to Section 11.2 entitled “**MEDICAL BENEFITS EXCLUSIONS AND LIMITATIONS**,” as set forth in Article XI of the Plan, are hereby deleted in their entireties.

3) Section 11.2 entitled “**MEDICAL BENEFIT EXCLUSIONS AND LIMITATIONS**,” as set forth in Article XI of the Plan and as otherwise amended, is amended by adding thereto the following new subsection:

“AJ. Charges for drugs, medications, or products which are addressed in the Plan’s Biosimilar Program described in Article VII of the Highland Computer Forms, Inc. Employee Benefit Plan for Prescription Benefits, regardless of whether such drug, medication, or product is covered through such Program.”

Highland Computer Forms, Inc. hereby adopts the above amendments to the Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description effective as of January 1, 2026.

**HIGHLAND COMPUTER FORMS, INC.
EMPLOYEE BENEFIT PLAN
SUMMARY PLAN DESCRIPTION**

SUMMARY OF MATERIAL NO. 2

This Summary of Material Modifications is a description of important changes which have been made to the Plan. You should read these changes carefully and keep this document with your copy of the Summary Plan Description. If you have any questions about these changes, you should contact the Plan Administrator for more information.

The Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description (herein referred to as “SPD”) is hereby amended and modified as set forth below. Such amendments are effective as of May 1, 2025.

1) Section 2.4 entitled “**MEDICAL COPAYMENT AMOUNTS**,” as set forth in Article II of the Plan and as otherwise amended, is amended by adding thereto the following new item:

“**Biofeedback Therapy**.....\$30.00”

2) Subsections V, W, and X in Section 8.1 entitled “**MEDICAL BENEFITS – COVERED EXPENSES**,” as set forth in Article VIII of the Plan, are amended in their entireties as follows:

“V. Charges for occupational therapy performed by a licensed occupational therapist, subject to the limitations listed in Section 2.5. Such therapy must be ordered by a Physician. Recreational programs, maintenance therapy or supplies used in occupational therapy are not covered under this Plan.”

“W. Charges for physical therapy, including aquatic therapy/hydrotherapy when indicated for the Medically Necessary treatment of certain musculoskeletal conditions, performed by a licensed physical therapist. The therapy must be in accord with a Physician’s exact orders as to type, frequency and duration.”

“X. Charges for speech therapy performed by a licensed speech therapist. Therapy must be ordered by a Physician.”

3) Subsection AM in Section 8.1 entitled “**MEDICAL BENEFITS – COVERED EXPENSES**,” as set forth in Article VIII of the Plan, is amended in its entirety as follows:

“AM. Charges for all genetic and genomic testing and related counseling, but only if deemed Medically Necessary in connection with the treatment of an existing condition, or if included in the Recommended Wellness Services. Prior authorization through the Plan Administrator is recommended.”

4) Section 8.1 entitled “**MEDICAL BENEFITS – COVERED EXPENSES**,” as set forth in Article VIII of the Plan and as otherwise amended, is amended by adding thereto the following new subsections:

“AV. Charges for biofeedback therapy.”

“AW. Charges for nutritional counseling if diagnosis is listed as a Covered Expense under this Plan.”

“AX. Charges for developmental delay.”

5) Section 11.2 entitled “**MEDICAL PLAN BENEFIT EXCLUSIONS AND LIMITATIONS**,” as set forth in Article XI of the Plan and as otherwise amended, is amended by adding thereto the following new subsection:

“AI. Charges for Stelara.”

Highland Computer Forms, Inc. hereby adopts the above amendments to the Highland Computer Forms, Inc. Employee Benefit Plan effective as of May 1, 2025.

**HIGHLAND COMPUTER FORMS, INC.
EMPLOYEE BENEFIT PLAN
SUMMARY PLAN DESCRIPTION**

SUMMARY OF MATERIAL MODIFICATIONS #1

This Summary of Material Modifications is a description of important changes which have been made to the Plan. You should read these changes carefully and keep this document with your copy of the Summary Plan Description. If you have any questions about these changes, you should contact the Plan Administrator for more information.

The Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description (the “SPD”) is hereby amended and modified as set forth below. Such amendments are effective as of May 1, 2025.

1.) Article I of the SPD, entitled, “**PLAN INFORMATION**,” is amended, to add thereto the following new item:

“ORGAN & TISSUE TRANSPLANT POLICY

Tokio Marine HCC
(833) 254-9537”

2.) The item listed as “**Treatment of Autism Spectrum Disorders**①,” in Section 2.4, entitled, “**MEDICAL COPAYMENT AMOUNTS**,” as set forth in Article II of the SPD, is amended, is hereby deleted in its entirety.

3.) Section 2.4, entitled, “**MEDICAL COPAYMENT AMOUNTS**,” as set forth in Article II of the SPD and as otherwise amended, is amended by adding thereto the following item:

“ABA Therapy..... \$30.00”

4.) Article VIII of the Plan, entitled, “**MEDICAL BENEFITS**,” as set forth in the SPD, is amended. by adding thereto the following new section:

“8.2 ORGAN AND TISSUE TRANSPLANT BENEFITS

Human organ and tissue transplant benefits are provided according to the terms and conditions set forth in an Organ & Tissue Transplant Policy (Transplant Policy), that is separate from this Plan, that has been issued to the Employer. Transplant related benefits will be provided to each Covered Person during the transplant benefit period specified in the Transplant Policy. Expenses billed by the transplant network provider that are not covered by the Transplant Policy are subject to this Plan’s benefits and the payment terms and conditions of the provider’s contracted rates. Such Transplant Policy pays benefits for certain organ and tissue transplants without regard to any benefits that may or may not be provided by this Plan. Any benefits obtained through the Transplant Policy are not covered under this Plan.

Transplant related benefits through the Transplant Policy are only available to individuals that:

- A. are eligible for medical benefits under the health plan;
- B. meet all the terms and conditions outlined in the Transplant Policy; and
- C. have fulfilled any Pre-Existing Condition waiting period (if applicable) as defined in the Transplant Policy.

Covered Persons that are subject to a Pre-Existing Condition waiting period under the Transplant Policy will receive transplant benefits according to the terms and conditions of this Plan until the Pre-Existing Condition waiting period under the Transplant Policy has elapsed.

The Organ and Tissue Transplant Policy is provided by Tokio Marine HCC (TMHCC) and issued either by National Union Fire Insurance Company of Pittsburgh, PA or HCC Life Insurance Company. Please contact TMHCC's Transplant Unit toll-free at (833) 254-9537 for benefit information, pre-authorization of transplant services, and transplant network provider access.

Pre-authorization of transplant services is required prior to seeing a transplant provider for a consult and/or evaluation. Failure to do so could result in reduced benefits.

In order to obtain the highest benefit, providers in a transplant network approved by and accessed through TMHCC's Transplant Unit must be used.

For more information, or for a copy of this Transplant Policy, contact your medical plan administrator and or the Employer's Human Resources department."

5.) Section 11.1, entitled, "**GENERAL PLAN BENEFIT EXCLUSIONS AND LIMITATIONS**," as set forth in Article VIII of the SPD, is amended, by adding thereto the following new Subsection:

"M. Charges for any services or supplies for which benefits are provided through the Transplant Policy described in Section 8.2."

Highland Computer Forms, Inc. hereby adopts the above amendments to the Highland Computer Forms, Inc. Employee Benefit Plan Summary Plan Description effective May 1, 2025.

**HIGHLAND COMPUTER FORMS, INC.
EMPLOYEE BENEFIT PLAN
SUMMARY PLAN DESCRIPTION**

**REVISED EFFECTIVE OCTOBER 1, 2024
(unless otherwise noted)**

THIS DOCUMENT CONTAINS ALL PROVISIONS OF THE PLAN. ANY CONFLICT OR AMBIGUITY ARISING BETWEEN THIS DOCUMENT AND ANY OTHER DOCUMENT OR COMMUNICATION, INCLUDING, BUT NOT LIMITED TO, ANY SUMMARY PLAN DESCRIPTION, BROCHURE, OR ORAL OR VIDEO PRESENTATION, DESCRIBING THE RIGHTS, BENEFITS, OR OBLIGATIONS OF THE COMPANY AND PARTICIPANTS UNDER THE PLAN SHALL BE RESOLVED IN FAVOR OF THIS PLAN DOCUMENT.

MEDICAL BENEFITS ADMINISTRATORS, INC.

Established in 1989, Medical Benefits Administrators, Inc. (MBA) is a subsidiary of Medical Benefits Mutual Life Insurance Co., one of the oldest health insurance firms in the United States. In 1938, the Company entered the insurance business operating under the name Hospital Services Association. Later, it became known as HSA of Ohio.

The name, Medical Benefits Mutual, was adopted in 1987, signaling the Company's establishment as a full-fledged mutual life insurance company. Medical Benefits Administrators, Inc. builds on this great service tradition and commitment to the future by delivering the services the marketplace demands.

MBA is pleased to have been chosen as your Benefit Manager. MBA is committed to the fundamental criteria that distinguish us from the crowd. The first is a commitment to excellent claims administration. The second is a commitment to long term relationships with the people we serve.

We will appreciate your comments and strive to make any dealings with us as simple as possible. If you have any questions about a claim, we invite you to call us at (800) 423-3151, e-mail us at medben@medben.com or to drop in at our offices at 1975 Tamarack Road, Newark, Ohio 43055.

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ARTICLE I
PLAN INFORMATION

NAME OF PLAN

The name of the Plan is the Highland Computer Forms, Inc. Employee Benefit Plan.

PURPOSE OF THE PLAN

Highland Computer Forms, Inc. executes this document, including any amendments, to establish a health benefit plan for the exclusive benefit of its participating employees and their eligible Dependents and to grant them legally enforceable rights under this Plan. While Highland Computer Forms, Inc. has every intention of continuing this Plan indefinitely, it reserves the right to amend or terminate the Plan, and the benefits provided hereunder, at any time.

The Plan Administrator has issued a Summary Plan Description to each Participant that summarizes the benefits to which that person is entitled, to whom benefits are payable, and the provisions of this Plan principally affecting the Participant and his or her covered Dependents.

PLAN EFFECTIVE DATE

The Plan Effective Date of this revision of the Plan is October 1, 2024. This Plan was originally effective on May 1, 2005.

AMENDMENT OR TERMINATION

Highland Computer Forms, Inc. may amend or terminate the Plan at any time by means of a writing signed by a person authorized by Highland Computer Forms, Inc. to do so. Any such amendment or termination shall become effective upon its execution or on such date as may be specified in that writing. Such amendment, modification or termination may result in the termination of Participant and Dependent coverage under the Plan. Expenses Incurred prior to any Plan termination will be paid as provided under the terms of the Plan prior to such termination. Any termination of the Plan will be communicated by Highland Computer Forms, Inc. to the Participants.

Upon Plan termination, any Plan assets remaining in the Plan's account(s) will be distributed by the Plan Administrator to the Plan Sponsor and/or Participants, in accordance with method(s) set forth in ERISA, or any other applicable law or regulation. The Plan Administrator shall pay all eligible Plan benefits and expenses before any distribution is made.

The terms of the Plan cannot be amended or modified by oral statement(s). Only the Plan Administrator can interpret the terms of the Plan.

Highland Computer Forms, Inc. reserves the right, at any time and from time to time, to modify or amend, in whole or in part, any or all of the provisions of the Plan.

PLAN ADMINISTRATOR TAX ID NUMBER (EIN)

31-0971103

PLAN ADMINISTRATOR

Highland Computer Forms, Inc.
1025 West Main Street
Hillsboro, Ohio 45133
(937) 393-4215

PLAN NUMBER

501

GROUP NUMBER

10372

PLAN YEAR

The Plan Year is a time period defined for fiscal purposes and used for certain Plan reporting and disclosure requirements. The Plan Year will begin on January 1st and end on December 31st of the same year.

CALENDAR YEAR

The Calendar Year is the period beginning January 1st and ending December 31st that is used in the application of Out-of-Pocket limits and benefit maximum amounts.

TYPE OF ADMINISTRATION

Contract Administration.

DESCRIPTION OF PLAN

The Plan is an employee health and welfare benefit plan providing medical benefits, dental benefits, and short term disability benefits. A copy of the Plan documents and insurance contracts, if any, are on file at the Plan Administrator's office and may be read by any Covered Person at any reasonable time. In the event of any discrepancy between any summary of this Plan and the actual provisions of the Plan document, the Plan document shall govern.

This Plan is self-funded by the Company or Employers, and administered in accordance with the provisions of ERISA. As such, the provisions of ERISA preempt the application of state insurance law to this Plan.

The Plan shall not be deemed to constitute a contract between the Company and any employee or to be a consideration for, or an inducement or condition of, the employment of any employee. Nothing in the Plan shall be deemed to give any employee the right to be retained in the service of the Company or to interfere with the right of the Company to discharge any employee at any time.

NAMED FIDUCIARY

Highland Computer Forms, Inc.
1025 West Main Street
Hillsboro, Ohio 45133
(937) 393-4215

AGENT FOR SERVICE OF LEGAL PROCESS

Highland Computer Forms, Inc.
1025 West Main Street
Hillsboro, Ohio 45133
(937) 393-4215

In addition, service of legal process may be made upon the Plan Administrator or a Plan Trustee, if a Trustee has been appointed.

FUNDING

The Plan is funded by the Employer. Funds for payment of claims considered under the Plan are forwarded to account(s) from which claims are to be paid. Additionally, funds for payment of premiums and fees associated with the Plan will be forwarded from such account.

ASSIGNMENT

A Covered Person's benefits may not be assigned, except by consent of the Company, other than to providers of Plan benefits.

SOURCE OF CONTRIBUTIONS

The Plan is funded by contributions made by the Employer and employees who are participating under the Plan. Participant Contributions are currently required for both Participant and Dependent Coverage.

The Company shall, from time to time, evaluate the funding method of the Plan benefits and determine the amount to be contributed by the Employer and the amount to be contributed, if any, by the Participants for each type of coverage.

BENEFIT MANAGER

Medical Benefits Administrators, Inc.
1975 Tamarack Road
P. O. Box 1099
Newark, Ohio 43058-1099
(740) 522-8425
(800) 423-3151
www.medben.com

UTILIZATION REVIEW SERVICE

Inter-Facility Air Ambulance Review Service
Sentinel Air Medical Alliance, LLC
(877) 542-8828

Other

American Health Holding, Inc.
(888) 877-8084

GRANDFATHERED STATUS UNDER PPACA

This Plan is currently considered to be non-grandfathered for the purposes of the Patient Protection and Affordable Care Act.

MENTAL HEALTH PARITY

Pursuant to the Mental Health Parity Act (MHPA) of 1996, the Mental Health Parity and Addiction Equity Act of 2008 (MHPAEA), and the Mental Health Parity provisions of the Consolidated Appropriations Act of 2020, along with their implementing regulations, this Plan applies its terms uniformly and enforces parity between covered health care benefits and covered mental health and substance disorder benefits relating to financial cost sharing restrictions, quantitative treatment limitations, and non-quantitative treatment limitations. For further details, please contact the Plan Administrator.

ARTICLE II

SCHEDULE OF BENEFITS

2.1 COVERAGES AVAILABLE UNDER THIS PLAN

This Plan will allow Participants and their eligible Dependents to select the following health care options:

- A. medical coverage; and/or
- B. dental coverage.

A Participant can select either or both coverages. All covered Family members must be enrolled in the same coverage. The coverages as described below shall only apply to a Covered Person to the extent that the Covered Person has been enrolled in, and coverage has become effective for, the type of coverage selected, as described in Article V.

Short term disability coverage is also provided through this Plan, but is only available for Participants who were active employees on the day prior to the date the disability commenced.

In order to be eligible for the coverage described in Section 2.2 through Section 2.5 and Article VIII, the Covered Person must be properly enrolled in the medical coverage as described in Article V.

2.2 SCHEDULE OF MEDICAL BENEFITS

The Plan complies with the provisions of the No Surprises Act (“NSA”).

If the Covered Person requires Emergency Services in an Emergency, the NSA level of benefits will apply until all of the following conditions are met:

- A. the Covered Person’s attending physician or treating provider determines that the Covered Person is able to travel using non-medical transportation or non-Emergency medical transportation to an available Participating Health Care Facility, if applicable, located within a reasonable travel distance;
- B. the non-Preferred Provider or facility furnishing such additional items and services satisfies the notice and consent criteria set forth the No Surprises Act; and
- C. the Covered Person (or an authorized representative) is in a condition to receive the information described in the notice and consent described above.

If the Covered Person is a Continuing Care Patient for which the treating provider has lost or terminated its contractual relationship status for a reason other than failure to meet applicable quality standards or for fraud, the NSA level of benefits shall apply beginning with the date of the notice of termination and ending on the earliest of the following:

- A. ninety (90) days; or
- B. the date the Covered Person no longer qualifies as a Continuing Care Patient.

If the Covered Person receives Protected Services, as defined in Section 3.1, and the provider has not satisfied the notice and consent criteria set forth in the No Surprises Act, the NSA level of benefits shall apply.

Except as outlined in “No Surprises Act – Emergency Services and Surprise Bills” in Article XII, if the charge billed by a non-Preferred Provider for any Covered Expense is higher than the Maximum Allowable Charge determined by the Plan, Covered Persons are responsible for the excess, unless the provider accepts assignment of benefits as consideration in full for services rendered. The Plan Administrator reserves the right to revoke any previously-given assignment of benefits or to proactively prohibit assignment of benefits to anyone, including any Provider, at its discretion.

To receive benefit consideration, Covered Persons may need to submit claims for services provided by non-Preferred Providers to the Benefit Manager.

If a Covered Person receives information with respect to an item or service from the Plan, its representative, or a database maintained by the Plan or its representative indicating that a particular Provider is a Participating Health Care Facility, if applicable, and the Covered Person receives such item or service in reliance on that information, the Covered Person's Coinsurance, Copayment, Deductible, and out-of-pocket maximum will be calculated as if the Provider had been a Participating Health Care Facility, if applicable, despite that information proving inaccurate.

This Schedule of Medical Benefits is intended to provide only a general description of a Covered Person's medical benefits. This Plan contains limitations and restrictions that are described later in this booklet and could affect any benefits that may be payable.

2.3 OUT-OF-POCKET LIMITS

Calendar Year Out-of-Pocket Limits (includes all Copayments)

Through December 31, 2024

<u>Per Individual</u>	\$9,450.00
<u>Per Family Limit</u>	\$18,900.00

Effective January 1, 2025

<u>Per Individual</u>	\$9,200.00
<u>Per Family Limit</u>	\$18,400.00

2.4 MEDICAL COPAYMENT AMOUNTS

Copayments will be applied on a per admission or per date of service basis and reflect amounts to be paid by the Covered Person. Covered Expenses that exceed the Copayments will be paid in full by the Plan unless a limitation listed in Section 2.5 applies, or the charges are attributable to any penalty in the Plan. If more than one (1) Copayment would apply during the same visit, total Copayments will be limited to the highest Copayment that would otherwise apply. Copayments will no longer apply in any Calendar Year once the applicable Out-of-Pocket limits are satisfied for that year.

Cincinnati Children's Hospital facility services are not covered for Covered Persons age twenty-six (26) and older.

Colonoscopy (Routine and Medically Necessary)..... None

Other Routine Wellness Services

Rental, or Purchase, if Less, of Breast Pumps and Related Counseling/Supplies None
Elective Sterilizations

All Females None

Male Participants/Spouses Only..... None

FDA-Approved Non-Oral Contraceptives, and Contraception Related

Medical Services None

Tobacco Cessation Counseling^① None

Other Covered Wellness Services^① None

Diabetic Self-Management not included in Recommended Wellness Services.....\$30.00

Physician's Office Visits not Specified Elsewhere

Primary Care Physician\$30.00

Specialist.....\$50.00

Office-Based & Non-Office-Based Allergy Injections Administration.....\$5.00

Other Office-Based Services (if no office visit charge)

Diagnostic Services, Injections, Medical Supplies and Office-Based Prescriptions ^①	
<i>Primary Care Physician</i>	\$30.00
<i>Specialist</i>	\$50.00
Office-Based Surgery	
<i>Through December 31, 2024</i>	\$350.00
<i>Effective January 1, 2025</i>	\$100.00

Chemotherapy^① \$50.00

Administration of Injections and Chemotherapy Services None

Initial Wig following Chemotherapy \$50.00

Dialysis

Inpatient	None
Outpatient (Facility & Office Based)	\$50.00

Durable Medical Equipment, Prosthetics and Orthotics^① \$50.00

Outpatient Cardiac Rehabilitation \$50.00

Physical, Occupational, Speech Therapy^① \$30.00

Dental Treatment/Procedures paid as Medical

Dental Treatment of Injury ^①	None
Oral Surgery/Impactions	\$350.00
Medically Necessary Hospitalization in connection with Dental Procedures	\$350.00
Anesthesia Services in connection with Dental Procedures	None

Facility Charges for Other Outpatient Surgical Procedures

Through December 31, 2024	\$350.00
Effective January 1, 2025	\$500.00

Services Provided in an Emergency Room or Independent Freestanding

Emergency Department (if not admitted)	\$200.00
Hospital-Based Prescriptions ^①	None
Other Charges, including Facility, Physician and Other Services/Supplies during Visit	\$200.00

Urgent Care Facility, including Physician and All Services and Supplies during

Visit, including Pain Injections^① \$50.00

Chiropractic Services^① \$30.00

Outpatient Physician's Consultations \$30.00

Pain Injections^①

Through December 31, 2024	\$350.00
Effective January 1, 2025	
<i>Office Based</i>	\$350.00
<i>Facility Based</i>	\$500.00

Other Physician's Charges None

Facility-Based CT Scans \$150.00

Facility-Based MRI and PET Scans \$250.00

Other Facility-Based Diagnostic Services None

Skilled Nursing Facilities (per date of service)^① \$100.00

Hospice, including Bereavement Counseling^① None

Ambulance Services, including Air None

Other Outpatient Hospital Charges None

Hospital Room & Board, Intensive Care Units^②, and all Other Inpatient Hospital

1st 3 Days (per day, per admission)	
Through December 31, 2024.....	\$500.00
Effective January 1, 2025.....	\$750.00
Day 4 and After.....	None

Human Organ & Tissue Transplants, including Related Visits

Through Special Transplant Network.....	None
Not Through Special Transplant Network	
Office Visits, including Services and Supplies provided during Same Visit, Diagnostic Services, Injections, Medical Supplies & Office-Based Prescriptions ^①	
Primary Care Physician	\$60.00
Specialist.....	\$100.00
Other Office Based Services (if no office visit charge)	
Diagnostic Services, Injections, Medical Supplies and Office-Based Prescriptions ^①	
Primary Care Physician	\$60.00
Specialist	\$100.00
Office/Free Standing Facility/Hospital-Based Outpatient Surgery	\$1,000.00
Hospital and Free Standing Facility-Based CT Scans	\$600.00
Hospital and Free Standing Facility-Based MRI and PET Scans.....	\$1,000.00
Hospital Room and Board (per day, for the first 3 days, per admission)	\$2,000.00
Extended Care Facility ^①	\$400.00
Outpatient Physician's Consultations	\$100.00
Physical/Occupational/Speech Therapy and Diabetic Counseling.....	\$100.00
Durable Medical Equipment & Prosthetic Devices	\$200.00
Other Services	Paid Same as Other Illness

Treatment of Mental/Nervous Disorders, Alcohol and Other Drug Abuse

Office Visits & Individual or Group Counseling.....	\$30.00
Intensive Outpatient Treatment/Partial Hospitalization (per date of service).....	\$30.00
Other Covered Services	Paid Same as Other Illness

Treatment of Autism Spectrum Disorders^① Paid Same as Other Illness

Other Covered Services & Supplies^① None

EXPLANATION

- ① Please see additional limitations in Section 2.5, Medical Plan Benefit Maximums.
- ② Covered Expenses for Hospital Room & Board will be determined based on the Hospital's daily Semi-Private room rate. Charges for Intensive Care Units will be considered at the Maximum Allowable Charge for such a unit.

2.5 MEDICAL PLAN BENEFIT MAXIMUMS

The medical Plan benefit maximums and limitations are shown below.

Skilled Nursing Facility	Sixty (60) days per Calendar Year maximum
Extended Care Facility	Sixty (60) days per Calendar Year maximum
Home Health Care	Forty (40) visits per Calendar Year maximum
Pain Injections	Three (3) per Calendar Year maximum (additional injections may be approved at the Plan Administrator's discretion if Medically Necessary)

Other Outpatient Drugs, Wherever Obtained, including Injectables, Allergy Serums/Venoms & Chemotherapy. This Does Not Apply to Drugs included under the Recommended Wellness Services	Ten Thousand dollars (\$10,000.00) maximum per medication, per Outpatient visit
Chiropractic Spinal Manipulation Services	Thirty-five (35) visits per Calendar Year maximum
Dental Treatment of Injury	Treatment must be complete within three-hundred sixty-five (365) days.
Hospice Bereavement Counseling	Three (3) visits per occurrence, within six (6) months after death
Occupational Therapy	Fifteen (15) visits per Calendar Year maximum
Routine Hearing & Visual Acuity Screening	Through age twenty-one (21)
Routine Mammography	
<u>Up to Age 34</u>	Not Covered
<u>Age 35 to Age 39</u>	One (1) Baseline
<u>Age 40 and Older</u>	One (1) per Calendar Year maximum
Pap Smear, including Pap-Net Test	Covered semi-annually
Wigs	One (1) after chemotherapy
Tobacco Cessation Counseling	Two (2) attempts to stop using tobacco, with up to four (4) visits per attempt per Calendar Year.
Diabetic Shoes or Custom Shoe Inserts	One (1) pair of shoes per Calendar Year maximum or three (3) pairs of custom inserts per Calendar Year maximum

2.6 SCHEDULE OF DENTAL BENEFITS

This Schedule of Dental Benefits is intended to provide only a general description of a Covered Person's dental benefits under this Plan. This Plan contains limitations and restrictions that are described later in the Plan document and could affect any benefits that may be payable.

In order to be eligible for the coverage described in Section 2.6 through Section 2.10, and Article IX, the Covered Person must be properly enrolled in the dental coverage as described in Article V.

2.7 DENTAL DEDUCTIBLE

Per Individual \$25.00

The Dental Deductible applies per Calendar Year to Class II, Class III and Class IV services.

2.8 DENTAL COINSURANCE AMOUNTS

	<u>Deductible</u>	<u>Coinsurance</u>
Class I (Preventive and Diagnostic)	None	100%
Class II (Basic)	Applies	80%
Class III (Major)	Applies	50%
Class IV (Orthodontic)	Applies	60%

Please see additional limitations in the schedule of Dental Plan Maximum Benefits set forth in Section 2.9 of the Plan.

2.9 DENTAL PLAN MAXIMUM BENEFITS

The dental Plan maximum benefits and limitations are shown below. Both Calendar Year and Lifetime maximums indicate the actual benefits payable under the Plan.

Class I, Class II and Class III, combined	\$1,500.00 per Calendar Year maximum
Class IV	\$500.00 per Lifetime maximum; Limited to Dependent children aged eighteen (18) and younger

2.10 PREDETERMINATION OF BENEFITS

Before starting a course of treatment for which the charge is expected to be six hundred dollars (\$600.00) or more, a dental treatment plan should be submitted in an acceptable form to the Benefit Manager. A Predetermination of Benefits payable under this Plan will then be provided.

For more information about Predetermination of Benefits, see Section 9.2.

2.11 SCHEDULE OF SHORT TERM DISABILITY BENEFITS

Only Participants who were Actively at Work for the Employer on the last scheduled workday prior to the date the disability commenced are eligible for this benefit.

This Schedule of Short Term Disability Benefits is intended to provide only a general description of a Participant's short term disability benefits under this Plan. This Plan contains limitations and restrictions that are described later in the Plan document and could affect any benefits that may be payable.

Waiting Period (*Illness or Injury*) Benefits start on the eighth (8th) day of Total Disability, per disability

Recurring/Subsequent Disability

Same/Related Cause Must return to work for two (2) continuous weeks to be considered new disability

Unrelated Cause Must return to work for at least one (1) day to be considered new disability

Weekly Benefit Two-thirds of Covered Weekly Earnings, to a maximum of four hundred dollars (\$400.00)

Maximum Benefit Period Twelve (12) weeks per disability

For more information about the Short Term Disability Benefits, see Article X.

ARTICLE III

DEFINITIONS

All terms that are defined in this Article III are capitalized wherever they appear in this Plan.

3.1 GENERAL PLAN DEFINITIONS

ACTIVELY AT WORK or ACTIVE WORK

The terms “Actively at Work” or “Active Work” mean the active expenditure of time and energy in the service of the Company. A Participant shall be deemed Actively at Work while working the full number of hours shown in Section 5.2 and while in a relationship with the Employer within the meaning of “employee” for federal tax withholding purposes. In addition, individuals acting as independent contractors; leased employees; consultants; a member of the Board of Directors; temporary, freelance, incidental, seasonal or occasional employees; individuals on retainers; or retirees are not considered Actively at Work unless each meets the requirements specified in Section 5.2. This term shall not apply to any provision of this Plan to the extent that such application would be deemed to violate the requirements of HIPAA.

ADVERSE BENEFIT DETERMINATION

The term “Adverse Benefit Determination” means any of the following:

- A. a denial, reduction, or termination of, or a failure to provide or make payment (in whole or in part) for, a benefit, including any such denial, reduction, termination, or failure to provide or make payment that is based on a determination of a Covered Person’s eligibility to participate in the Plan;
- B. a denial, reduction, or termination of, or a failure to provide or make payment (in whole or in part) for, a benefit resulting from the application of any utilization review, as well as a failure to cover an item or service for which benefits are otherwise provided because it is determined to be Experimental or Investigative or not Medically Necessary or appropriate;
- C. a reduction or termination by the Plan Administrator of a previously approved course of treatment, other than by Plan termination or amendment;
- D. a determination as to whether a Covered Person is entitled to a reasonable alternative standard for a reward under a wellness program;
- E. a determination as to whether the Plan is complying with the nonquantitative treatment limitation provisions of Code section 9812 and § 54.9812-1, which generally require, among other things, parity in the application of medical management techniques);
- F. a consideration of whether the Plan is complying with the surprise billing and cost-sharing protections set forth in the No Surprises Act; or
- G. any retroactive rescission of coverage (other than due to the failure to make Participant Contributions, fraud or intentional misrepresentation of a material fact), whether or not there is an adverse effect on any particular benefit at that time.

BENEFIT MANAGER

The term “Benefit Manager” means the individual or business entity, if any, appointed and retained by the Plan Administrator to supervise the management, consideration, investigation and settlement of claims, maintain records, and submit reports and other such duties as may be set forth in a written agreement. If no Benefit Manager is appointed or retained (as a result of the termination or expiration of such agreement or other reason) or if the term is used in connection with a duty not expressly assigned to and assumed by the Benefit Manager in writing, the term will mean the Plan Administrator.

As of the Plan Effective Date of this revision of the Plan, the Benefit Manager of the Plan is Medical Benefits Administrators, Inc.

CALENDAR YEAR

The term “Calendar Year” means the period of time from January 1st, at 12:00 A.M. midnight, through the next December 31st.

CLEAN CLAIM

The term “Clean Claim” means a billing for a service and/or supply that is submitted to the Plan by a Covered Person or provider that has no defect, impropriety or special circumstance, including incomplete documentation that delays timely payment. It must clearly identify the Covered Person receiving the services or supplies and the Plan to which it is being submitted, and be submitted on an appropriate form that has been properly and entirely completed, as described in Section 4.1 and Section 4.2, including all data elements required by the applicable form. If a claim that has been submitted to this Plan is determined by the Plan Administrator to not constitute a Clean Claim within this definition, the Covered Person and/or the provider will be notified of the defects, and it will not be considered to have been received by the Plan until all required information is received.

CLOSE RELATIVE

The term “Close Relative” means the spouse, parent, child, brother or sister of the Covered Person, whether the relationship is by blood or exists in law.

COBRA

The term “COBRA” means the Consolidated Omnibus Budget Reconciliation Act of 1985, as amended.

COMPANY

The term “Company” means Highland Computer Forms, Inc., the Plan sponsor.

COVERED EXPENSES

The term “Covered Expenses” means expenses Incurred by a Covered Person for any Medically Necessary treatments, services or supplies that are not specifically excluded from coverage elsewhere in this Plan, or other charges that are specifically listed as covered under this Plan.

COVERED PERSON

The term “Covered Person” means any person meeting the eligibility requirements for coverage as specified in this Plan and who is properly enrolled in the Plan.

DEPENDENT

For purposes of coverage for medical benefits under the Plan, the term “Dependent” means:

- A. the Participant’s legal spouse who is not eligible for similar health coverage through his or her own employer, or, if eligible, is enrolled in such coverage. Such spouse must have met all requirements of a valid marriage contract in the state in which such parties were married. In no event shall the term “spouse” include an individual who is the same sex as the Participant, regardless of whether such relationship is recognized in any way under any state law; or
- B. the Participant’s child who meets all of the following conditions:
 - 1. is the Participant’s natural child, adopted child, stepchild, a child for whom the Participant has Legal Guardianship, a child for whom the Participant or the Participant’s spouse is required to provide coverage pursuant to a court or administrative order, or a child Placed for Adoption with the Participant; and
 - 2. is less than twenty-six (26) years of age. The age requirement above is waived for any mentally or physically handicapped child who is Totally Disabled, incapable of self-sustaining employment by reason of mental or physical handicap, and still primarily dependent upon the covered Employee for support and maintenance, provided the child suffered such incapacity prior to attaining the limiting age listed above. Proof of incapacity must be furnished to the Plan Administrator, or its

designee, within thirty-one (31) days of the date the child's coverage would have ended due to age.

The Plan Administrator has the right to obtain sufficient proof of Dependent status from any Participant under the Plan who is requesting coverage of his or her Dependents.

This definition and all provisions of this Plan are intended to comply with state and federal law as both regard "Qualified Medical Child Support Orders" and "Medical Child Support Orders," as those terms are defined in the law. The Plan Administrator has established procedures governing "Qualified Medical Child Support Orders." Covered Persons under this Plan can receive upon request, free of charge, a copy of such procedures from the Plan Administrator.

The term "Dependent" excludes these situations:

- A. a spouse who is legally separated or divorced from the Participant. Such separation/divorce must have met all the requirements of a valid legal separation or divorce in the state granting it;
- B. any spouse on active military duty in the military service of any country; or
- C. any person who is covered under this Plan as an individual Participant.

For purposes of coverage for dental benefits under the Plan, the term "Dependent" means:

- A. the Participant's legal spouse who is not eligible for health coverage through his or her own employer. Such spouse must have met all requirements of a valid marriage contract in the state in which such parties were married. In no event shall the term "spouse" include an individual who is the same sex as the Participant, regardless of whether such relationship is recognized in any way under any state law; or
- B. the Participant's child who meets all of the following conditions:
 - 1. is unmarried;
 - 2. resides in the same household as the Participant. This requirement is waived if the Participant is obligated to provide dental coverage for a child under an order or judgment of a court of competent jurisdiction;
 - 3. is one (1) of the following:
 - a. the Participant's natural child, adopted child or is a child Placed For Adoption with the Participant;
 - b. a stepchild, as long as a natural parent remains married to the Participant and such spouse also resides in the Participant's household;
 - c. a child for whom the Participant or the Participant's spouse is required to provide coverage pursuant to a court or administrative order; or
 - d. a child for whom the Participant has Legal Guardianship;
 - 4. is considered a "Dependent" of the Participant for tax exemption purposes under Section 152 of the Internal Revenue Code of 1986, as amended. This requirement is waived if the Participant is obligated to provide dental care coverage for a child under an order or judgment of a court of competent jurisdiction; and
 - 5. is less than nineteen (19) years of age. This requirement is waived if the child is at least nineteen (19) years of age but less than twenty-five (25) years of age, is still primarily dependent upon the Participant for support and maintenance, and is a Full-Time Student.

The age requirement above is also waived for any unmarried mentally or physically handicapped child who is Totally Disabled, incapable of self-sustaining employment by reason of mental or physical handicap, and still primarily dependent upon the covered Employee for support and maintenance provided the child suffered such incapacity prior to attaining the limiting age listed above. Proof of incapacity must be furnished to the

Plan Administrator, or its designee, within thirty-one (31) days of the date the child's coverage would have ended due to age.

The Plan Administrator has the right to obtain sufficient proof of Dependent status from any Participant under the Plan who is requesting coverage of his or her Dependents.

This definition and all provisions of this Plan are intended to comply with state and federal law as both regard "Qualified Medical Child Support Orders" and "Medical Child Support Orders," as those terms are defined in the law. The Plan Administrator has established procedures governing "Qualified Medical Child Support Orders." Covered Persons under this Plan can receive upon request, free of charge, a copy of such procedures from the Plan Administrator.

The term "Dependent" excludes these situations:

- A. a spouse who is legally separated or divorced from the Participant. Such separation/divorce must have met all the requirements of a valid legal separation or divorce in the state granting it;
- B. any person on active military duty in the military service of any country; and
- C. any person who is covered under this Plan as an individual Participant, or as the Dependent of another Participant.

DEPENDENT COVERAGE

The term "Dependent Coverage" means coverage under the Plan for benefits payable as a consequence of an Illness or Injury of a Dependent.

EMPLOYER

The term "Employer" means the Company and any entity that is affiliated with the Company within the meaning of Section 414(b), (c) or (m) of the Internal Revenue Code of 1986, as amended, that adopts this Plan for the benefit of its employees, whose participation in the Plan is approved by the President (or any duly authorized officer) of the Company. An Employer may withdraw from the Plan by delivering to the Plan Administrator written notice of its withdrawal no later than thirty (30) days prior to the date withdrawal is to be effective.

ERISA

The term "ERISA" refers to the Employee Retirement Income Security Act of 1974, as amended.

FAMILY

The term "Family" means a covered Participant and his or her covered Dependents.

FULL-TIME EMPLOYEE

The term "Full-Time Employee" means an employee who, on the date of hire, is reasonably expected to work, on average, at least thirty (30) hours per week (or one hundred thirty (130) hours per month) on an annual basis. Such an employee shall start his or her waiting period, if any, on the date of hire.

FULL-TIME STUDENT

The term "Full-Time Student" means a Dependent child who is enrolled in an accredited school on a full-time basis, as defined by the institution in question. Full-Time Student status continues only between semester/quarters if the student is enrolled as a Full-Time Student in the next regular semester/quarter. If the student is not enrolled as a Full-Time Student at that time, coverage will be terminated retroactively to the last day of the attended school term.

A Dependent child will continue to be considered a Full-Time Student under this Plan if he or she becomes unable to continue attendance at such institution on a full-time basis due to an Illness or Injury, for up to twelve (12) months beyond the date such child last attended the institution on a full-time basis, unless such child otherwise fails to meet the Plan's definition of a Dependent.

HEALTH CARE REFORM, PPACA, AFFORDABLE CARE ACT or ACA

The terms “Health Care Reform,” “PPACA,” “Affordable Care Act” or “ACA” mean the Patient Protection and Affordable Care Act, as amended by the Health Care and Education Reconciliation Act of 2010, and as otherwise amended, including all current final regulations that are issued regarding such acts.

HEALTH INFORMATION

The term “Health Information” means any information, whether oral or recorded, in any form or medium that:

- A. is created or received by this Plan, or a Plan designee; and
- B. relates to any of the following:
 - 1. the past, present or future physical or mental health or condition of an individual;
 - 2. the provision of health care to an individual; or
 - 3. the past, present or future payment for the provision of health care to an individual.

HIPAA

The term “HIPAA” means the Health Insurance Portability and Accountability Act of 1996, as amended.

INJURY

The term “Injury” means an accidental physical Injury to the body caused by unexpected external means.

ILLNESS

The term “Illness” means bodily disorder, disease, physical sickness or Mental/Nervous Disorder. Illness includes Pregnancy, childbirth, miscarriage or complications of Pregnancy.

INCURRED

The term “Incurred” means the date a service is rendered, or a supply is obtained. With respect to a course of treatment or procedure which includes several steps or phases of treatment, Covered Expenses are Incurred for the various steps or phases as the services related to each step are rendered and not when services relating to the initial step or phase are rendered. More specifically, Covered Expenses for the entire procedure or course of treatment are not Incurred upon commencement of the first stage of the procedure or course of treatment.

LEGAL GUARDIAN or LEGAL GUARDIANSHIP

The terms “Legal Guardian” or “Legal Guardianship” mean a person, or the status of a person and his or her ward, who has been appointed by a state court with specific jurisdiction over guardianships and estates, to have the care and management of a minor child. An order granting a person legal custody of a minor child, without the appointment of the person as the child’s Legal Guardian, does not create a Legal Guardianship.

MAXIMUM ALLOWABLE CHARGE

The term “Maximum Allowable Charge” shall mean the amount payable for a specific covered item under this Plan. The Maximum Allowable Charge will be a negotiated rate, if one exists.

For claims subject to the No Surprises Act (see Section 12.13 “No Surprises Act – Emergency Services and Surprises Bills”) if no negotiated rate exists, the Maximum Allowable Charge will be the first of the following amounts that apply:

- A. an amount determined by an applicable all-payer model agreement;
- B. an amount determined by applicable state law;
- C. the Qualifying Payment Amount “QPA;” or
- D. an amount deemed payable by a Certified IDR Entity or a court of competent jurisdiction, if applicable.

If none of the above factors are applicable, the Maximum Allowable Charge will be determined by the Plan to be the Medicare reimbursement rates presently utilized by the Centers for Medicare and Medicaid Services (“CMS”) by one hundred eighty percent (180%) for Inpatient and Outpatient Hospital expenses, one hundred and twenty-five percent (125%) for dialysis services (unless prohibited by network agreement) and one hundred and thirty percent (130%) for Physicians and other medical providers.

If no Medicare reimbursement rate is available for a given item of service or supply, Medicare reimbursement rates will be calculated based on one of the following:

- A. Visium Medicare Equivalency tables (prices established by CMS utilizing standard Medicare Payment methods and/or based upon supplemental Medicare or Medicaid pricing data for items Medicare doesn’t cover based on data from CMS);
- B. Visium Approximation tool (prices established by CMS utilizing standard Medicare payment methods and/or based upon prevailing Medicare rates in the community for non-Medicare facilities for similar services and/or supplies provided by similarly skilled and trained providers of care); or
- C. Visium Care Crosswalk (prices established by CMS utilizing standard Medicare payment methods for items in alternate settings based on Medicare rates provided for similar services and/or supplies paid to similarly skilled and trained providers of care in traditional settings).

If and only if none of the factors above is applicable, the Plan Administrator will exercise its discretion to determine the Maximum Allowable Charge based on any of the following: Medicare cost data, amounts actually collected by Providers in the area for similar services, or average wholesale price (AWP) or manufacturer’s retail pricing (MRP). These ancillary factors will take into account generally accepted billing standards and practices.

When more than one treatment option is available, and one option is no more effective than another, the least costly option that is no less effective than any other option will be considered within the Maximum Allowable Charge. The Maximum Allowable Charge will be limited to an amount which, in the Plan Administrator’s discretion, is charged for services or supplies that are not unreasonably caused by the treating Provider, including errors in medical care that are clearly identifiable, preventable, and serious in their consequence for patients. A finding of Provider negligence or malpractice is not required for services or fees to be considered ineligible pursuant to this provision.

MEASUREMENT PERIOD

The term “Measurement Period” means the look back period of time, as determined by the Plan Administrator, for use in determining whether new Variable Hour Employees (and On-Going Employees who are not eligible under the provisions of Section 5.2 A) are employed for an average of at least thirty (30) hours per week and are therefore eligible for coverage under the Plan during the next Stability Period. The Employer sponsoring this Plan uses a twelve (12) month Measurement Period, starting on the date of hire for new Variable Hour Employees (and ending (1) year later), or on December 1st and ending November 30th of the next Calendar Year for other employees.

If an employee experiences a break in service during a Measurement Period, the existing Measurement Period will resume once he or she returns to active employment with the Employer if the break in service is less than the period of active employment prior to the break, and less than thirteen (13) weeks in length. If the break in service is more than either the employee’s total employment before the break, or thirteen (13) weeks, a new initial Measurement Period will commence once he or she resumes employment. Any such break in service that is attributable to FMLA, Service in the Uniformed Services, jury duty, or any other statutory continuation will be disregarded for the purposes of determining what the average number of hours of employment was during the entire Measurement Period.

The Employer will notify all new Variable Hour Employees who become eligible for coverage under this Plan by the end of the month following the end of the initial Measurement Period, and prior to the beginning of the initial Stability Period. On-Going Employees will be notified by beginning of the next Stability Period as to their eligibility during that period.

NAMED FIDUCIARY

The term “Named Fiduciary” means the individual or entity that has the ultimate authority to control and manage the overall operation of the Plan.

NEWBORN

The term “Newborn” means an infant from the date of birth until the earlier of the initial Hospital discharge or the last day of the mother’s covered admission for a vaginal or cesarean delivery.

ON-GOING EMPLOYEE

The term “On-Going Employee” means any employee of the Employer who has been employed for at least one (1) full standard Measurement Period.

PARTICIPANT

The term “Participant” means a person who meets the eligibility requirements listed in Section 5.2 and who is properly enrolled in the Plan.

PARTICIPANT CONTRIBUTION

The term “Participant Contribution” means that amount that is due from an eligible employee in order for that employee to obtain Participant and/or Dependent coverage(s) under the Plan. The Company shall determine the amount of the Participant Contribution that may vary depending upon the type of coverage an eligible employee desires to obtain. Eligible Participants will be advised of any required Participant Contributions at the time each applies for Participant and/or Dependent coverage. Participants in the Plan will be notified by the Plan Administrator prior to an increase in the required Participant Contribution amount. Participants in the Plan that are not required to make Participant Contributions at the time of enrollment will be notified by the Plan Administrator prior to the date a Participant Contribution requirement is made effective.

PHYSICIAN

The term “Physician” means a Doctor of Medicine (M.D.) or a Doctor of Osteopathy (D.O.) who is licensed and regulated by a state or federal agency and is acting within the scope of his or her license.

PLACED FOR ADOPTION or PLACEMENT FOR ADOPTION

The terms “Placed for Adoption” or “Placement for Adoption” mean the assumption and retention by such Participant hereunder of a legal obligation for total or partial support of such child in anticipation of adoption of such child. The child’s placement with such Participant terminates upon the termination of such legal obligation.

PLAN

The term “Plan” means the sickness and accident plan, as described in and administered by the Highland Computer Forms, Inc. Employee Benefit Plan.

PLAN ADMINISTRATOR

The entity responsible for the day-to-day functions and management of the Plan. The Plan Administrator may employ persons or firms to process claims and perform other Plan related services. Highland Computer Forms, Inc. is the Plan Administrator as of the Plan Effective Date of this revision of the Plan.

PLAN YEAR

The term “Plan Year” means a period of time used for certain reporting and disclosure requirements of the Plan. The Plan Year will begin on January 1st and end on December 31st of the same year.

PLAN EFFECTIVE DATE

The Plan Effective Date of this revision of the Plan is October 1, 2024. This Plan was originally effective on May 1, 2005.

PRIMARY CARE PHYSICIAN or PCP

The term “Primary Care Physician” or “PCP” mean a Physician who provides both the first contact for a person with an undiagnosed health concern as well as continuing care of varied medical conditions, not limited by cause, organ system, or diagnosis. Primary Care Physicians include those trained and actively practicing in family practice, general practice, pediatrics, internal medicine or gynecology.

PROFESSIONAL PROVIDER

The term “Professional Provider” means any of the following licensed medical professionals:
ing licensed medical professionals:

- A. a Physician;
- B. a doctor of dental surgery (D.D.S.);
- C. a doctor of podiatry (D.P.M.);
- D. a doctor of chiropractic (D.C.);
- E. an audiologist;
- F. a certified nurse anesthetist;
- G. a licensed professional counselor;
- H. a licensed professional physical therapist;
- I. a midwife;
- J. an occupational therapist;
- K. an optometrist (O.D.);
- L. a physiotherapist;
- M. a psychologist (Ph.D.);
- N. a speech language pathologist; and
- O. any other practitioner of the healing arts who is licensed and regulated by a state or federal agency and is acting within the scope of his or her license.

PROTECTED HEALTH INFORMATION

The term “Protected Health Information” means Health Information that either identifies an individual, or for which there is a reasonable basis to believe can be used to identify an individual and that is one (1) of the following:

- A. transmitted by electronic media, including:
 - 1. the internet;
 - 2. an extranet;
 - 3. leased lines;
 - 4. dial-up lines;
 - 5. private networks; and
 - 6. those transmissions that are physically moved from one location to another using magnetic tape, disk, or compact disk media;
- B. maintained in any electronic media; or
- C. transmitted or maintained in any other form or medium.

SERVICE IN THE UNIFORMED SERVICES

The term “Service in the Uniformed Services” means performance of duty in the Armed Forces or Uniformed Services for a period of five (5) years or less, on a voluntary or involuntary basis,

including active duty, active duty for training, initial active duty for training, inactive duty training, full-time National Guard duty in the Armed Forces, the Army National Guard, Air National Guard, the commissioned corps of the Public Health Service, or any other category of persons designated by the President of the United States in time of war or emergency. Service in the Uniformed Services also includes a period for which an individual is absent from a position of employment for the purpose of an examination to determine the fitness of the person for duty in the Armed Forces or the commissioned corps of the Public Health Service.

STABILITY PERIOD

The term “Stability Period” means the period of time, as determined by the Plan Administrator, for which new Variable Hour Employees and On-Going Employees are eligible for coverage under the Plan, as determined during the latest prior Measurement Period. The Employer sponsoring this Plan uses a twelve (12) month Stability Period, starting the first (1st) of the month following twelve (12) months from the date of hire for new Variable Hour Employees (and ending one (1) year later), or on January 1st and ending December 31st of the same Calendar Year for Ongoing Employees. If a Variable Hour Employee is determined to work an average of at least thirty (30) hours per week during his or her initial Measurement Period following his or her date of hire, he or she will continue to be eligible for coverage during the current ongoing Stability Period from the end of such employees initial Measurement Period to the end of the current Stability Period (provided he or she is still employed by the Employer during such Stability Period), even if determined to be ineligible during a subsequent overlapping Measurement Period.

If an employee becomes ineligible for coverage due to a break in service that occurs during a Stability Period for which coverage is being provided under this Plan, but returns to active employment with the Employer within thirteen (13) weeks and prior to the end of the same Stability Period, he or she will once again become eligible for coverage from the date he or she resumes active employment until the end of such Stability Period.

SUMMARY HEALTH INFORMATION

The term “Summary Health Information” means information that may be individually identifiable Health Information that:

- A. summarizes the claims history, claims expenses or type of claims experienced by Covered Persons under this Plan; and
- B. from which the following information has been removed:
 - 1. names;
 - 2. geographic subdivisions smaller than the level of a five (5) digit zip code, including, but not limited to, street addresses;
 - 3. all elements of dates (except year) for dates directly related to an individual, including, but not limited to, birth dates and dates of admission and discharge;
 - 4. telephone numbers;
 - 5. fax numbers;
 - 6. electronic mail addresses;
 - 7. social security numbers;
 - 8. medical record numbers;
 - 9. Plan identification numbers; or
 - 10. other identifiers as listed in 45 C.F.R. § 164.514(b)(2)(i).

TEMPOROMANDIBULAR JOINT DISORDERS or TMJ

The terms “Temporomandibular Joint Disorders” or “TMJ” mean the treatment of jaw joint disorders, including conditions of structures linking the jaw bone and skull and the complex of muscles, nerves and other tissues related to the temporomandibular joint. Care and treatment

shall include, but are not limited to orthodontics, Crowns, Inlays, physical therapy and any Appliance that is attached to or rests on the teeth.

TOTAL DISABILITY or TOTALLY DISABLED

The terms “Total Disability” or “Totally Disabled” mean, in relation to a Dependent child, the complete inability, as a result of an Injury or Illness, to perform the normal activities of a person of like age and sex in good health.

USERRA

The term “USERRA” means the Uniformed Services Employment and Re-employment Rights Act of 1994, as amended.

3.2 MEDICAL PLAN DEFINITIONS

ALCOHOLISM

The term “Alcoholism” means regular excessive compulsive drinking of alcohol.

In making the determination as to whether the Covered Person’s condition meets the definition of Alcoholism under this Plan, the Plan Administrator shall use recognized authorities, including designations contained in the most current editions of the *International Classification of Diseases* (ICD) of the World Health Organization and the *Diagnostic and Statistical Manual of Mental Diseases* (DSM) published by the American Psychiatric Association.

AMBULATORY SURGICAL FACILITY

The term “Ambulatory Surgical Facility” means a facility that is licensed as such in the jurisdiction in which it is located that:

- A. is used mainly for performing Outpatient surgery;
- B. has a staff of Physicians;
- C. has continuous Physician and nursing care by registered nurses (R.N.s); and
- D. does not provide for overnight stays.

APPROVED CLINICAL TRIAL

The term “Approved Clinical Trial” means a phase I, phase II, phase III or phase IV clinical trial that is conducted in relation to the prevention, detection or treatment of cancer or other Life Threatening Condition and is one (1) of the following:

- A. it is a federally funded trial that is approved or funded, including in-kind contributions, by one or more of the following entities:
 - 1. the Centers for Disease Control and Prevention;
 - 2. the Agency for Health Care Research and Quality;
 - 3. the Centers for Medicare & Medicaid Services;
 - 4. the National Institutes of Health;
 - 5. the United States Department of Defense;
 - 6. the United States Department of Veterans’ Affairs;
 - 7. cooperative group or center of any of the above entities;
 - 8. the United States Department of Energy; or
 - 9. a qualified non-governmental research entity identified in the guidelines issued by the National Institutes of Health for center support grants;
- B. a clinical trial conducted under an FDA investigational new drug application; or
- C. a drug trial that is exempt from the requirement of an FDA investigation new drug application.

AUTISM SPECTRUM DISORDER

The term “Autism Spectrum Disorder” means a range of complex behavioral disorders, characterized by varying degrees of impairment of communications skills, social skills and are often accompanied by cognitive and language delays. This includes Autism, Asperger’s syndrome, Rett’s disorder, childhood disintegrative disorder and pervasive development disorder.

BASELINE

The term “Baseline” means the initial test results to which the results in future years will be compared in order to detect abnormalities.

BIRTHING CENTER

The term “Birthing Center” means any freestanding health facility, place, professional office or institution that is not a Hospital or a Hospital where births occur in a home-like atmosphere. This facility must be licensed and operated in accordance with the laws pertaining to Birthing Centers in the jurisdiction where the facility is located.

The Birthing Center must:

- A. provide facilities for obstetrical delivery and short-term recovery after delivery;
- B. provide care under the full-time supervision of a Physician and either a registered nurse (R.N.) or a licensed nurse-midwife; and
- C. have a written agreement with a Hospital in the same locality for immediate acceptance of patients who develop complications or require pre- or post-delivery confinement.

CERTIFIED IDR ENTITY

The term “Certified IDR Entity” shall mean an entity responsible for conducting determinations under the No Surprises Act and that has been properly certified by the Department of Health and Human Services, the Department of Labor, and the Department of the Treasury.

CONTINUING CARE PATIENT

The term “Continuing Care Patient” means an individual who, with respect to a provider or facility, is:

- A. undergoing a course of treatment for a Serious and Complex Condition from the provider or facility;
- B. undergoing a course of institutional or Inpatient care from the provider or facility;
- C. scheduled to undergo non-elective surgery from the provider or facility with respect to such a surgery;
- D. pregnant and undergoing a course of treatment for the pregnancy from the provider or facility; or
- E. determined to be terminally ill (as determined under section 1861(dd)(3)(A) of the Social Security Act) and is receiving treatment for such illness from such provider or facility.

COPAYMENT

The term “Copayment” means a specific dollar amount of the Covered Expenses that the Covered Person must pay before the Plan pays benefits for a particular service or supply. The Copayment applies to the Out-of-Pocket maximum as described in Section 2.3.

CUSTODIAL CARE

The term “Custodial Care” means care (including Room and Board needed to provide that care) that is given principally for personal hygiene or for assistance in daily activities and can, according to generally accepted medical standards, be performed by persons who have no medical training. Examples of Custodial Care include:

- A. help in walking and getting out of bed;
- B. assistance in bathing, dressing or feeding; or
- C. supervision over medication that could normally be self-administered.

DRUG

The term “Drug” means any substance - other than “food” (HCPCS Level II B codes) used in the prevention, alleviation, treatment or cure of a disease. In order to be accepted and paid, the claim for a Drug must be accompanied by the valid HCPCS Level II J-code or NDC unless otherwise identified.

DURABLE MEDICAL EQUIPMENT

The term “Durable Medical Equipment” means equipment that:

- A. can withstand repeated use;
- B. is primarily and customarily used to serve a medical purpose;
- C. generally is not useful to a person in the absence of an Illness or Injury; and
- D. is appropriate for use in the home.

EMERGENCY

The term “Emergency” means a medical condition manifesting itself by acute symptoms of sufficient severity (including severe pain) so that a prudent layperson, who possesses an average knowledge of health and medicine, could reasonably expect the absence of immediate medical attention to result in:

- A. placing the health of the individual (or, with respect to a pregnant woman, the health of the woman or her unborn child) in serious jeopardy;
- B. serious impairment to bodily functions; or
- C. serious dysfunction of any bodily organ or part.

EMERGENCY SERVICES

The term “Emergency Services” shall mean, with respect to an Emergency medical condition, the following:

- A. an appropriate medical screening examination (as required under section 1867 of the Social Security Act, 42 U.S.C. 1395dd) that is within the capability of the emergency department of a Hospital or of an Independent Freestanding Emergency Department, as applicable, including ancillary services routinely available to the emergency department to evaluate such Emergency medical condition; and
- B. within the capabilities of the staff and facilities available at the Hospital or the Independent Freestanding Emergency Department, as applicable, such further medical examination and treatment as are required under section 1867 of the Social Security Act (42 U.S.C. 1395dd), or as would be required under such section if such section applied to an Independent Freestanding Emergency Department, to stabilize the patient (regardless of the department of the Hospital in which such further examination or treatment is furnished).

When furnished with respect to an Emergency medical condition, Emergency Services shall also include an item or service provided by a non-Preferred Provider or Non-Participating Health Care Facility (regardless of the department of the Hospital in which items or services are furnished) after the Covered Person is stabilized and as part of Outpatient observation or an Inpatient or Outpatient stay with respect to the visit in which the Emergency Services are furnished, until such time as the Provider determines that the Covered Person is able to travel using non-medical transportation or non-Emergency medical transportation, and the Covered Person is in a condition to, and in fact does, give informed consent to the Provider to be treated as a non-Preferred Provider.

EXPERIMENTAL or INVESTIGATIVE

The terms “Experimental” or “Investigative” mean services, supplies, care and treatment that do not constitute accepted medical practice properly within the range of appropriate medical practice under the standards of the case and by the standards of a reasonably substantial, qualified,

responsible, relevant segment of the medical community or government oversight agencies at the time services were rendered.

The Plan Administrator must make an independent evaluation of the Experimental/non-Experimental standings of specific technologies. The Plan Administrator shall be guided by a reasonable interpretation of Plan provisions. The decisions shall be made in good faith and rendered following a detailed factual background investigation of the claim and the proposed treatment. The decision of the Plan Administrator will be final and binding on the Plan. The Plan Administrator will be guided by the following principles:

- A. if the Drug or device cannot be lawfully marketed without approval of the U.S. Food and Drug Administration (FDA) and approval for marketing has not been given at the time the Drug or device is furnished;
- B. if the Drug, device, medical treatment or procedure, or the patient informed consent document utilized with the Drug, device, treatment or procedure, was reviewed and approved by the treating facility's Institutional Review Board or other body serving a similar function, or if federal law requires such review or approval. The Plan Administrator will determine what constitutes reliable evidence;
- C. if reliable evidence shows that the Drug, device, medical treatment or procedure is the subject of on-going phase I or phase II clinical trials, is the research, Experimental, study or investigational arm of on-going phase III clinical trials, or is otherwise under study to determine its maximum tolerated dose, its toxicity, its safety, its efficacy or its efficacy as compared with a standard means of treatment or diagnosis. This does not exclude coverage for Routine Patient Costs provided as part of an Approved Clinical Trial for the treatment of cancer or another Life Threatening Condition or disease for a Qualified Individual; or
- D. if reliable evidence shows that the prevailing opinion among experts regarding the Drug, device, medical treatment or procedure is that further studies or clinical trials are necessary to determine its maximum tolerated dose, its toxicity, its safety, its efficacy or its efficacy as compared with a standard means of treatment or diagnosis.

Drugs are also considered Experimental if they are not commercially available for purchase and/or they are not approved by the FDA for general use.

The Plan Administrator, in its sole discretion, shall determine whether services, supplies, care and treatment are Experimental or Investigative under the Plan.

FDA

The term "FDA" means the United States Food and Drug Administration, an agency of the United States Department of Health and Human Services that is charged with the responsibility for regulation and supervision of food safety, tobacco products, dietary supplements, prescription and over-the-counter pharmaceutical drugs (medications), vaccines, biopharmaceuticals, blood transfusions, medical devices, electromagnetic radiation emitting devices (ERED), cosmetics, animal foods & feed and veterinary products within the United States.

GENERIC DRUG

The term "Generic Drug" means a prescription Drug that has the equivalency of the brand name Drug with the same use and metabolic disintegration. This Plan will consider as a Generic Drug any Food and Drug Administration (FDA) approved generic pharmaceutical dispensed according to the professional standards of a licensed pharmacist and clearly designated by the pharmacist as being generic.

HOME HEALTH CARE AGENCY

The term "Home Health Care Agency" means an organization that meets all of the following criteria:

- A. its main function is to provide home health care services and supplies that are Covered Expenses under this Plan;

- B. it is approved for participation in Medicare as a Home Health Care Agency; and
- C. it is licensed as such by the state in which it is located, if licensing is required.

HOME HEALTH CARE PLAN

The term "Home Health Care Plan" means a plan of care that meets the following requirements:

- A. it is a formal written plan made by the patient's attending Physician which is reviewed at least every thirty (30) days;
- B. it must state the diagnosis;
- C. it must certify that the home health care is in place of Hospital confinement; and
- D. it must specify the type and extent of home health care required for the treatment of the patient.

HOSPICE

The term "Hospice" means an organization whose main function is to provide Hospice care services and supplies that are Covered Expenses under this Plan and which are licensed as a Hospice by the state in which it is located, if licensing is required.

HOSPICE CARE PLAN

The term "Hospice Care Plan" means a plan of terminal patient care that is established and conducted by a Hospice and supervised by a Physician.

HOSPICE UNIT

The term "Hospice Unit" means a facility or separate Hospital unit that provides treatment under a Hospice Care Plan and admits at least two (2) unrelated persons who are expected to die within six (6) months.

HOSPITAL

The term "Hospital" means an institution that is engaged primarily in providing Medical Care and treatment of sick and injured persons on an Inpatient basis at the patient's expense and that fully meets all the following criteria:

- A. it is accredited as a Hospital by the Joint Commission;
- B. it is approved for participation in Medicare as a Hospital;
- C. it maintains diagnostic and therapeutic facilities on the premises for surgical and medical diagnosis and treatment of sick and injured persons by or under the supervision of a staff of Physicians;
- D. it continuously provides on the premises twenty-four (24) hour-a-day nursing services by or under the supervision of registered nurses (R.N.s); and
- E. it is operated continuously with organized facilities for operative surgery on the premises.

The definition of "Hospital" shall also include the following:

- A. a facility operating legally as a psychiatric Hospital or residential treatment facility for mental health and licensed as such by the state in which the facility operates; and
- B. a facility operating primarily for the treatment of Alcoholism and/or Substance Abuse if it meets the following criteria:
 - 1. it maintains permanent and full-time facilities for bed care and full-time confinement of at least fifteen (15) resident patients;
 - 2. it has a Physician in regular attendance;
 - 3. it continuously provides twenty-four (24) hour a day nursing service by a registered nurse (R.N.);
 - 4. it has a full-time psychiatrist or psychologist on the staff; and
 - 5. it is primarily engaged in providing diagnostic and therapeutic services and facilities for treatment of Alcoholism and/or Substance Abuse.

INDEPENDENT FREESTANDING EMERGENCY DEPARTMENT

The term “Independent Freestanding Emergency Department” means a health care facility that is geographically separate and distinct, and licensed separately, from a Hospital under applicable state law, and which provides any Emergency Services.

INPATIENT

The term “Inpatient” refers to the classification of a Covered Person when that person is admitted to a Hospital, Hospice Unit, Skilled Nursing Facility or other covered facility for treatment and charges are made for Room and Board to the Covered Person as a result of such admission. After twenty-three (23) observation hours, a confinement will be considered an Inpatient confinement.

INTENSIVE CARE UNIT

The term “Intensive Care Unit” means a separate, clearly designated service area which is maintained within a Hospital solely for the care and treatment of patients who are critically ill. This also includes what is referred to as a “coronary care unit” or an “acute care unit.” It has:

- A. facilities for special nursing care not available in regular rooms and wards of the Hospital;
- B. special life saving equipment which is immediately available at all times;
- C. at least two (2) beds for the accommodation of the critically ill; and
- D. at least one (1) registered nurse (R.N.) in continuous and constant attendance twenty-four (24) hours a day.

LIFE THREATENING CONDITION

The term “Life Threatening Condition” means any disease or condition from which the likelihood of death is probable, unless the course of the disease or condition is interrupted.

MEDICAL CARE

The term “Medical Care” means professional services given by a Physician or other provider to treat an Injury, ailment, condition, disease, disorder or Illness, including medical advice, treatment, medical diagnosis and the taking of prescription Drugs.

MEDICAL CARE FACILITY

The term “Medical Care Facility” means a Hospital, a facility that treats one or more specific ailments or any type of Skilled Nursing Facility.

MEDICALLY NECESSARY or MEDICAL NECESSITY

The terms “Medically Necessary” or “Medical Necessity” mean a procedure, treatment, service, supply, equipment, drug, or medicine that is:

- A. deemed appropriate, essential, and is recommended or approved by a Physician, Dentist or other medical practitioner who is practicing within the scope of his or her license and specialty or primary area of practice;
- B. consistent with the patient’s condition and accepted standards of good medical practice;
- C. not considered Experimental or Investigative;
- D. medically proven to be effective treatment of the condition;
- E. not performed mainly for the convenience of the patient or provider of medical services;
- F. not conducted for research purposes; and
- G. within the scope, duration and intensity of that level of care which is required to provide safe, adequate and appropriate diagnosis or treatment.

The fact that a Physician has prescribed, ordered, recommended or approved a procedure, treatment, service, supply, equipment, drug or medicine does not, of itself, make such procedure, treatment, service, supply, equipment, drug or medicine Medically Necessary under the Plan, nor does it make the charge a Covered Expense. The Plan reserves the right to make the final determination of Medical Necessity on the basis of final diagnosis and supporting medical data.

This determination will be based on, and consistent with, standards approved by the Plan's medical review consultants.

MEDICARE

The term "Medicare" means the programs established by Title I of Public Law 89-98, as amended, entitled "Health Insurance for the Aged Act," and that includes parts A, B, C and D of Subchapter XVIII of the Social Security Act, as amended from time to time.

MENTAL/NERVOUS DISORDER

The term "Mental/Nervous Disorder" means any disease or condition, regardless of whether the cause is organic, that is classified as a mental disorder in the current edition of *International Classification of Diseases*, published by the World Health Organization or is listed in the current edition of *Diagnostic and Statistical Manual of Mental Disorders*, published by the American Psychiatric Association.

NEVER EVENTS

The term "Never Events" means errors or omissions in Medical Care that are clearly identifiable, preventable, and serious in their consequences for patients. Examples of Never Events include, but are not limited to:

- A. surgery on the wrong body part;
- B. a foreign body left in a patient after surgery;
- C. a mismatched blood transfusion;
- D. a major medication error;
- E. a severe "pressure ulcer" acquired in the Hospital;
- F. falls or traumas experienced by a patient while confined in a healthcare facility; and
- G. preventable post-operative deaths.

NO SURPRISES ACT

The term "No Surprises Act" means the law which is part of the Consolidated Appropriations Act of 2021 (H.R. 133; Division BB – Private Health Insurance and Public Health Provisions) and its promulgating regulations.

OUT-OF-POCKET

The term "Out-of-Pocket" means the amount of Covered Expenses that are the responsibility of the Covered Person and that accumulate towards the Plan's Out-of-Pocket maximum, not including amounts for:

- A. expenses that are not covered under this Plan;
- B. in excess of the Maximum Allowable Charge for a service or supply;
- C. in excess of any maximum benefit listed in the Plan; or
- D. attributable to any penalty.

OUTPATIENT

The term "Outpatient" refers to the classification of a Covered Person when that Covered Person receives Medical Care, treatment, services or supplies at a clinic, a Physician's office, or at a Hospital, if not a registered bed patient at that Hospital or other covered facility.

PAIN INJECTION

The term "Pain Injection" means the administration of a Drug, including, but not limited to steroids, narcotics or opiates, for the purpose of the reduction or alleviation of pain by injection, rather than orally or through other means. The cost of a Pain Injection includes the charges for the Drug, in addition to the cost of administration.

PARTICIPATING HEALTH CARE FACILITY

The term “Participating Health Care Facility” shall mean a Hospital or Hospital Outpatient department, critical access Hospital, ambulatory surgical center, or other Provider as required by law, which has a direct contract or case agreement between such health care facility and the Plan with respect to the furnishing of a healthcare item or service. Such contractual relationship for purposes of this definition shall be with respect to the parties to the agreement and/or particular individual(s) involved.

PHARMACY

The term “Pharmacy” means a licensed establishment where covered Drugs are filled and dispensed by a pharmacist licensed under the laws of the state where he or she practices.

PREGNANCY

The term “Pregnancy” means that physical state that results in childbirth, abortion or miscarriage, and any medical complications arising out of, or resulting from, such state.

PROSTHETIC DEVICE or PROSTHESIS

The terms “Prosthetic Device” or “Prosthesis” mean an artificial device that replaces a missing body part or supplements a defective body part.

PROTECTED SERVICES

The term “Protected Services” means those services identified under the No Surprises Act for the purpose of the prevention of surprise billing by non-Preferred Providers. These include the following services when provided by a non-Preferred Provider:

- A. Emergency Services in an Emergency;
- B. the following non-Emergency services whether such services are provided by a Physician or non-Physician practitioner at a Participating Health Care Facility, is applicable:
 - 1. diagnostic services, including radiology and laboratory services;
 - 2. professional services for pathology, radiology, anesthesiology, and neonatology;
 - 3. the services of an emergency room Physician, Hospitalist, assistant surgeon or intensivist; or
 - 4. items or services furnished as a result of unforeseen, urgent medical needs that arise at the time such item or service is furnished; and
- C. non-Preferred air ambulance services (but only if air ambulance service is a Covered Expense under this Plan).

QUALIFIED INDIVIDUAL

The term “Qualified Individual” means an individual who is properly enrolled in the Plan and who is eligible to participate in an Approved Clinical Trial according to the trial protocol with respect to treatment of cancer or another Life Threatening Condition or disease. To be a Qualified Individual, there is an additional requirement that a determination be made that the individual’s participation in the Approved Clinical Trial is appropriate to treat the disease or condition. That determination can be made based on the referring health care professional’s conclusion or based on the provision of medical and scientific information by the individual.

QUALIFYING PAYMENT AMOUNT

The term “Qualifying Payment Amount” means the median of the contracted rates recognized by the Plan or recognized by all plans serviced by the Plan’s Third Party Administrator (if calculated by the Third Party Administrator), for the same or a similar item or service provided by a Provider in the same or similar specialty in the same geographic region. If there are insufficient (meaning at least three) contracted rates available to determine a Qualifying Payment Amount, said amount will be determined by referencing a state all-payer claims database or, if unavailable, any eligible third-party database in accordance with applicable law.

RECOGNIZED AMOUNT

The term “Recognized Amount” shall mean an amount determined under an applicable all-payer model agreement, or if unavailable, an amount determined by applicable state law. If no such amounts are available, the Recognized Amount shall mean the lesser of a Provider’s billed charge or the Qualifying Payment Amount.

RECOMMENDED WELLNESS SERVICE

The term “Recommended Wellness Service” means a service or supply that is not intended to treat an existing medical condition, but rather is intended to detect or prevent potential future problems or assist the Covered Person in maintaining his or her health. They are recommended by recognized medical bodies, and are required to be covered without cost sharing by non-grandfathered health plans under the Affordable Care Act. These recommendations include the following:

- A. evidence-based preventive services with an A or B recommendation from the U.S. Preventive Services Task Force, including the 2002 recommendation regarding breast cancer screening (www.uspreventiveservicestaskforce.org);
- B. immunizations recommended by the Advisory Committee on Immunization Practices, as updated annually (www.cdc.gov/vaccines); and
- C. guidelines supported by the Health Resources and Services Administration that are applicable to children and women, including:
 - 1. services provided to children under the Bright Futures recommendations of the American Academy of Pediatrics (brightfutures.aap.org) and the Secretary’s Advisory Committee on Heritable Disorders in Newborns and Children (SACHDNC) national recommendations on Newborn screening (www.hrsa.gov/advisorycommittees/mchbadvisory/heritabledisorders/recommendedpanel/index.html); and
 - 2. women’s health services recommendations developed by the Institute of Medicine (www.hrsa.gov/womens-guidelines).

Any changes to the above recommendations will take effect for this Plan at the beginning of the first Plan Year beginning one (1) year after the issuance of such new recommendation or a change in the existing recommendations by the above entities, unless the change was prompted by safety or other concerns that make it inadvisable to continue to cover the service or supply.

ROOM AND BOARD

The term “Room and Board” refers to all charges, by whatever name called, that are made by a Hospital, Hospice Unit or Skilled Nursing Facility as a condition of occupancy. Such charges do not include the professional services of Physicians or intensive nursing care by whatever name called.

ROUTINE PATIENT COSTS

The term “Routine Patient Costs” means all items and services consistent with the coverage provided under the Plan that is typically covered for a Qualified Individual for treatment of cancer or another Life Threatening Condition or disease who is not enrolled in a clinical trial. However, costs associated with the following are excluded from that definition, and the Plan is not required under federal law to pay for the following:

- A. the cost of the investigational item, device or service;
- B. the cost of items and services provided solely to satisfy data collection and analysis needs and that are not used in direct clinical management; or
- C. the cost for a service that is clearly inconsistent with widely accepted and established standards of care for a particular diagnosis.

SEMI-PRIVATE

The term “Semi-Private” refers to a class of accommodations in a Hospital or other covered facility in which at least two (2) patient beds are available per room.

SERIOUS AND COMPLEX CONDITION

The term “Serious and Complex Condition” means with respect to a Covered Person:

- A. in the case of an acute illness, a condition that is serious enough to require specialized medical treatment to avoid the reasonable possibility of death or permanent harm; or
- B. in the case of a chronic illness or condition, a condition that is:
 - 1. life threatening, degenerative, potentially disabling, or congenital; and
 - 2. requires specialized medical care over a prolonged period of time.

SKILLED NURSING FACILITY

The term “Skilled Nursing Facility” means a facility that fully meets all of the following criteria:

- A. it is licensed to provide professional nursing services on an Inpatient basis to persons convalescing from Injury or Illness in the jurisdiction in which it is located. The service must be rendered by a registered nurse (R.N.) or by a licensed practical nurse (L.P.N.) under the direction of a registered nurse. Services to help restore patients to self-care in essential daily living activities must be provided;
- B. its services are provided for compensation and under the full-time supervision of a Physician;
- C. it provides twenty-four (24) hour per day nursing services by licensed nurses, under the direction of a full-time registered nurse;
- D. it maintains a complete medical record on each patient;
- E. it has an effective utilization review plan;
- F. it is not, other than incidentally, a place for rest, the aged, Drug addicts, alcoholics, mental retardates, Custodial Care, educational care or care of Mental/Nervous Disorders, Alcoholism or Substance Abuse; and
- G. it is approved for participation as such a facility in the Medicare program.

This term also applies to charges Incurred in a facility referring to itself as an extended care facility, convalescent nursing home, rehabilitation hospital or any other similar nomenclature.

SPECIALIST

The term “Specialist” means a Physician who primarily practices in any medical specialty, such as neurology, cardiology, or pulmonary, and who is not a Primary Care Physician.

SPECIALTY DRUG

The term “Specialty Drug” means a scientifically or “bioengineered” oral or injectable medicine that targets and treats a specific or “niche” condition, and includes one (1) or more of the following features:

- A. it is usually a complex compound;
- B. it is offered by the manufacturer at a premium price that is generally significantly higher than those for traditional medications;
- C. it is primarily prescribed and administered by a Specialist, such as an oncologist or pulmonologist;
- D. it often requires special or unique storage and handling; and
- E. it requires disease management services, such as patient education and monitoring.

The Plan Administrator, in its discretion and in consultation with pharmaceutical experts, will determine if a drug is considered a Specialty Drug under this Plan.

SPINAL MANIPULATION

The term “Spinal Manipulation” means skeletal adjustments, manipulation or other treatment in connection with the detection and correction by manual or mechanical means of structural imbalance or subluxation in the human body. Such treatment is done by a Physician or Professional Provider to remove nerve interference resulting from, or related to, distortion, misalignment or subluxation of, or in, the vertebral column.

SUBSTANCE ABUSE

The term “Substance Abuse” means the physical habitual dependence on Drugs (other than alcohol). This does not include dependence on tobacco and ordinary caffeine-containing drinks.

In making the determination as to whether the Covered Person’s condition meets the definition of Substance Abuse under this Plan, the Plan Administrator shall use recognized authorities, including designations contained in the most current editions of the *International Classification of Diseases* (ICD) of the World Health Organization and the *Diagnostic and Statistical Manual of Mental Diseases* (DSM) published by the American Psychiatric Association.

3.3 DENTAL PLAN DEFINITIONS**COINSURANCE**

The term “Coinsurance” means the amount of the dental Covered Expenses, up to any applicable maximum, that are payable by either the Plan or the Covered Person. Under Class II, III and IV, such Coinsurance is payable after the Calendar Year Deductible has been satisfied.

CUSTOMARY

The term “Customary” refers to the designation of a charge as being the usual charge made by a Dentist or other provider of services and supplies, medication or equipment that does not exceed the general level of charges made by other providers rendering or furnishing such care or treatment within the same general geographic area, taking into consideration differences in demographics of specific locations and using generally accepted standards of medical practice. The term “area” in this definition means a county or such other area as is necessary to obtain a representative cross section of such charges. Due consideration will be given to the nature and severity of the condition being treated and any medical complications or unusual circumstances that require additional time, skill or expertise. In regards to services or supplies provided by Preferred Providers, this term refers to the contracted rate for the service or supply in question, as determined by the agreement between the Plan and the network to which the provider belongs.

DEDUCTIBLE

The term “Deductible” means the amount of Class II, III and IV dental Covered Expenses Incurred by a each eligible Covered Person in a Calendar Year before any other dental Covered Expenses can be considered for payment at the percentages stated in the Schedule of Benefits and this Plan.

DENTALLY NECESSARY or DENTAL NECESSITY

The terms “Dentally Necessary” or “Dental Necessity” mean health care services and supplies that are determined by the Plan Administrator to be dentally appropriate, and: determined by the Plan Administrator to be dentally appropriate, and:

- A. necessary to meet the basic dental needs of the Covered Person;
- B. rendered in the most cost-efficient manner and type of setting appropriate for the delivery of the service or supply;
- C. consistent in type, frequency and duration of treatment with scientifically based guidelines of national dental, research or health care coverage organizations or governmental agencies that are accepted by the Plan Administrator;
- D. consistent with the diagnosis of the condition;
- E. required for reasons other than the convenience of the Covered Person or his or her Dentist; and

- F. demonstrated through prevailing peer-reviewed dental literature to be safe and effective for treating or diagnosing the dental condition or Illness for which their use is proposed.

DENTIST

The term “Dentist” means any dental practitioner who is duly licensed and qualified under the law of the jurisdiction in which treatment is received to render dental services, perform dental surgery or administer anesthetics for dental surgery.

LIFETIME

The term “Lifetime” is a word used in the Plan in reference to benefit maximums and limitations. The term “Lifetime” means the total time period of a Covered Person’s coverage under this Plan, regardless of the number of breaks in that coverage. Under no circumstances does the term “Lifetime” mean the duration of a Covered Person’s life.

REASONABLE

The term “Reasonable” refers to the designation of a charge as being appropriate based on the services or supplies actually supplied by a provider to a Covered Person. While the charge made for such service may be considered to be Customary within the general context of billing practices for similar services, the true circumstances of the case may warrant a lesser or higher charge than the Customary charge for the services and/or supplies that were, in fact, provided to the Covered Person. The Plan Administrator shall have the right to review provider’s records relative to the service or supply, and shall determine, in its absolute discretion, whether or not the charge made by the provider for the service or supply is Reasonable. In making this determination, the Plan Administrator will take into consideration additional charges that were attributable to the errors, negligence or inefficiency of the provider, and may consult with medical experts in the related fields to determine whether such charges would be considered Reasonable within the context in which they were provided.

3.4 COMMON DENTAL TERMS

ABUTMENT

A tooth or root that retains or supports a fixed Bridge or a removable prosthesis.

ACID ETCH

The etching of a tooth with a mild acid to aid in the retention of composite filling material.

ACRYLIC

Plastic material used in the fabrication of Dentures and Crowns and occasionally as a restorative filling material.

AMALGAM

A metal alloy usually consisting of silver, tin, zinc and copper combined with liquid pure mercury and used as restorative material in operative dentistry.

ANESTHESIA

Local - The condition produced by the administration of specific agents to achieve the loss of pain sensation in a specific location or area of the body.

General - The condition produced by the administration of specific agents to render the patient completely unconscious and without pain sensation.

ANTERIOR TEETH

The central incisors, lateral incisors and cuspids.

APPLIANCE

A device used to provide function, therapeutic (healing) effect, space maintenance, or as an application of force to teeth to provide movement or growth changes as in orthodontics.

Fixed - One that is attached to the teeth by cement or by adhesive materials and cannot be removed by the patient.

Removable - One that can be taken in and out of the mouth by the patient.

Prosthetic - Used to provide replacement for a missing tooth.

BITEWING

A type of dental x-ray film that has a central tab or wing upon which the teeth close to hold the film in position. They are commonly called detecting x-rays because they show decay better than other x-rays.

BRIDGEWORK, BRIDGE OR PROSTHETIC APPLIANCE

Fixed - Pontics or replacement teeth retained with Crowns or Inlays cemented to the natural teeth, which are used as Abutments.

Fixed, Removable - One that the dentist can remove but the patient cannot.

Removable - A Partial Denture retained by attachments that permit removal of the Denture. Normally held by clasps.

CARIES

A disease of progressive destruction of the teeth from bacterially produced acids on tooth surfaces.

COMPOSITE

Tooth colored filling material primarily used in the Anterior Teeth.

CROWN

A natural Crown is the portion of a tooth covered by enamel. An artificial Crown (cap) restores the anatomy, function and esthetics of the natural Crown.

DENTAL HYGIENIST

A person who has been trained to clean teeth, and provide additional services and information on the prevention of oral disease.

DENTURE

A device replacing missing teeth. The term usually refers to full or partial Dentures but it actually means any substitute for missing natural teeth.

ENDODONTIC THERAPY

Treatment of diseases of the dental pulp and their sequelae.

FLUORIDE

A solution of Fluoride that is applied topically to the teeth for the purpose of preventing dental decay.

IMPLANT

A device surgically inserted into or onto the jawbone. It may support a Crown or Crowns, partial Denture, complete Denture or may be used as an Abutment for a fixed Bridge.

IMPRESSION

A negative reproduction of a given area. It is made in order to produce a positive form or cast of the recorded teeth and/or soft tissues of the mouth.

INLAY

A restoration usually of cast metal made to fit a prepared tooth cavity and then cemented into place.

MALOCCLUSION

An abnormal contact and/or position of the opposing teeth when brought together.

OCCLUSION

The contact relationship of the upper and lower teeth when they are brought together.

ONLAY

A cast Restoration that covers the entire chewing surface of the tooth.

PALLIATIVE

An alleviating measure. To relieve, but not cure.

PARTIAL DENTURE

A prosthesis replacing one (1) or more, but less than all, of the natural teeth and associated structures; may be removable or fixed, one side or two sides.

PEDODONTICS

The specialty of children's dentistry.

PERIODONTICS

The science of examination, diagnosis, and treatment of diseases affecting the supporting structures of the teeth.

PONTIC

The part of a fixed Bridge that is suspended between the Abutments and that replaces a missing tooth or teeth.

POSTERIOR TEETH

The bicuspid and molars.

PROPHYLAXIS

The removal of tarter and stains from the teeth. The cleaning of the teeth by a Dentist or Dental Hygienist.

REBASE

A process of refitting a Denture by the replacement of the entire Denture-base material without changing the occlusal relations of the teeth.

RELINE

To resurface the tissue-borne areas of a Denture with new material.

RESTORATION

A broad term applied to any Inlay, Crown, Bridge, Partial Dentures, or complete Denture that restores or replaces loss of tooth structure, teeth or oral tissue. The term applies to the end result of repairing and restoring or reforming the shape, form and function of part or all of a tooth or teeth.

ROOT CANAL THERAPY

The complete removal of the pulp tissues of a tooth, sterilization of the pulp chamber and root canals, and filling these spaces with a sealing material.

SCALING

The removal of calculus (tarter) and stains from teeth with special instruments.

SEALANT

A resinous agent applied to the grooves and pits of teeth to reduce decay.

SILICATE

A relatively hard and translucent restorative material that is used primarily in the Anterior Teeth.

SPLINTING

Stabilizing or immobilizing teeth to gain strength and/or facilitate healing.

TOPICAL APPLICATION

Painting the surface of teeth, as in Fluoride treatment or application of an anesthetic formula to the surface of the gum.

VERTICAL DIMENSION

The degree of jaw separation when the teeth are in contact.

3.5 SHORT TERM DISABILITY DEFINITIONS

COVERED WEEKLY EARNINGS

The term “Covered Weekly Earnings” means the Participant’s rate of weekly earnings from the Employer on the date the Participant becomes Totally Disabled for the period of disability in question. Covered Weekly Earnings does not include any amount payable for any of the following:

- A. overtime pay;
- B. commissions; or
- C. bonuses.

TOTAL DISABILITY or TOTALLY DISABLED

The terms “Total Disability” or “Totally Disabled” mean the Participant’s complete inability, as a result of Illness or Injury, to perform any and every duty of his or her occupation, or of a similar occupation, for which the Participant is reasonably capable due to education and training.

ARTICLE IV

CLAIM AND APPEAL PROCEDURES

4.1 INITIAL FILING OF CLAIMS

A Clean Claim for benefits should be filed within ninety (90) days after the occurrence or commencement of any loss covered by this Plan. Failure to give such notice and proof within the time required will neither invalidate nor reduce any claim if it is shown that written notice and proof are given no later than one (1) year after the claim is Incurred, unless the Covered Person is legally incapacitated.

Upon termination of the Plan, final claims must be received within ninety (90) days of termination. In any of the events described above, notice and proof of claim will be determined at the discretion of the Plan Administrator, subject to the requirements listed below.

Claims should be submitted to the appropriate address listed on the Covered Person's identification card, and can be submitted either by the provider or the Covered Person. Such claim should be on any of the following appropriate forms (or their successor forms):

- A. CMS 1500;
- B. UB-92;
- C. UB-04 or CMS 1450;
- D. NCPDP Form 1983; or
- E. J512 claim forms.

A Clean Claim can be submitted by the provider in electronic format if the provider submits it in accordance with the electronic transaction requirements of the Health Insurance Portability and Accountability Act of 1996 (HIPAA) and subsequent laws.

In order to be considered a Clean Claim, such claim must use the most current CPT code in effect as published by the American Medical Association, the *International Statistical Classification of Diseases and Related Health Problems* ("ICD") codes, including ICD-9 and ICD-10, published by the World Health Organization, the most current dental code in effect as published by the American Dental Association in the *Code for Dental Procedures or Nomenclature* or the most current HCPCS code in effect, as published by U.S. Department of Health and Human Services, Centers for Medicare and Medicaid Services.

If the Plan is not the primary carrier for a Covered Person who has, or had at the time the claim was Incurred, more than one health plan that would provide benefits for the services or supplies for which the claim is being made, including, but not limited to Medicare, copies of the explanations of benefit payment from all carriers who would pay benefits before the Plan should be submitted with the claim. For more information regarding which plan pays first, see Section 12.1, or contact the Benefit Manager.

4.2 REQUESTS FOR ADDITIONAL INFORMATION

If the claim is not submitted in accordance with the procedures listed in Section 4.1, the claim will not be considered to be a Clean Claim, and the Participant or Covered Person will be notified of the claim deficiencies, and requested to refile it in the proper format.

If the Plan Administrator or the Benefit Manager needs more information to process the claim, a letter will be sent to the Participant, the Covered Person, the provider or other parties requesting additional information. In some situations, information is needed on a periodic basis, including:

- A. information regarding other coverage. This may include providing copies of medical child support orders for children of divorced parents; and
- B. verification of handicapped status for over age Dependent children.

Other information may be requested on a case-by-case basis, including information pertaining to accident details or potential third-party liability.

The requested information must be provided within forty-five (45) days of the date the Participant or Covered Person receives notice of the required additional information. If the information is not received within this time period, the claim will be denied for failure to provide the needed information.

4.3 APPEALS OF ADVERSE BENEFIT DETERMINATIONS

The Covered Person can appeal a decision by the Plan that coverage for a service or supply is denied or reduced under the Plan, or any other eligibility issue, including a rescission of coverage for an individual, provided such appeal is made in writing within one hundred eighty (180) days of the Covered Person or Participant's receipt of the explanation of benefit payment, the precertification letter reflecting the denial or reduction or any other notification made by the Plan of an adverse decision involving the individual. Any individual other than the Covered Person who wishes to submit an appeal on the Covered Person's behalf (other than a parent or Legal Guardian filing an appeal for a minor child) must be designated by the Covered Person, in a writing signed by the Covered Person, as his or her authorized representative specifically for the purpose of the appeal. An assignment of benefits is not sufficient to designate another person as an "authorized representative" for the purpose of an appeal. These appeal procedures shall not apply to any contractual dispute between a provider and the Plan as to amounts due the provider, rather than the Covered Person, under the terms of any agreement between the provider and the Plan that does not affect the amount payable by the Covered Person.

A request for review in which the Covered Person is requesting an expedited appeal of a pre-service claim as an "urgent care" case, as described in Section 6.1, can be submitted either orally or in writing and can be submitted by a provider with knowledge of the Covered Person's condition without prior designation by the Covered Person. If a course of treatment has been previously approved by the Plan to be provided over a period of time or for a number of treatments, no reduction or termination of coverage for such treatment (other than termination of the individual's coverage under this Plan) will be made without allowing the Covered Person sufficient advance notification and the opportunity to appeal this termination or reduction.

The appeal request should be addressed as follows (unless the Adverse Benefit Determination notification indicates otherwise):

Plan Administrator
Highland Computer Forms, Inc. Employee Benefit Plan
c/o Benefit Manager
Medical Benefits Administrators, Inc.
P.O. Box 1099
Newark, Ohio 43058-1099

The writing should clearly be identified as an appeal, and include the name of the Plan, the Covered Person whose claims are the subject of the appeal, the Participant's identification number, and the identity of the specific treatment, service or supply for which coverage was denied or limited under the Plan.

The Covered Person should submit with the appeal written comments, documents, records and other information relating to the claim for benefits, even if such information was not submitted as part of the initial claim or request for preauthorization or precertification. The Covered Person will also have the right to present testimony as part of the appeal.

The Covered Person has the right to request information from the Plan Administrator as part of the appeals process, as described in Section 4.4.

Appeals submitted under this Plan will be adjudicated in a manner designed to ensure the independence and impartiality of the person making the decision. The Plan Administrator has the sole authority for the final decision on all Plan matters, including appeals.

4.4 ACCESS TO DOCUMENTS, RECORDS OR OTHER INFORMATION

A Covered Person is entitled to examine the claim file and present testimony as part of the internal claims and review process. He or she will also receive, free of charge, copies of documents, records and other information relevant to his or her claim for benefits, including any new or additional information generated by the Plan Administrator that is received during the appeals process, and the rationale behind the Plan's adverse decision. Such information will be provided within sufficient time to respond prior to the final decision of the appeal by the Plan Administrator. Such information is considered to be relevant if it:

- A. was relied upon by the Plan Administrator in making the benefit determination;
- B. was submitted, considered or generated in the course of making the benefit determination;
- C. demonstrates compliance with the administrative processes required by applicable law;
- D. constitutes a statement of policy or guidance with respect to the Plan concerning the denial of a treatment option or benefit; or
- E. involves the identity of medical or vocational experts whose advice was obtained in connection with the claim.

In addition, if an Adverse Benefit Determination is based upon the Medical Necessity or Experimental nature of the service or supply, the Covered Person can request an explanation of the scientific or clinical judgment of the determination, free of charge.

4.5 EXTERNAL REVIEW RIGHTS AND PROCEDURES

If the Covered Person is not satisfied with the Plan Administrator's decision on his or her appeal of a medical issue, he or she may file a request for an external review with the Plan Administrator at the address listed above for submitting an appeal. The Federal external review process, in accordance with the current Affordable Care Act regulations and other applicable law, applies only to the following:

- A. any eligible Adverse Benefit Determination (including a Final Internal Adverse Benefit Determination) by a plan or issuer that involves medical judgment (including, but not limited to, those based on the plan's or issuer's requirements for Medical Necessity, appropriateness, health care setting, level of care, or effectiveness of a covered benefit; its determination that a treatment is Experimental or Investigational; its determination as to whether a Covered Person is entitled to a reasonable alternative standard for a reward under a wellness program; its determination as to whether the Plan is complying with the nonquantitative treatment limitation provisions of Code section 9812 and § 54.9812-1, which generally require, among other things, parity in the application of medical management techniques), as determined by the external reviewer;
- B. an Adverse Benefit Determination that involves consideration of whether the Plan is complying with the surprise billing and cost-sharing protections set forth in the No Surprises Act; and
- C. a rescission of coverage (whether or not the rescission has any effect on any particular benefit at that time).

The request must be filed within four (4) months after the date of receipt of the Plan Administrator's determination on his or her appeal. If there is no corresponding date four (4) months after the date of receipt of notice, then the request must be filed by the first (1st) day of the fifth (5th) month following the receipt of the Plan's determination on his or her appeal. The Covered Person can make a request for an expedited review of a pre-certification denial if the timeframe for completion of a standard review would seriously jeopardize the life or health of the Covered Person or would jeopardize his or her ability to regain maximum function, or if the determination concerns an admission, availability of care, continued stay or health care item or service for which the Covered Person received Emergency services, but has not been discharged from a facility. A standard external review would generally be completed within fifty (50) days of

the Plan's receipt of the request, while an expedited review must be completed by the independent review organization (IRO) within seventy-two (72) hours of the IRO's receipt of such request. The Plan Administrator will review the request and determine whether or not the request meets the criteria for external review or an expedited review, including whether or not the person was a Covered Person under the Plan at the time the claim arose, whether the person has exhausted the Plan's appeal process, and whether the sufficient information has been submitted to process the external review. A notification will be issued by the Plan Administrator regarding the Covered Person's incomplete request for an external review. If the request is incomplete, the Covered Person will be given additional time to complete the external review request. Once a determination has been made by the Plan Administrator that the request qualifies for external review, it will be forwarded by the Plan Administrator to a qualified IRO. The IRO will notify the Covered Person if the request is accepted for review, and, if a standard review, that he or she can submit additional information that is relevant to the request within ten (10) days of the notification. The IRO may also request additional information from the Covered Person and/or the Plan. Additional information provided by the Covered Person will be provided to the Plan Administrator. If, based on this additional information, the Plan Administrator determines that the initial determination should be reversed, and that coverage should be provided under the Plan, all parties will be notified, and the external review will be closed. Otherwise, after the IRO has completed the review, the Covered Person and the Plan Administrator will be notified of the IRO's determination. If the IRO determines that coverage under the Plan should have been provided, the Plan will promptly pay any additional benefits deemed due on the Covered Person's behalf. However, either the Plan or the Covered Person has the right to appeal the decision or utilize any other remedy available under any applicable state or federal law, if either disagrees with the decision of the IRO.

4.6 ADDITIONAL APPEAL RIGHTS

If, after the Covered Person has exhausted all appeal and review rights listed above, he or she is still not satisfied with the disposition of the claim, such Covered Person has the right to bring an action under section 502(a) of the Employee Retirement Income Security Act (ERISA).

No action at law or in equity shall be brought to recover benefits under the Plan prior to the exhaustion of all claims and appeals procedures described in this Article, nor shall such action be brought at all unless brought within three (3) years from the expiration of the time within which proof is required by the Plan.

4.7 EXAMINATION

The Plan Administrator shall have the right and opportunity to have the Covered Person examined whose Injury or Illness is the basis of a claim hereunder when and as often as it may reasonably require during the pending claim. The Plan Administrator shall also have the right and opportunity to have an autopsy performed in case of death, where it is not forbidden by law.

4.8 PLAN ADMINISTRATOR DISCRETION

Nothing in this Plan precludes the Plan Administrator from exercising full discretionary authority and responsibility with respect to all aspects of Plan administration and interpretation. The Plan Administrator shall have all powers necessary to carry out the purposes of the Plan, including supplying any omissions in accordance with the intent of the Plan and deciding all questions concerning eligibility for participation in the Plan and concerning the amount of benefits payable to a Covered Person.

ARTICLE V

COVERAGE AND ELIGIBILITY

5.1 COVERAGE UNDER THIS PLAN

Coverage provided under the Plan for a Participant shall be in accordance with the Participant Eligibility, Participant Effective Date and Participant Termination provisions included herein.

In addition to the short term disability coverage that is available to Participants only, this Plan includes a medical option and a dental option. At the time of enrollment, a Participant must select which options, if any, in which such Participant and his or her Dependents should be enrolled. All Family members must be enrolled in the same options. A Participant can only change his or her plan options or enroll in coverage that was previously waived during this Plan's open enrollment period, as described in Section 5.8, unless he or she qualifies for a special enrollment, as described in Section 5.7.

5.2 PARTICIPANT ELIGIBILITY

Only individuals who meet all of the conditions of one (1) of the following categories shall be deemed eligible for coverage as a Participant under the Plan:

- A. an employee who is expected by the Employer, as of the date of his or her hire, to be a Full-Time Employee who has satisfied a thirty (30) day waiting period, commencing with his or her date of hire. This waiting period may be waived, in whole or in part, under the following circumstances:
 - 1. if the employee is no longer eligible for coverage because he or she failed to be actively employed for at least one hundred thirty (130) hours in any month prior to being employed for a full standard Measurement Period, but is employed in a subsequent month for at least this number of hours, coverage will be reinstated at the beginning of the month in which the employee once again works one hundred thirty (130) hours, as long as a period of thirteen (13) weeks or longer has not elapsed when the employee was credited with no hours of employment;
 - 2. if an employee is employed by the Employer for any or all of this thirty (30) day waiting period prior to his or her entry into Service in the Uniformed Services, this period of previous employment shall be credited towards the partial or full satisfaction of any waiting period imposed under this Plan if the employee is re-employed by the Employer at the expiration of the term of Service in the Uniformed Services, provided such employee applies for reemployment within the applicable time frame listed in the Uniformed Services Employment and Reemployment Rights Act of 1994 (USERRA), as described in Section 5.15; or
 - 3. if the Covered Person qualifies for reinstatement as described in Section 5.13;
- B. if not eligible for coverage under A, a Variable Hour Employee or Ongoing Employee of the Employer who has been determined during the most recent Measurement Period that is applicable to such employee to work an average of at least thirty (30) hours per week during such Measurement Period. Coverage for such employee will become effective (or be continued) as of the first day of the next Stability Period that applies to such employee, as long as such employee is still employed on that date.

A new Variable Hour Employee, or an On-Going Employee who was not previously eligible who has a Change in Employment Status during a Measurement Period will be treated as a Full-Time Employee as of the earlier of:

- 1. the first (1st) day of the month following the Change in Employment Status; or
- 2. the first day of the next Stability Period applicable to such person (provided the employee averages more than thirty (30) hours of service per week during the prior applicable Measurement Period).

For purposes of this Plan, Change in Employment Status means a material change in the employee's position of employment or other employment status that, had the employee begun employment in the new position or status, the employee would have reasonably been expected to work thirty (30) or more hours of service per week; or

C. is a Board Member of the Employer.

When determining eligibility for coverage under this Plan, any hours for which the Participant is off from work, but utilizing any paid time off made available by the employer including, but not limited to, vacation time and sick time, as applicable, will be considered hours worked by such Participant. Participants must agree to any applicable Participant Contribution for such coverage.

5.3 DEPENDENT COVERAGES

A Participant eligible to elect Dependent Coverage shall be any Participant whose Dependents meet the definition of a Dependent, set forth in Article III of the Plan. A Participant must make written request for Dependent Coverage and agree to any applicable Participant Contribution for such coverage. Each Participant will become eligible to elect Dependent Coverage on the latest of the following:

- A. the date he or she becomes eligible for Participant coverage; or
- B. the date on which he or she first acquires a Dependent.

If both spouses are employed by the Company, and both are eligible to elect Dependent coverage, either spouse, but not both, may elect Dependent Coverage for the eligible Dependents. In addition, no person can be covered under this Plan as the Dependent of more than one (1) Participant.

5.4 PARTICIPANT EFFECTIVE DATE

Each eligible employee who makes written request for Participant coverage hereunder, on a form approved by the Plan Administrator, subject to the provisions of this section, and who agrees to the applicable Participant Contribution for such coverage, shall become effective on the first of the month following the date he or she becomes eligible, provided the written application for such coverage is made within thirty-one (31) days of the date he or she becomes eligible for Participant Coverage.

Any eligible person who wishes to make an application for Participant coverage other than as described above, or as described in Section 5.7, shall be required to wait until the next Plan open enrollment period, as described in Section 5.8, before such application can be submitted.

5.5 DEPENDENT EFFECTIVE DATE

Each Participant who makes written request for Dependent Coverage hereunder within the thirty-one (31) day period immediately following the first day on which he or she is eligible for Dependent Coverage or when a special enrollment, as described in Section 5.7, applies to such Dependent, on a form approved by the Plan Administrator, subject to the provisions of this section, and who agrees to the applicable Participant Contribution for such coverage, shall become eligible for Dependent Coverage on the later of the date specified in the special enrollment period or the date the Participant becomes covered, as applicable.

Any Participant who wishes to make an application for Dependent Coverage other than as described above, or as described in Section 5.7, shall be required to wait until the next Plan open enrollment period, as described in Section 5.8, before such application can be submitted.

5.6 NEWBORN CHILDREN

If the Participant already has Family Dependent Coverage in effect as of the date of birth, the Participant's Newborn will be automatically covered. If the Participant does not have Family Dependent Coverage in effect as of the date of birth, application must be made for the Newborn within thirty-one (31) days after the birth. In either case, coverage will be effective on the date of birth. Any Participant who wishes to make an application for Dependent Coverage other than as

described above, or as described in Section 5.7, shall be required to wait until the next Plan open enrollment period, as described in Section 5.8, before such application can be submitted.

5.7 SPECIAL ENROLLMENT PERIODS

An eligible person for whom written application for coverage is submitted under any of the circumstances listed below will be eligible for coverage on the date specified below:

- A. within thirty-one (31) days of the date of a Dependent child's birth. The eligible employee, the Newborn and the Dependent spouse are eligible to enroll during this special enrollment period. Coverage shall become effective on the date of the Dependent child's birth;
- B. within thirty-one (31) days after the adoption of a Dependent child, or the Placement for Adoption with the employee of such a child. The eligible employee, the newly acquired Dependent child and the Dependent spouse are eligible to enroll during this special enrollment period. Coverage shall become effective on the date of the adoption or Placement for Adoption;
- C. within thirty-one (31) days of the date of the eligible employee's marriage. The eligible employee, the new Dependent spouse, and any other eligible Dependent children who are eligible as a result of the marriage are eligible to enroll during this special enrollment period. Coverage shall become effective on the date of the marriage;
- D. within thirty-one (31) days of the entry of an order requiring the employee to provide medical coverage for a Dependent child. The eligible employee and the Dependent child or children who are the subject of the court order are eligible to enroll during this special enrollment period. Coverage shall become effective on the date of the court order;
- E. within thirty-one (31) days of the date the employee experiences a "change in family status" under the Employer's Section 125/cafeteria plan. Only the family members who are affected by the "change in family status" are eligible to enroll during this special enrollment period. Coverage shall become effective on the date the change becomes effective under the Section 125/cafeteria plan;
- F. within sixty (60) days of the date an eligible employee and/or his or her Dependent(s) first become eligible for coverage under a state Medicaid or Children's Health Insurance Program (CHIP), or, if covered, become ineligible for coverage through such programs. The eligible employee and any eligible Family member who becomes eligible or loses eligibility through such programs are eligible to enroll during this special enrollment period. Coverage shall become effective on the date of eligibility/ineligibility for Medicaid/CHIP; or
- G. within thirty-one (31) days of the date coverage under another group health plan or health insurance coverage was lost, if:
 - 1. the reason the eligible employee and/or Dependent did not enroll for coverage under this Plan when initially eligible was the existence of the other coverage; and
 - 2. the person lost coverage under the other plan due to one (1) of the following:
 - a. if covered under a COBRA continuation provision, the exhaustion of COBRA continuation coverage under the other plan;
 - b. the loss of eligibility for coverage due to legal separation, divorce, death, termination of employment, reduction in hours of employment or other involuntary loss of eligibility (with the exception of terminations due to fraud or failure to pay premiums);
 - c. the overall lifetime maximum benefit under the other coverage has been exhausted so that no further expenses will be payable under such coverage; or
 - d. the termination of employer contributions towards such other coverage.

The employee must either be already enrolled as a Participant in this Plan, or be among those losing coverage under the other plan, and only those Family members losing coverage are eligible to enroll during this special enrollment period. Coverage for which a person is eligible under this provision shall become effective on the day following the date coverage under the prior plan is terminated.

In no event shall any person become covered under this Plan prior to the date the Participant becomes a Covered Person, or prior to the end of the waiting period listed in Section 5.2.

A Participant can also change Plan options during a special enrollment period.

Any Family member not listed above as eligible for the special enrollment can only be enrolled during the Plan's annual open enrollment period.

5.8 OPEN ENROLLMENT

The Plan will have an annual open enrollment period during which otherwise eligible persons who were not enrolled when initially eligible (or who previously terminated coverage) and do not qualify for one of the special enrollment periods described in Section 5.7 can be enrolled in the Plan, or, if previously enrolled, change coverage options. The open enrollment will be conducted during the month of December. The specific dates that it will be conducted during any Calendar Year will be announced, in advance, to all interested persons by the Plan Administrator. Changes made during this open enrollment period will become effective January 1st of the Calendar Year following the open enrollment.

5.9 PARTICIPANT TERMINATION

Participant coverage terminates immediately upon the earliest of the following dates:

- A. if covered as a regular, Full-Time Employee, as described in Section 5.2A, the date in which the Participant is no longer paid for working the number of hours required for Full-Time Employee status, or otherwise fails to meet the eligibility requirements listed in Section 5.2;
- B. if covered under the provisions of Section 5.2B, the earlier of the last day of the last Stability Period during which the employee was eligible if the employee failed to average thirty (30) hours per week during the latest Measurement Period that applies to such employee, or the date such employee's employment with the Employer is terminated. This will be considered to be a reduction in hours Qualifying Event for the purposes of this Plan's COBRA continuation provisions;
- C. if covered as a Board Member, the date he or she is no longer serving in that capacity;
- D. the date specified in the notification from the Plan Administrator that coverage is terminated due to fraud or a material fraudulent act committed or contributed to by the Participant, including, but not limited to, intentionally submitting false claims to the Plan, or knowingly allowing the use of a Plan identification card to obtain Plan benefits by a person who is not authorized to do so;
- E. the last day of the period for which a Participant Contribution was made following the date the Participant fails to make any required Participant Contribution for coverage; or
- F. the date the Plan is terminated or, with respect to any benefit of the Plan, the date of termination of any such benefit.

In addition, coverage may continue under the Plan, under certain circumstances and in accordance with applicable federal laws. Such continuation may be at the Participant's or Dependent's own expense. For further clarification, refer to the Family and Medical Leave provisions as described in Section 5.14, the COBRA continuation coverage as described in Article VII, Section 5.11 and Section 5.12. This Plan will also comply with the continuation provisions contained in the Uniformed Services Employment and Reemployment Rights Act of 1994 (USERRA) as they apply to Participants entering Service in the Uniformed Services, as described in Section 5.15.

5.10 DEPENDENT TERMINATION

Dependent Coverage terminates immediately upon the earliest of the following dates:

- A. the date the Participant's coverage ceases under this Plan;
- B. the last day of the month in which the Participant's Dependent child ceases to be an eligible Dependent due to reaching the limiting age of twenty-six (26);
- C. otherwise, the date the Dependent ceases to be a Dependent, as defined in the Plan;
- D. the date the Dependent spouse becomes eligible for his or her own health plan coverage. The spouse will be terminated as of the effective date of eligibility in the other plan;
- E. the date specified in the notification from the Plan Administrator that coverage is terminated due to fraud or a material fraudulent act committed or contributed to by the Dependent, including, but not limited to, intentionally submitting false claims to the Plan, or knowingly allowing the use of a Plan identification card to obtain Plan benefits by a person who is not authorized to do so;
- F. the last day of the period for which a Participant Contribution for Dependent Coverage was made following the date the Participant fails to make any required Participant Contribution for Dependent Coverage; or
- G. the date of cancellation of Dependent benefits under this Plan.

In addition, coverage may continue under the Plan, under certain circumstances and in accordance with applicable federal laws. Such continuation may be at the Participant's or Dependent's own expense. For further clarification, refer to the COBRA continuation coverage as described in Article VII.

5.11 CONTINUATION OF COVERAGE UNDER SEVERANCE AGREEMENT

Coverage for a Participant and his or her eligible Dependents under this Plan may be continued if the Participant has terminated employment but is being continued under an Employer-approved severance agreement until the earliest of the following dates:

- A. the date the Participant fails to make any required Participant Contribution for this coverage;
- B. the date specified in the written agreement that continuation benefits should cease;
- C. the date that is twelve (12) months from the date the Participant was no longer Actively at Work;
- D. the date the Participant elects to drop this coverage, or, in regards to any Dependent, the date such Dependent becomes ineligible or coverage is voluntarily terminated for such Dependent (once such coverage is terminated, it cannot be reinstated);
- E. the date the Participant becomes eligible for coverage as an employee under any other similar health plan sponsored by another employer; or
- F. the date that this Plan is terminated.

Any continuation rights that the Participant may be entitled to under the provisions of COBRA, as described in Article VII, shall begin after the period of continuation described above.

5.12 CONTINUATION OF COVERAGE DURING SHORT TERM DISABILITY OR WORKER'S COMPENSATION DISABILITY

Coverage for a Participant and his or her eligible Dependents under this Plan may be continued if the Participant is no longer Actively at Work due to a disability for which he or she is receiving short term disability or worker's compensation benefits under this Plan until the earliest of the following dates:

- A. the date the Participant is required to return to Active Work by the Employer, and he or she fails to do so;
- B. the date that the Participant's Physician certifies that he or she is able to return to full-time employment, and such Participant fails to do so;

- C. the date the Participant fails to make any required Participant Contribution for this coverage;
- D. if receiving worker's compensation benefits, six (6) months from the date the Participant was first absent due to the work-related Injury;
- E. if otherwise receiving short term disability benefits, the date that the maximum period of short term disability benefits, as listed in Section 2.11, has been exhausted;
- F. the date the Participant elects to drop this coverage, or, in regards to any Dependent, the date such Dependent becomes ineligible or coverage is voluntarily terminated for such Dependent (once such coverage is terminated, it cannot be reinstated);
- G. the date the Participant becomes eligible for coverage as an employee under any other similar health plan sponsored by another employer; or
- H. the date that this Plan is terminated.

Any continuation rights that the Participant may be entitled to under the provisions of COBRA, as described in Article VII, shall begin after the period of continuation described above.

5.13 REINSTATEMENT

If a Participant's coverage is terminated under this Plan because he or she no longer meets the eligibility requirements listed in Section 5.2, and such Participant once again meets such requirements within six (6) months of the date of termination, coverage for such Participant and his or her then eligible Dependents will be reinstated in this Plan effective the date he or she once again meets such requirements, provided written application for coverage is submitted within thirty-one (31) days of such re-eligibility.

5.14 FAMILY AND MEDICAL LEAVE PROVISIONS

This Plan intends to comply with the Family and Medical Leave Act of 1993 (FMLA) regarding the maintenance of health benefits during any period that an eligible employee takes a leave of absence in accordance with the Employer's FMLA policy, if the Employer is subject to such law. In applicable situations, FMLA allows an eligible employee to maintain group health plan coverage at the level and under the conditions coverage would have been provided if the employee had continued in employment continuously for the duration of such leave. Employee eligibility requirements, the obligations of the Employer and employees concerning conditions of leave, and notification and reporting requirements are specified in the Employer's FMLA policy. If the Employer is subject to FMLA, any Plan provision that conflicts with FMLA are superseded by FMLA to the extent such provision conflicts with FMLA. Questions regarding rights and/or obligations under FMLA should be directed to an Employer representative or the Plan Administrator.

5.15 USERRA RIGHTS

A Participant under this Plan who is no longer Actively at Work due to his or her Service in the Uniformed Services can elect, under the provisions of the Uniformed Services Employment and Reemployment Rights Act of 1994 (USERRA) to continue Participant and Dependent Coverage under this Plan for up to twenty-four (24) months after such coverage would otherwise have terminated. This period of continued coverage shall run concurrently with any continuation for which any Covered Person would have been entitled to under the provisions of COBRA due to the Participant's termination or reduction in hours of employment. If the Service in the Uniformed Services is for thirty-one (31) days or more, the Participant Contribution for such coverage will be one hundred two percent (102%) of the full cost of the coverage, without any Employer contribution. If the Service in the Uniformed Services is less than thirty-one (31) days, the Participant Contribution shall be the same as would have applied if the Participant were still an active employee.

If coverage is not continued as described above, or the Service in the Uniformed Services exceeds the time limit listed above, upon release from his or her Service in the Uniformed Services, coverage will be reinstated in the Plan effective the date the employee is reemployed by the

Employer, provided the employee reapplies for employment or reports back to work within the following applicable time:

- A. if the period of service was less than thirty-one (31) days, the beginning of the next regularly scheduled work period on the first full day after release from Service in the Uniformed Services, taking into account safe travel home plus an eight (8) hour rest period;
- B. if the period of service was more than thirty (30) days, but less than one hundred eighty-one (181) days, within fourteen (14) days of release from Service in the Uniformed Services; and
- C. if the period of service was more than one hundred eighty (180) days, but less than five (5) years, within ninety (90) days of the release from Service in the Uniformed Services.

This period may be extended for up to two (2) years from the date the Service in the Uniformed Services ended, under the provisions of USERRA, if the person is unable to return to active employment due to a disability Incurred while performing Service in the Uniformed Services.

The Plan Administrator reserves the right to request verification of any Service in the Uniformed Services, including copies of military orders or the applicable Form DD 214.

ARTICLE VI

COST MANAGEMENT SERVICES

6.1 UTILIZATION REVIEW

The Plan has a utilization pre-certification provision. Pre-admission certification should be obtained for every Inpatient admission to a covered facility, including, but not limited to Hospitals, Skilled Nursing Facilities, Hospices, psychiatric treatment facilities and Alcoholism and Substance Abuse treatment facilities, except Emergency admissions, Urgent Care admissions, and minimum stays following childbirth. (“Emergency” and “Urgent Care” admissions are defined below). A “minimum stay following childbirth” is either:

- A. a stay following a normal vaginal delivery that is forty-eight (48) hours or less; or
- B. a stay following a cesarean section that is ninety-six (96) hours or less.

If a Hospital stay following childbirth will exceed the limitations listed above, the Pre-Certification Center must be notified as soon as the Covered Person and/or her provider have determined that the hospitalization will exceed such limitations, but not later than the end of the applicable period listed above.

Pre-admission certification may be made through the Utilization Review Service. The telephone number for the Utilization Review Service is listed in Article I, Plan Information, and on the medical identification card. A Covered Person may inform his or her health care provider that he or she participates in a program that has pre-admission certification provisions. In order to obtain pre-admission certification:

- A. contact the Utilization Review Service and report the upcoming Hospital or other facility stay no later than forty-eight (48) hours prior to the admission;
- B. notice can be given by:
 - 1. the Hospital or other covered facility;
 - 2. the Covered Person’s admitting Physician;
 - 3. the Covered Person;
 - 4. a family member of the Covered Person; or
 - 5. a representative of the Employer; and
- C. the Utilization Review Service must be provided with information necessary to make a decision as to the Medical Necessity of the admission.

The Utilization Review Service may request additional information that is necessary to make the determination from the Covered Person or a provider. In the case of an Urgent Care request, such information must be provided within forty-eight (48) hours of the request. A decision will be made as soon as reasonably possible, but no later than seventy-two (72) hours of the Plan’s receipt of all information necessary to make the determination. If the request does not involve Urgent Care, the information must be provided within forty-five (45) days of such request. An “Urgent Care” request is one that, if a determination is not made on an expedited basis, the life or health of the Covered Person, or the ability of the Covered Person to regain maximum function, could be seriously jeopardized, or, in the opinion of the attending Physician, the Covered Person would be subjected to severe pain that cannot be adequately managed without the care or treatment that is the subject of the request. When pre-admission certification is provided to the Covered Person, a certain number of Inpatient days for the stay will be assigned. Charges for Inpatient days that are determined by the Utilization Review Service to not be Medically Necessary are not covered under this Plan.

The Plan Administrator shall have full discretionary authority and responsibility with respect to all aspects of Plan administration, including utilization review. If a Utilization Review Service is designated by the Plan Administrator, the Utilization Review Service agrees to recognize the ultimate authority of the Plan Administrator.

6.2 CONTINUED STAY REVIEW

During a Covered Person's Inpatient stay, a Continued Stay Review will be conducted. This review applies to all Hospital or other facility admissions. The purpose of Continued Stay Review is to:

- A. provide the Utilization Review Service with an update as to the Covered Person's condition and/or progress; and, if necessary,
- B. enable the Utilization Review Service to re-evaluate the Medical Necessity of a continued Inpatient stay.

The Utilization Review Service has the right to initiate a Continued Stay Review for any Inpatient admission. The Utilization Review Service will always confirm the outcome of the Continued Stay Review by telephone or in writing. This notification will go to the Covered Person and/or the Covered Person's Physician. The notification always includes any newly authorized length of stay.

If a stay is longer than the specified number of Inpatient days that the Utilization Review Service considers to be Medically Necessary, Covered Expenses will be denied for any charges Incurred for the days not Medically Necessary. This will occur if the Utilization Review Service is informed that the confinement is no longer Medically Necessary and the Covered Person knowingly chooses to remain in the Hospital or other facility.

If the Covered Person's Physician and the Covered Person disagree with the findings of the Utilization Review Service, the Covered Person may file an appeal, in accordance with the procedures described in Article IV, with the Plan Administrator. The Plan Administrator has final authority over any such decisions.

6.3 WEEKEND ADMISSION REVIEW

All weekend (Friday, Saturday, and Sunday) Hospital admissions will be reviewed. Coverage is limited to Medically Necessary admissions.

6.4 EMERGENCY AND URGENT CARE REVIEW

If a Covered Person is admitted to a Hospital or other covered facility for an Emergency or Urgent Care admission, notice of the admission may be provided to the Utilization Review Service no later than forty-eight (48) hours after the admission or as soon as reasonably possible. Notice may be given to the Utilization Review Service by:

- A. the Hospital or other facility;
- B. the Covered Person's admitting Physician;
- C. the Covered Person;
- D. a family member of the Covered Person; or
- E. a representative of the Employer.

The Utilization Review Service will review the case with the Covered Person's Physician to determine if a continued Inpatient stay is Medically Necessary. Charges for Inpatient days that are determined by the Utilization Review Service to not be Medically Necessary are not covered under this Plan.

An Emergency admission is an admission to a Hospital through the emergency room of that facility for treatment of a life threatening illness or injury. An Urgent Care admission is an unplanned admission or an admission scheduled less than forty-eight (48) hours prior, for a condition requiring prompt medical attention. An Urgent Care admission is not an admission through the emergency room.

6.5 DISCHARGE PLANNING

Review for Discharge Planning occurs during hospitalization review. The purpose is to:

- A. identify patients requiring extended care following discharge; and
- B. determine the most appropriate setting for continued care.

6.6 PRE-CERTIFICATION OF OUTPATIENT SURGERY AND PROTON BEAM THERAPY

The Plan recommends that all non office-based Outpatient surgery and proton beam therapy be pre-approved by the Utilization Review Service prior to any Outpatient surgical procedure or therapy. As soon as possible after a Covered Person's Physician has determined that surgery/therapy is necessary, but not later than forty-eight (48) hours prior to the surgery/therapy, the Covered Person's Physician, the Covered Person or the Hospital or facility where the procedure is to be performed should notify the Utilization Review Service and submit any documentation required by such service. The Utilization Review Service reserves the right to request additional records or information from the Covered Person, the Covered Person's Physician, and Hospital or other facility or provider that is related to the surgical procedure or proton beam therapy.

6.7 INDIVIDUAL BENEFITS MANAGEMENT

Individual Benefits Management is designed to inform Covered Persons of more cost effective settings for treatment. On an exception basis and subject to approval, the Utilization Review Service may provide benefits for settings not expressly provided for under the Plan, but which are not prohibited by law, rule or federal policy. All requests for Individual Benefits Management will be individually reviewed by the Utilization Review Service.

Services and Supplies provided in connection with Individual Benefits Management must be:

- A. for an acute level of care;
- B. Medically Necessary; and
- C. provided in a more cost effective setting.

The Utilization Review Service has the right to deny an extension of benefits under Individual Benefits Management. The Utilization Review Service also has the right to administer benefits pursuant to the terms of the Plan, exclusive of this provision. If benefits are provided to a Covered Person, under this provision for Individual Benefits Management, that are outside of the conditions, limitations and/or exclusions of this Plan, the Covered Person has no right to expect that the same or similar benefits (provided outside of the conditions, limitations and/or exclusions of this Plan) will be provided to that Covered Person in the future.

The Plan Administrator shall have full discretionary authority and responsibility with respect to all aspects of Plan administration, including utilization review. If a Utilization Review Service is designated by the Plan Administrator, the Utilization Review Service agrees to recognize the ultimate authority of the Plan Administrator.

6.8 SECOND SURGICAL OPINION

The Plan will provide benefits for a second (and third, if necessary) opinion consultation to determine the Medical Necessity of an elective surgical procedure. An elective surgical procedure is one that can be scheduled in advance; that is, it is not performed in an Emergency or for a condition of a life-threatening nature.

The patient may choose any board-certified specialist who is affiliated in the appropriate specialty who is not an associate of the Physician who originally recommended or will perform the surgery. The Plan Administrator and the Utilization Review Service reserve the right to direct the Covered Person to a Physician of their choosing for a second surgical opinion.

ARTICLE VII

CONTINUATION COVERAGE UNDER COBRA

7.1 RIGHT TO ELECT CONTINUATION COVERAGE

If a Qualified Beneficiary loses coverage under the Group Health Plan due to a Qualifying Event, he or she may elect to continue coverage under the Group Health Plan in accordance with COBRA upon payment of the monthly contribution specified from time to time by the Company. A Qualified Beneficiary must elect the coverage within the sixty (60) day period beginning on the later of:

- A. the date of the Qualifying Event; or
- B. the date the Qualified Beneficiary was notified of his or her right to continue coverage.

If a Covered Employee has been determined to be an Eligible TAA Recipient or an Eligible Alternative TAA Recipient, as those terms are defined in the Trade Act of 2002, such Covered Employee and his or her Dependents who lost coverage under the Plan due to a job loss that qualified such employee for TAA assistance shall be entitled to a second sixty (60) day election period (if continuation coverage was not elected during the period described above) beginning on the first day of the month in which the Covered Employee is determined to be TAA eligible, provided such election is made within six (6) months of the original loss of coverage. If elected under this provision, coverage shall begin on the first day of the month in which the Covered Employee is determined to be TAA eligible.

7.2 NOTIFICATION OF QUALIFYING EVENT

If the Qualifying Event is divorce, legal separation or a Dependent child's ineligibility under a Group Health Plan, the Qualified Beneficiary must notify the Company, in writing addressed to the Plan Administrator, of the Qualifying Event within sixty (60) days of the event, or sixty (60) days of the date the Qualified Beneficiary would lose coverage because of the event, in order for coverage to continue. Appropriate documentation of the Qualifying Event must be submitted, including, as appropriate, final divorce and legal separation decrees issued and properly signed by the court. In addition, a Totally Disabled Qualified Beneficiary must notify the Company in accordance with the section below entitled "Total Disability" in order for coverage to continue.

7.3 LENGTH OF CONTINUATION COVERAGE

A Qualified Beneficiary who loses coverage may continue coverage under the Group Health Plan for:

- A. a Qualified Beneficiary who loses coverage due to the reduction in hours or termination of employment (other than for gross misconduct) of a Covered Employee:
 - 1. for up to eighteen (18) months from the date of the Qualifying Event; or
 - 2. if a Qualified Beneficiary is Totally Disabled at any time during the first sixty (60) days of Continuation Coverage, he or she may continue coverage for up to twenty-nine (29) months from the date of the Qualifying Event, provided the Qualified Beneficiary notifies the Company of the determination of his or her Total Disability under the Social Security Act:
 - a. before the end of the original eighteen (18) month continuation period; and
 - b. within sixty (60) days following the date of such determination; or
- B. a Qualified Beneficiary who loses coverage due to the Covered Employee's death, divorce, or Medicare eligibility and Dependent children who have become ineligible for coverage may continue under the Group Health Plan for up to thirty-six (36) months from the date of the Qualifying Event.

7.4 TERMINATION OF CONTINUATION OF COVERAGE

Continuation Coverage will automatically end earlier than the applicable eighteen (18) or thirty-six (36) month period for a Qualified Beneficiary if:

- A. the required monthly contribution for coverage is not received by the Company within thirty (30) days following the date it is due;
- B. the Qualified Beneficiary becomes covered under any other Group Health Plan containing an exclusion or limitation relating to a pre-existing condition, and such exclusion or limitation applies to the Qualified Beneficiary, then the Qualified Beneficiary shall be eligible for Continuation Coverage as long as the exclusion or limitation relating to the pre-existing condition applies to the Qualified Beneficiary;
- C. for Totally Disabled Qualified Beneficiaries continuing coverage for up to twenty-nine (29) months, the last day of the month coincident with or following thirty (30) days from the date of a final determination by the Social Security Administration that such Qualified Beneficiary is no longer Totally Disabled;
- D. the Qualified Beneficiary becomes entitled to Medicare benefits; or
- E. the Company ceases to offer any Group Health Plans.

7.5 MULTIPLE QUALIFYING EVENTS

If a Qualified Beneficiary is continuing coverage due to a Qualifying Event for which the maximum Continuation Coverage is eighteen (18) months, and a second Qualifying Event occurs during the eighteen (18) month period, the Qualified Beneficiary may elect, in accordance with the section entitled "Right to Elect Continuation Coverage," to continue coverage under the Group Health Plan for up to thirty-six (36) months from the date of the first Qualifying Event.

7.6 TOTAL DISABILITY

In the case of a Qualified Beneficiary who is determined under Title II or XVI of the Social Security Act (hereinafter the "Act") to have been Totally Disabled at the time of a Qualifying Event or at any time during the first sixty (60) days of the Qualified Beneficiary's Continuation Coverage (if the Qualifying Event is termination of employment or reduction in hours), that Qualified Beneficiary may continue coverage (including coverage for Dependents who were covered under the Continuation Coverage) for a total of twenty-nine (29) months as long as the Qualified Beneficiary notifies the Employer, in writing addressed to the Plan Administrator:

- A. prior to the end of eighteen (18) months of Continuation Coverage that he or she was disabled as of the date of the Qualifying Event; and
- B. within sixty (60) days of the determination of Total Disability under the Act.

A copy of the determination letter from Social Security must be submitted with the notification.

The Employer will charge the Qualified Beneficiary an increased contribution for Continuation Coverage extended beyond eighteen (18) months pursuant to this Section.

If during the period of extended coverage for Total Disability (Continuation Coverage months nineteen (19) through twenty-nine (29)) a Qualified Beneficiary is determined to be no longer Totally Disabled under the Act:

- A. the Qualified Beneficiary shall notify the Employer of this determination within thirty (30) days; and
- B. Continuation Coverage shall terminate the last day of the month following thirty (30) days from the date of the final determination under the Act that the Qualified Beneficiary is no longer Totally Disabled.

7.7 CARRYOVER OF COPAYMENTS AND PLAN MAXIMUMS

If Continuation Coverage under the Group Health Plan is elected by a Qualified Beneficiary under COBRA, expenses already credited to the Plan's applicable Copayment features for the year will be carried forward into the Continuation Coverage elected for that year.

Similarly, amounts applied toward any maximum payments under the Plan will also be carried forward into the Continuation Coverage. Coverage will not be continued for any benefits for which Plan maximums have been reached.

7.8 PAYMENTS OF PREMIUM

The Group Health Plan will determine the amount of premium to be charged for Continuation Coverage for any period, which will be a reasonable estimate of the cost of providing coverage for such period for similarly situated individuals, determined on an actuarial basis and considering such factors as the Secretary of Labor may prescribe.

The Group Health Plan may require a Qualified Beneficiary to pay a contribution for coverage that does not exceed one hundred two percent (102%) of the applicable premium for that period.

For Qualified Beneficiaries whose coverage is continued pursuant to the Section entitled "Total Disability" of this provision, the Group Health Plan may require the Qualified Beneficiary to pay a contribution for coverage that does not exceed one hundred fifty percent (150%) of the applicable premium for continuation coverage months nineteen (19) through twenty-nine (29).

Contributions for coverage may, at the election of the payer, be paid in monthly installments.

If Continuation Coverage is elected, the first monthly contribution for coverage must be made within forty-five (45) days of the date of election.

Without further notice from the Company, the Qualified Beneficiary must pay the monthly contribution for coverage by the first day of the month for which coverage is to be effective. If payment is not received by the Company within thirty (30) days of the payment's due date, Continuation Coverage will terminate in accordance with the section entitled "Termination of Continuation Coverage," Subsection A.

No claim will be payable under this provision for any period for which the contribution for coverage is not received from or on behalf of the Qualified Beneficiary.

7.9 DEFINITIONS

For purposes of this Article VII, unless specifically stated otherwise, the following definitions apply:

- A. "COBRA" means the Consolidated Omnibus Budget Reconciliation Act of 1985, as amended.
- B. "Code" means the Internal Revenue Code of 1986, as amended.
- C. "Company" means the Employer, as defined in Article III.
- D. "Continuation Coverage" means the Group Health Plan coverage elected by a Qualified Beneficiary under COBRA.
- E. "Covered Employee" has the same meaning as that term is defined in COBRA and the regulations thereunder.
- F. "Group Health Plan" has the same meaning as that term is defined in COBRA and the regulations thereunder.
- G. "Qualified Beneficiary" means:
 - 1. a Covered Employee whose employment terminates (other than for gross misconduct) or whose hours are reduced, rendering the Covered Employee ineligible for coverage under the Plan; and
 - 2. a covered spouse or Dependent who becomes eligible for coverage under the Plan due to a Qualifying Event, as defined below. Qualified Beneficiary also includes any child who is born to or Placed for Adoption with the Covered Employee during the period of Continuation Coverage.
- H. "Qualifying Event" means the following events that, but for Continuation Coverage, would result in the loss of coverage of a Qualified Beneficiary:

1. termination of a Covered Employee's employment (other than gross misconduct) or reduction in the Covered Employee's hours of employment;
 2. the death of the Covered Employee;
 3. the divorce or legal separation of the Covered Employee from his or her spouse;
 4. the Covered Employee becoming entitled to Medicare coverage; or
 5. a child ceasing to be eligible as a Dependent child under the terms of the Group Health Plan.
- I. "Totally Disabled" or "Total Disability" means totally disabled as determined under Title II or Title XVI of the Social Security Act.

ARTICLE VIII

DESCRIPTION OF MEDICAL BENEFITS

In order to be eligible for the benefits described in this article, the Covered Person must be enrolled under the medical Plan option.

8.1 MEDICAL BENEFITS – COVERED EXPENSES

In order to be eligible for benefits under this section of the Plan, charges actually Incurred by a Covered Person must be for services or supplies administered or ordered by a Physician, be provided by a properly licensed or certified health care professional or entity, and be Medically Necessary for the diagnosis and treatment of an Illness or Injury unless otherwise specifically covered, including coverage under the Recommended Wellness Services. In addition, such charges will only be covered to the extent that they do not exceed the Maximum Allowable Charge for the service or supply in question.

Covered charges include the following:

A. Charges made by a Hospital for:

1. Room and Board, subject to the limitations listed in Section 2.4, including confinement in an Intensive Care Unit; and
2. other necessary ancillary services and supplies provided to the Covered Person on an Inpatient or Outpatient basis, including charges Incurred in an emergency room.

Pre-certification is recommended for all Inpatient confinements, Outpatient surgical procedures and proton beam therapy. For more information, see Article VI.

All Drugs administered or dispensed by a Hospital on an Outpatient basis are subject to the limitations for Drugs and Pain Injections listed in Section 2.5.

B. Charges for pre-admission testing, including diagnostic laboratory tests and x-ray exams when:

1. performed on an Outpatient basis within seven (7) days prior to a Hospital confinement;
2. related to the condition that causes the confinement; and
3. performed in place of tests while Hospital confined.

Covered Expenses for this testing will be payable as pre-admission testing even if tests show the condition requires medical treatment prior to Hospital confinement or the Hospital confinement is not required.

C. Charges for services and supplies provided in an Ambulatory Surgical Facility, urgent care facility or other licensed Outpatient free-standing medical facility. Precertification is recommended for all surgical procedures and proton beam therapy, as described in Section 6.6. Drugs provided in a free-standing facility are subject to the limitations for Drugs and Pain Injections listed in Section 2.5.

D. Charges for Room and Board, nursing care and other necessary services and supplies furnished by a Skilled Nursing Facility, subject to the limitations listed in Section 2.5. In order to be a Covered Expense under this Plan, the following conditions must be met:

1. the Covered Person must be confined as a bed patient in the facility;
2. the confinement must start within fourteen (14) days of a Hospital confinement of at least three (3) days;
3. the attending Physician must certify that the confinement is needed for further care of the condition that caused the Hospital confinement; and
4. the attending Physician must complete a treatment plan which includes a diagnosis, the proposed course of treatment and the projected date of discharge from the Skilled Nursing Facility.

It is recommended that all confinements in a Skilled Nursing Facility be pre-certified, as described in Article VI.

- E. Charges for Hospice care services and supplies when the attending Physician has diagnosed the Covered Person's condition as being terminal, determined that the person is not expected to live more than six (6) months and placed the person under a Hospice Care Plan. Covered Expenses include:

1. Inpatient care in a Hospice Unit or other licensed facility. Pre-certification is recommended, as described in Article VI;
2. home care; and
3. Family counseling during the bereavement period for the patient's immediate covered Family members provided by a licensed social worker or a licensed pastoral counselor, subject to the limitations listed in Section 2.5.

Prescription Drugs, including injectables, provided through the Hospice are subject to the limitations on Drugs and Pain Injections listed in Section 2.5.

- F. Charges for services and supplies provided through a Home Health Care Agency pursuant to a Home Health Care Plan, subject to the limitations listed in Section 2.5. Covered Expenses include:

1. part-time or intermittent nursing care by or under the supervision of a registered nurse (R.N.);
2. part-time or intermittent home health aide services provided through a Home Health Care Agency (this does not include general housekeeping services or Custodial Care);
3. physical, occupational and speech therapy;
4. medical supplies; and
5. laboratory services by or on behalf of the Home Health Care Agency.

Prescription Drugs, including injectables, provided through the Home Health Care Agency are subject to the limitations on Drugs and Pain Injections listed in Section 2.5.

A home health care visit will be considered a periodic visit by either a nurse or therapist, as the case may be, or four (4) hours of home health aide services.

- G. Charges for facility and professional services for diagnostic x-rays and laboratory services.
- H. Charges for local Medically Necessary professional land ambulance service if the service is to the nearest Hospital or Skilled Nursing Facility where necessary treatment can be provided. Air ambulance or trips beyond the local area will be covered if the Plan Administrator determines that they are Medically Necessary.

For a Covered Person who is in a Hospital or other health care facility under the care or supervision of a licensed health care provider, a prior authorization should be obtained before transport of the Covered Person via air transport, or any form of flight, to another Hospital or facility. Prior authorization can be obtained by contacting the designated inter-facility air ambulance review service, as described in Article I.

The designated inter-facility air ambulance review service may discuss with the Physician and/or Hospital/facility the diagnosis and the need for inter-facility patient transport versus alternatives.

- I. Charges for the professional services of a Physician for medical services provided in a Hospital or other covered facility, an office or clinic or in the Covered Person's home. Prescription Drugs, including injectables, provided in the Physician's office are subject to the limitations on Drugs and Pain Injections listed in Section 2.5.

- J. Charges for the professional services of a Physician in connection with covered surgical procedures performed on an Inpatient or Outpatient basis. Charges for Covered Expenses, including both Physician and Facility expenses, for robotic surgical procedures and related expenses will be limited to the Maximum Allowable Charge for the same surgical procedure performed under standard methods. Covered Expenses for multiple surgical procedures will be determined, as follows:
1. if bilateral or multiple surgical procedures are performed by one (1) surgeon during the same operative session, Covered Expenses will be limited to the Maximum Allowable Charge for the primary procedures and fifty percent (50%) of the Maximum Allowable Charge for each additional procedure performed through the same incision. Any procedure that would not be an integral part of the primary procedure or is unrelated to the diagnosis will be considered “incidental” and no benefits will be provided for such procedures;
 2. if multiple unrelated surgical procedures are performed by two (2) or more surgeons on separate operative fields during the same operative session, Covered Expenses will be limited to the Maximum Allowable Charge for each surgeon’s primary procedure. If two (2) or more surgeons perform a procedure that is normally performed by one (1) surgeon, Covered Expenses for all surgeons will not exceed the Maximum Allowable Charge for that procedure; and
 3. if an assistant surgeon is required, the assistant surgeon’s covered charge will not exceed twenty percent (20%) of the Covered Expenses for the primary surgeon.
- K. Charges for cosmetic treatment, but only in connection with the following:
1. reconstructive surgery to repair damage caused by infection or disease. Covered Expenses include breast reconstruction in connection with a mastectomy, including:
 - a. reconstruction of the breast on which the mastectomy was performed;
 - b. surgery and reconstruction of the other breast to produce a symmetrical appearance; and
 - c. prostheses and physical complications of all stages of mastectomy, including lymphedemas.Such reconstruction must be performed in a manner determined in consultation with the attending Physician and the Covered Person;
 2. surgery performed on a Dependent child to correct a functional defect caused by a congenital disease or anomaly; or
 3. surgery to repair damage due to an Injury or trauma, provided it is performed within one (1) year following the accident causing the Injury or trauma.
- L. Charges for voluntary sterilization procedures, regardless of Medical Necessity, for a Participant, the spouse of a Participant and other Dependent females. Sterilization procedures performed on Dependent male children, or the reversal of any sterilization, are not covered under this Plan.
- M. Charges for the following routine wellness services:
1. Recommended Wellness Services. Routine mammographies included in these recommendations and any prescription Drugs, including immunizations, are subject to the limitations listed in Section 2.5. Prior authorization is recommended for any genetic/genomic testing included in such recommendations; and
 2. other prostate examinations, including PSA tests.
- N. Charges related to the Pregnancy of any female Covered Person, including charges made by a Hospital, Birthing Center, Physician or nurse midwife, and any related diagnostic procedures. Elective abortions are only covered under this Plan if the mother’s life would be endangered by the continued Pregnancy and if not prohibited by applicable law.

Group health plans and health insurance issuers may not, under Federal law, restrict benefits for any Hospital length of stay in connection with childbirth for the mother or Newborn child to less than forty-eight (48) hours following a vaginal delivery, or less than ninety-six (96) hours following a cesarean section. However, Federal law generally does not prohibit the mother's or Newborn's attending provider, after consulting with the mother, from discharging the mother or her Newborn earlier than forty-eight (48) hours (or ninety-six (96) hours as applicable). In any case, plans and issuers may not, under Federal law, require that a provider obtain authorization from the Plan or the insurance issuer for prescribing a length of stay not in excess of forty-eight (48) hours (or ninety-six (96) hours).

- O. Charges Incurred for the treatment of a well Newborn in the Hospital following birth, up to the mother's discharge, including nursery charges, Physician's fees for the initial examination following the birth, necessary or required testing and routine circumcisions. The child must be an eligible Dependent and enrolled in the Plan as described in Article V.
- P. Charges for chiropractic care, including visits, diagnostic procedures and Spinal Manipulation performed by a Physician or licensed chiropractor (D.C.), subject to the limitations listed in Section 2.5.
- Q. Charges for Inpatient private duty nursing care by a licensed nurse (R.N., L.P.N. or L.V.N.) that is Medically Necessary and not Custodial Care, but only when the Hospital's Intensive Care Unit is filled or the Hospital has no Intensive Care Unit. Outpatient private duty nursing care is only covered if provided through a Home Health Care Agency, as described above.
- R. Charges for cardiac rehabilitation as deemed Medically Necessary provided services are rendered:
 - 1. under the supervision of a Physician;
 - 2. in connection with a myocardial infarction, coronary occlusion or coronary bypass surgery;
 - 3. initiated within twelve (12) weeks after other treatment for the medical condition ends; and
 - 4. in a Medical Care Facility.
- S. Charges for radiation or chemotherapy and treatment with radioactive substances. The materials and services of technicians are included. All associated Drugs are subject to the limitations for Drugs listed in Section 2.5. Proton beam therapy should be pre-certified, as described in Section 6.6.
- T. Charges for dialysis services provided in a home, Physician's office or other covered Medical Facility.
- U. Charges for Medically Necessary sleep studies, but only when approved, in advance, by the Plan Administrator.
- V. Charges for occupational therapy performed by a licensed occupational therapist, subject to the limitations listed in Section 2.5. Such therapy must be ordered by a Physician, result from an Injury or Sickness and improve a body function. Recreational programs, maintenance therapy or supplies used in occupational therapy are not covered under this Plan.
- W. Charges for physical therapy, including aquatic therapy/hydrotherapy when indicated for the Medically Necessary treatment of certain musculoskeletal conditions, performed by a licensed physical therapist. The therapy must be in accord with a Physician's exact orders as to type, frequency and duration and provided to improve a body function.

- X. Charges for speech therapy performed by a licensed speech therapist. Therapy must be ordered by a Physician and be necessary due to the following:
 - 1. surgery for correction of a congenital condition of the oral cavity, throat or nasal complex (other than a frenectomy) of a Covered Person;
 - 2. an Injury; or
 - 3. an Illness other than a learning disability.
- Y. Charges for the rental of durable medical or surgical equipment if deemed Medically Necessary. These items may be bought rather than rented, with the cost not to exceed the fair market value of the equipment at the time of purchase, but only if agreed to in advance by the Plan Administrator. The rental, or purchase if less costly, of breastfeeding equipment including related services and supplies is also covered under this Plan.
- Z. Charges for any of the following, including costs associated with the administration:
 - 1. anesthetics;
 - 2. oxygen;
 - 3. blood and blood derivatives that are not donated or replaced; and
 - 4. intravenous injections and solutions. All associated Drugs are subject to the limitations for Drugs listed in Section 2.5.
- AA. Charges for the initial purchase, fitting and repair of Prosthetic Devices. Covered Expenses include, but are not limited to, the following:
 - 1. the initial contact lenses or glasses required following cataract surgery;
 - 2. lenses for aphakic patients;
 - 3. soft lenses or sclera shells intended for use as corneal bandages;
 - 4. the initial purchase of a wig after chemotherapy; and
 - 5. prosthetic bras following a mastectomy.
- AB. Charges for the initial purchase, fitting and repair of orthotic appliances such as braces, splints or other appliances which are required for support for an injured or deformed part of the body as a result of a disabling congenital condition or an Injury or Illness. Diabetic shoes and custom shoe inserts are covered, subject to the limitations listed in Section 2.5.
- AC. Charges for surgical dressings, splints, casts and other devices used in the reduction of fractures and dislocations.
- AD. Charges for services and supplies that are otherwise covered under the Plan that are related to the care and treatment required due to an organ or tissue transplant. The transplant must be performed to replace an organ or tissue. In addition Covered Expenses include charges related to a human donor, or for obtaining donor organs or tissues, when the recipient is a Covered Person. When the donor has other medical coverage providing benefits for his or her expenses, such coverage will pay first. The benefits under this Plan will be reduced by those payable under the donor's plan. Donor charges include those for:
 - 1. evaluating the organ or tissue;
 - 2. removing the organ or tissue from the donor; and
 - 3. transportation of the organ or tissue from within the United States and Canada to the place where the transplant is to take place. Travel and transportation for the transplant recipient, donor or any other person is not covered under this Plan.

The Plan Administrator has an agreement with a special transplant network that can provide services and supplies related to the transplant at a cost that is less, for the most part, than is charged to other patients of the facility. Covered Persons can benefit from

this agreement by incurring lower Out-of-Pocket charges related to the transplant and by not being subject to any Copayments in relation to the transplant. For more information about accessing this network, contact the Benefit Manager or the Utilization Review Service.

- AE. Charges for the diagnosis and treatment of infertility.
- AF. Charges for care of the mouth, teeth, gums and alveolar processes only if that care is for the following oral surgical procedures:
 - 1. excision of tumors and cysts of the jaws, cheeks, lips, tongue, roof and floor of the mouth;
 - 2. Emergency repair due to Injury to sound natural teeth. This repair must be made within twelve (12) months from the date of an accident;
 - 3. surgery needed to correct accidental injuries to the jaws, cheeks, lips, tongue, floor and roof of the mouth;
 - 4. excision of benign bony growths of the jaw and hard palate;
 - 5. external incision and drainage of cellulites;
 - 6. incision of sensory sinuses, salivary glands or ducts;
 - 7. removal of impacted teeth; and
 - 8. reduction of dislocations and excision of temporomandibular joints (TMJ).No charges will be covered under the medical provisions of this Plan for dental and oral surgical procedures involving orthodontic care of the teeth, periodontal disease and preparing the mouth for the fitting of or continued use of dentures.
- AG. Charges for Medically Necessary services for the care and treatment of jaw joint conditions, including Temporomandibular Joint Disorders (TMJ).
- AH. Charges for the treatment of Mental/Nervous Disorders, Alcoholism or Substance Abuse, including but not limited to the following:
 - 1. Inpatient services. Pre-Certification is recommended;
 - 2. Intensive Outpatient psychotherapy; and
 - 3. Outpatient services, including office visits and group/individual counseling.
- AI. Charges for diabetic counseling services.
- AJ. Charges for allergy treatment, including testing, injections, serum and venoms, subject to the limitations for Drugs listed in Section 2.5.
- AK. Charges for contraceptives, including all FDA approved non-oral contraceptives, devices, implants and injections prescribed for females, subject to the limitations for Drugs listed in Section 2.5, and related medical services.

Oral contraceptives are not covered, except through the Employer-sponsored prescription plan.
- AL. Charges related to tobacco cessation programs, for any of the following:
 - 1. screening for tobacco use; and
 - 2. the counseling services of a Physician for up to two (2) attempts to stop tobacco use per Calendar Year with up to four (4) counseling visits per attempt.
- AM. Charges for all genetic and genomic testing, but only if deemed Medically Necessary in connection with the treatment of an existing condition. Prior authorization through the Plan Administrator is recommended.
- AN. Charges for B-12 or iron injections when a Covered Person has a medical diagnosis beyond vitamin deficiency, such as pernicious anemia.

- AO. Charges for testosterone injections when Medically Necessary to treat a medical condition. Low testosterone due to aging is not considered Medically Necessary.
- AP. Charges enteral feedings/TPN when a Covered Person has a covered diagnosis and is dependent on enteral/parenteral feedings for their sole source of nutrition or if the Covered Person has a disease of the small bowel that necessitates such feedings to maintain weight and strength.
- AQ. Charges for massage therapy when performed with other covered manipulations or modalities, subject to the manipulation maximum listed in Section 2.5.
- AR. Charges for telehealth visits in the same manner as in-person health care provider visits, including visits with primary care providers, specialists, psychotherapists, covered consultants, and certain other medical or health services that are provided by an eligible health care provider and covered under the Plan. Services can include diagnostic evaluation and the prescribing of medication. Telehealth visits can be performed from offices, hospitals, and other locations such as nursing homes and assisted living facilities. Copayments, coinsurance, and deductibles shall be applied based on the tier, type of health care provider and locations where the telehealth services are being performed. Any exclusion in the Plan contrary to the foregoing shall be void based on the inclusion of telehealth services described herein.
- AS. Charges for Medically Necessary cultured chondrocyte implants, including matrix-induced autologous chondrocyte implantation (MACI), when prior authorization is obtained.
- AT. Charges for Specialty Drugs, as defined in Section 3.2. Specialty Drugs are limited to coverage for (ninety) 90 days from the date of first use, prescription, or administration. After this first ninety (90) day period, such Specialty Drugs are not covered. The following drugs are not subject to this provision and will be a Covered Expense beyond the ninety (90) day period described above:
 - 1. anti-coagulants;
 - 2. anti-rejection drugs;
 - 3. drugs provided in an Emergency or otherwise determined by the Plan Administrator, in its discretion, to be a Covered Expense;
 - 4. chemotherapy drugs;
 - 5. Rebif, Soliris, Gammagard, or Amypyra; and
 - 6. drugs that cost five thousand dollars (\$5,000.00) or less.
- AU. Charges for the treatment of Autism Spectrum Disorders, including, but not limited to applied behavioral analysis (ABA) therapy. Services are subject to any applicable limitations listed in Section 2.5. No limitations apply to ABA therapy.

ARTICLE IX

DESCRIPTION OF DENTAL BENEFITS

In order to be eligible for the benefits described in this article, the Covered Person must be enrolled under the dental Plan option.

9.1 DENTAL BENEFITS – COVERED EXPENSES

Covered dental charges are the Reasonable and Customary charges made by a Dentist or other Physician for necessary care, Appliances or other dental materials, as listed below.

9.2 PREDETERMINATION OF BENEFITS

Predetermination of benefits should be obtained whenever charges for a Course of Treatment will exceed the amount listed in Section 2.10, except for emergency treatment.

A predetermination of benefits is not a guarantee of payment under this Plan. Actual benefits will be based on the services performed, the status of the Covered Person's Deductible and benefit maximums at the time the claim is processed and the patient's eligibility and the Plan provisions at the time the charges are Incurred.

9.3 ALTERNATE TREATMENT PROVISION

Many dental conditions can be adequately treated in more than one way. This Plan will base Covered Expenses on the least expensive course of treatment that provides a professionally adequate result. If a Covered Person and/or his or her Dentist choose a more expensive treatment than is necessary to correct a dental problem according to accepted standards of dental practice, Covered Expenses will be limited to the Reasonable and Customary charges for the course of treatment that provides a professionally satisfactory result at the most cost-effective level. The Covered Person will be responsible for the additional costs associated with the more expensive course of treatment, in addition to any Deductibles or his or her share after any Coinsurance that is otherwise applicable.

9.4 CLASS I (PREVENTIVE AND DIAGNOSTIC) – COVERED EXPENSES

The following services are covered as Class I services, subject to the Coinsurance listed in Section 2.8, the Calendar Year maximum listed in Section 2.9, and the limitations listed below:

- A. routine oral examinations, limited to two (2) per Calendar Year;
- B. routine Scaling and cleaning (Prophylaxis), limited to two (2) per Calendar Year. Periodontal Prophylaxis are payable under Class II;
- C. emergency examinations and Palliative treatment for pain, including application of desensitizing medications;
- D. the following x-rays:
 - 1. full-mouth series, limited to one (1) every five (5) Calendar Years; and
 - 2. Bitewings, limited to two (2) series per Calendar Year;
- E. necessary diagnostic testing not ordinarily covered under the medical coverage, including:
 - 1. pulp vitality tests;
 - 2. bacteriologic studies;
 - 3. Caries susceptibility tests;
 - 4. histopathologic examinations; and
 - 5. accessions of tissue;
- F. diagnostic casts when done in evaluation in cases involving multiple missing teeth and a need for a removable Partial Denture or a fixed Bridge;

- G. Fluoride treatment, limited to one (1) treatment per Calendar Year;
- H. Sealants on the occlusal surface of a permanent Posterior Tooth, limited to Dependent children under age fourteen (14) and to once per tooth in any thirty-six (36) consecutive month period; and
- I. space maintainers to replace primary teeth, limited to Covered Persons under age nineteen (19). Recementation of space maintainers is payable under Class II.

9.5 CLASS II (BASIC) - COVERED EXPENSES

The following services are covered as Class II services, subject to the Deductible listed in Section 2.7, the Coinsurance listed in Section 2.8, the Calendar Year maximum listed in Section 2.9, and the limitations listed below:

- A. necessary dental x-rays not listed as a Covered Expense under Class I or Class IV;
- B. examinations by a dental specialist, including a periodontist, endodontist or oral surgeon, limited to once per specialty per thirty-six (36) month period;
- C. basic restorative services, such as fillings, including:
 - 1. resin or Composite on Anterior and Posterior Teeth;
 - 2. Amalgam;
 - 3. sedative fillings; and
 - 4. pin retention in addition to fillings.Gold fillings are payable under Class III;
- D. general anesthesia, if Dentally Necessary;
- E. therapeutic Drug injections;
- F. IV sedation;
- G. antibiotic injections;
- H. Endodontic services, including, but not limited to:
 - 1. pulpotomy and apexification;
 - 2. Root Canal Therapy, including retreatment of prior therapy; and
 - 3. apicoectomy;
- I. non-surgical periodontal procedures, including, but not limited to:
 - 1. periodontal Prophylaxis;
 - 2. debridement; and
 - 3. root planing and periodontal Scaling;
- J. surgical periodontal procedures, including, but not limited to:
 - 1. curettage;
 - 2. gingival flap procedures, including if performed with root planning;
 - 3. osseous surgery;
 - 4. gingivectomy;
 - 5. gingivoplasty;
 - 6. dressing changes;
 - 7. clinical crown lengthening;
 - 8. bone grafts;
 - 9. tissue regeneration;
 - 10. tissue grafting;

- 11. distal or proximal wedge procedures; and
- 12. surgical revision procedures;
- K. simple and surgical extractions not covered under the medical coverage, including pre- and post-operative care. This includes extractions performed for orthodontic purposes;
- L. other oral surgical procedures not covered under the medical coverage, including, but not limited to:
 - 1. surgical removal of a residual root;
 - 2. tooth reimplantation of accidentally evulsed or displaced tooth;
 - 3. surgical exposure to age eruption;
 - 4. alveoloplasty;
 - 5. removal of extosis;
 - 6. frenectomy;
 - 7. hyperplastic tissue removal;
 - 8. excision of pericoronal gingival; and
 - 9. incision and drainage of an abscess; and
- M. recementation of space maintainers, Crowns, Onlays, Inlays, fixed Bridgework and veneers.

9.6 CLASS III (MAJOR) – COVERED EXPENSES

The following services are covered as Class III services, subject to the Deductible listed in Section 2.7, the Coinsurance listed in Section 2.8, the Calendar Year maximum listed in Section 2.9, and the limitations listed below:

- A. gold fillings. Additional charges for gold Restorations will only be a Covered Expense when it is necessary to restore the tooth with gold;
- B. Crowns, including:
 - 1. gold. Additional charges for gold Restorations will only be a Covered Expense when it is necessary to restore the tooth with gold;
 - 2. resin;
 - 3. porcelain;
 - 4. stainless steel;
 - 5. cast;
 - 6. provisional Crowns when used as an emergency treatment of cracked tooth syndrome or for other healing needs; and
 - 7. build-up, including posts;
- C. Onlays and Inlays;
- D. labial veneers;
- E. fixed Bridgework or Dentures, including Partial Dentures, to replace one (1) or more natural teeth. Replacement is covered if:
 - 1. necessary to replace newly extracted or lost natural teeth;
 - 2. the existing Bridge or Denture was installed at least five (5) years prior to its replacement, and cannot be made serviceable; or
 - 3. the existing Denture is of an immediate temporary nature, and such replacement takes place within twelve (12) months of the initial placement;
- F. post removal;

- G. adjustments to Dentures performed more than six (6) months following the initial installation;
- H. additions of clasps or rests to existing Dentures;
- I. addition of teeth to existing Bridgework or Dentures;
- J. repairs to Crowns, Bridgework or Dentures;
- K. tissue conditioning in preparation for Dentures; and
- L. Rebases or Relines of Dentures performed more than six (6) months following the initial installation of the Denture.

9.7 CLASS IV (ORTHODONTIC) – COVERED EXPENSES

The Plan will pay Covered Dental Expenses that are related to orthodontic treatment (except extractions, which are payable under Class II), subject to the Deductible listed in Section 2.7, the Coinsurance listed in Section 2.8 and the Lifetime maximum listed in Section 2.9. Orthodontics is only covered under this Plan for Dependent children under age nineteen (19). Covered Expenses include:

- A. preliminary studies, including;
 - 1. x-rays;
 - 2. casts; and
 - 3. preparation of the treatment plan;
- B. active treatment, including Appliance therapy;
- C. retention Appliances; and
- D. minor treatment to control harmful habits, such as thumb sucking and tongue thrusting.

Benefits for comprehensive, full-banded orthodontic treatment programs will be made in installments, up to the maximum orthodontic benefit listed in Section 2.9.

ARTICLE X

DESCRIPTION OF SHORT TERM DISABILITY BENEFITS

Only Participants who are Actively at Work for the Employer on the last workday prior to the date the Total Disability commenced are eligible for this benefit.

10.1 ELIGIBILITY FOR SHORT TERM DISABILITY BENEFITS

In order to qualify for short term disability benefits under this Plan, the Participant must meet all of the following requirements:

- A. he or she must be Totally Disabled, as defined on page 25;
- B. his or her Total Disability must commence while such Participant is covered for short term disability benefits under this Plan;
- C. he or she must be continuously treated by a Physician or other Professional Provider during the period of Total Disability; and
- D. the Illness or Injury causing the Total Disability must be non-occupational, and does not arise from any work for wage or profit.

Exclusions and limitations that apply to the short term disability benefits are listed in Section 11.4.

10.2 SHORT TERM DISABILITY CLAIMS

Participants are responsible for submitting short term disability claims to the Plan on forms approved by the Plan Administrator. Such claims should be completed by the Participant's attending Physician, and include the following information:

- A. a history regarding when symptoms first appeared or an accident happened;
- B. a diagnosis;
- C. detail regarding the dates and nature of treatment;
- D. a statement as to the progress and prognosis of the Participant's condition;
- E. the Participant's suitability for rehabilitation; and
- F. the Physician's signature and tax ID number.

The Plan Administrator may request additional information from the Participant and/or his or her Physician, based upon the nature of the Illness or Injury.

The Plan Administrator reserves the right to request periodic physical examinations of the Participant from either the Participant's current Physician, or a Physician of the Plan Administrator's choosing during any period of disability for which the Participant is claiming benefits under this provision.

Failure to provide requested information or submit to an examination as described above will result in termination of short term disability benefits under this Plan.

10.3 PAYMENT OF SHORT TERM DISABILITY BENEFITS

Benefits for short term disability will commence once the Participant meets the waiting period listed in Section 2.11 and will be paid on a weekly basis during the period of Total Disability at the level described in that section, up to the maximum weeks described therein.

If the Participant returns to Active Work with the Employer prior to exhaustion of the maximum weeks described in Section 2.11, benefits will resume without an additional waiting period if such Participant is once again Totally Disabled prior to the periods of active employment described for a recurring or subsequent disability. Such a disability will be considered to be the same disability, and will be subject to the same maximum week limitation as the initial disability. If the Participant is once again Totally Disabled and unable to work beyond the specified period, it will be considered as a new period of disability, and a new waiting period and maximum benefit period shall apply.

ARTICLE XI

EXCLUSIONS AND LIMITATIONS

11.1 GENERAL PLAN BENEFIT EXCLUSIONS AND LIMITATIONS

The following exclusions and limitations apply to expenses Incurred by all Covered Persons and to all benefits provided by this Plan. Any exclusion listed below shall not apply to the extent that coverage for the service or supply is specifically provided under this Plan, or that the exclusion is prohibited under any applicable law. The exclusions listed below will not apply to the extent that they relate to an Injury the source of which is directly caused by an act of domestic violence committed on a Covered Person; an underlying medical condition (including both physical and mental health conditions) triggered by an act of domestic violence; or if such service is for an Emergency Service for the treatment of an Emergency. Exclusions that apply only to the medical coverage are listed in Section 11.2. Exclusions that apply only to the dental coverage are listed in Section 11.3. Exclusions that apply only to the short-term disability benefits are listed in Section 11.4.

- A. Charges for services or supplies provided by a program or agency funded by any government, with the exception of Medicaid, a group health plan provided by such governmental entity of its own employees, or as such exclusion is otherwise prohibited by applicable law.
- B. Charges for services or supplies received for treatment of an Injury or Illness caused or contributed to by the Covered Person's involvement in any of the following:
 - 1. engaging in an illegal act or occupation;
 - 2. committing or attempting to commit any crime, criminal act, assault or other felonious behavior; or
 - 3. participating in a riot or public disturbance.
- C. Charges that would not have been made in the absence of this or other health coverage, or for which the Covered Person has no legal obligation to pay.
- D. Charges for services or supplies that are not recommended and approved by a Physician or other Professional Provider, or for services and supplies provided during a period when the Covered Person is not under the regular care of a Physician or other Professional Provider. For the purposes of this provision "regular care" means ongoing medical supervision or treatment that is appropriate care for an Injury or Illness.
- E. Charges for any services or supplies that are determined by the Plan Administrator, in its discretion, to be Experimental or Investigative.
- F. Charges for professional services performed by a person who normally resides in the Covered Person's home, or who is the Covered Person's Close Relative.
- G. Charges related to an intentionally self-inflicted Injury or Illness, unless such act was caused by an underlying health condition, such as depression.
- H. Charges Incurred before the individual became a Covered Person under this Plan, or after such coverage has terminated.
- I. Charges for any Illness or Injury arising from war or an act of war, declared or undeclared.
- J. Charges for treatment of an Illness or Injury that has been determined by the Plan Administrator, in its discretion, to be occupational or arising from any work for wage or profit, including self-employment.

- K. Charges for facility expenses obtained through Cincinnati Children's Hospital for any Covered Person age twenty-six (26) and older. This exclusion shall not apply to non-facility Provider charges from Cincinnati Children's Hospital and associated providers.
- L. Charges for any treatment, services or supplies that is not specifically set forth as covered under this Plan.

11.2 MEDICAL BENEFIT EXCLUSIONS AND LIMITATIONS

The following exclusions and limitations apply to all medical expenses Incurred by all Covered Persons and to all medical benefits provided under the medical provisions of this Plan. Any exclusion listed below shall not apply to the extent that coverage for the service or supply is specifically provided under this Plan, or that the exclusion is prohibited under any applicable law. General exclusions that apply to the medical coverage are listed in Section 11.1.

- A. Charges that are not payable under this Plan due to application of any stated Plan maximum or limit, or because the charges are in excess of the Maximum Allowable Charge, or are otherwise not covered by this Plan, based upon the Plan Administrator's determination as set forth by and within the terms of this document.
- B. Charges for services or supplies that are determined by the Plan Administrator, in its discretion, to not be Medically Necessary, except as specifically listed as a Covered Expense under this Plan.
- C. Charges for or related to an elective abortion.
- D. Charges for or related to acupuncture treatment.
- E. Charges for any treatment required as a result of complications arising from a treatment or procedure that is not a Covered Expense under this Plan.
- F. Charges related to cosmetic treatment or procedures, except as specifically listed as a Covered Expense under this Plan.
- G. Charges for services and supplies in connection with Custodial Care, maintenance care or a rest cure.
- H. Charges for educational or vocational training, except as specifically listed as a Covered Expense under this Plan.
- I. Charges for exercise programs for the treatment of any condition, except as provided as part of or in connection with Physician supervised cardiac rehabilitation or occupational or physical therapy that is covered under this Plan.
- J. Charges for the following treatment of the eye:
 - 1. routine eye examinations, including refractions, unless included in the Recommended Wellness Services;
 - 2. lenses of the eyes and examinations for their fitting, except as specifically listed as a Covered Expense under this Plan; and
 - 3. eye surgery or procedures to correct nearsightedness or other refractive errors of the eye, including, but not limited to, radial keratotomy or LASIK procedures.
- K. Charges for treatment of the following foot-related conditions (except as specifically listed as a Covered Expense):
 - 1. weak, strained, flat, unstable or unbalanced feet;
 - 2. metatarsalgia or bunions; and
 - 3. corns, calluses or toenails, unless needed in the treatment of a metabolic or peripheral-vascular disease.

- L. Charges for services or supplies provided outside of the United States if the sole reason for the travel was for the purpose of obtaining such services or supplies.
- M. Charges in connection with hair loss, including, but not limited to, wigs (except as specifically listed as covered in connection with chemotherapy), hair transplants or any Drugs promising hair growth, whether or not prescribed by a Physician.
- N. Charges for or in connection with hearing aids, devices or implants used to improve or restore hearing (including cochlear implants or BAHA), and procedures for their fitting.
- O. Charges for professional services billed by a Physician or other Professional Provider or a nurse who is an employee of a Hospital or Skilled Nursing Facility, if such person is paid by the facility for the service.
- P. Charges related to the treatment of obesity or for weight loss or dietary control, whether it is a part of a treatment plan for another Illness, including, but not limited to:
 - 1. weight reduction programs;
 - 2. bariatric surgery;
 - 3. gastric bubble/balloon procedures;
 - 4. complications related to any weight loss procedures;
 - 5. services at a health spa or similar facility;
 - 6. services related to diatasis recti and suction lipectomy; and
 - 7. removal of excess skin or fat following obesity surgery, a diet or Pregnancy, regardless of Medical Necessity.
- Q. Charges for personal comfort items or other non-medical equipment, including, but not limited to:
 - 1. air conditioners;
 - 2. air-purification units;
 - 3. humidifiers;
 - 4. electric heating units;
 - 5. orthopedic mattresses;
 - 6. blood pressure instruments;
 - 7. scales;
 - 8. elastic bandages or stockings;
 - 9. non-prescription Drugs and medicines;
 - 10. first-aid supplies; and
 - 11. non-hospital adjustable beds.
- R. Charges for Cerezyme and Fabrazyme.
- S. Charges for the replacement of braces of the leg, arm or back or for artificial arms or legs, unless there is a sufficient change in the Covered Person's physical condition to make the original device no longer functional.
- T. Charges for services or supplies related to the treatment of non-congenital transsexualism, gender dysphoria or sexual reassignment or change, including, but not limited to, medications, implants, hormone therapy, surgery, medical treatment or psychiatric treatment.
- U. Charges for the voluntary sterilization of any male Dependent child, or charges for the reversal of any voluntary sterilization procedure.

- V. Charges for travel or accommodations, except as specifically listed as a Covered Expense under this Plan.
- W. Charges for or related to oral contraceptives. Coverage may be available through the Employer-sponsored prescription plan.
- X. Charges related to the donation of an organ or tissue, unless the transplant recipient is a Covered Person under this Plan.
- Y. Charges for the treatment of a condition that has been determined by the Plan Administrator, in its discretion, to be the result of a Never Event (see definition on page 31).
- Z. Charges submitted by a provider for services which have been unbundled contrary to the Centers for Medicare and Medicaid Services (CMS) guidelines under the Medicare program, unless prohibited by the network or participating provider contracts, if applicable.
- AA. Charges for any service billed by Northpointe Surgical Suites, Lancaster Specialty Surgery Center and Ohio Orthopedic Surgery Institute.
- AB. Charges for marital counseling.
- AC. Charges related to medical complications of procedures or treatment that are not covered under this Plan.
- AD. Charges for gene therapy.
- AE. Coverage is not available under the Plan for charges or expenses arising from care, supplies, treatment and/or services that are services, supplies, care or treatment to a Covered Person for Injury or Illness Incurred while the Covered Person was voluntarily taking or was under the influence of any controlled substance, drug, hallucinogen or narcotic not administered on the advice of a Physician, even if the cause of the Illness or Injury is not related to the use of the controlled substance, drug, hallucinogen or narcotic. Expenses will be covered for Injured Covered Persons other than the person using controlled substances and expenses will be covered for Substance Abuse treatment as specified in this Plan. This exclusion does not apply if the Injury:
 - 1. resulted from being the victim of an act of domestic violence; and
 - 2. resulted from an underlying, documented medical condition (including both physical and mental health conditions) triggered by the Injury or Illness Incurred while the Covered Person was voluntarily taking or was under the influence of any controlled substance, drug, hallucinogen or narcotic not administered on the advice of a Physician.
- AF. Charges for inter-facility air transport services that are not Medically Necessary. The Plan Administrator retains discretionary authority in determining Medical Necessity in this regard and will consider assessment by the designated inter-facility air ambulance review service in determining Medical Necessity of inter-facility patient air transport.
- AG. Charges for Specialty Drugs over five thousand dollars (\$5,000.00) used, prescribed, or administered after the first ninety (90) days, unless specifically listed as a Covered Expense.
- AH. Charges for Humira.

11.3 DENTAL BENEFIT EXCLUSIONS AND LIMITATIONS

The following exclusions and limitations apply to all dental expenses Incurred by all Covered Persons and to all dental benefits provided under the dental provisions of this Plan. Any exclusion listed below shall not apply to the extent that coverage for the service or supply is specifically

provided under this Plan, or that the exclusion is prohibited under any applicable law. General exclusions that apply to the dental coverage are listed in Section 11.1.

- A. Charges to the extent that they exceed the Reasonable and Customary charges for the service or supply in question.
- B. Charges for administrative costs associated with completing claim forms or for providing dental records.
- C. Charges for missed dental appointments.
- D. Excess charges for Crowns for teeth that are restorable by other means, or provided for the purpose of periodontal splinting.
- E. Charges for services and supplies that are Covered Expenses under the medical provisions of this Plan.
- F. Charges related to oral hygiene instruction, plaque control programs, behavior management, tobacco counseling or dietary instruction.
- G. Charges for or related to dental implants, including any Appliances and/or Crowns, and the surgical insertion or removal of implants.
- H. Charges for orthognathic surgery to correct malpositions in the bones of the jaw.
- I. Charges for maxillofacial prosthetics.
- J. Charges for the personalization or customizations of Dentures.
- K. Charges for the replacement of lost or stolen Appliances.
- L. Charges for Crowns, fillings or Appliances that are used to connect or splint teeth, or that alter the way the teeth meet, including altering the vertical dimension, restoring the bite (occlusion) or repairs, Relines and Rebases relating to such devices.
- M. Charges for dental supplies or procedures that is cosmetic, including, but not limited to, tooth bleaching and microabrasion.
- N. Charges for Drugs not specifically listed as covered under the dental provisions of this Plan, including, but not limited to, analgesics and Fluoride gel.
- O. Charges for additional local anesthetics.
- P. Charges for precision attachments.
- Q. Charges related to the treatment of Temporomandibular Joint Disorders or TMJ.
- R. Charges for Crowns, Dentures, Bridges or other dental Appliances if preparations were made, or impressions taken, prior to the date the person became a Covered Person under this Plan, or for services related to these that continue after such coverage have been terminated.
- S. Charges for Root Canal Therapy if active treatment began prior to the date the person became a Covered Person under this Plan, or to the extent that treatment continues after such coverage is terminated.
- T. Charges for treatment in excess of the Reasonable and Customary fee for the services and supplies that can provide a professionally adequate result, as described in Section 9.3.
- U. Charges for testing or oral pathology that is typically covered under medical coverage.
- V. Charges for any adjustments, Relines or Rebases of Dentures or other Prosthetic Appliances that are performed within six (6) months of their initial installation.

- W. Charges for the replacement of a temporary Crown more than one (1) year after its initial placement.
- X. Charges for services or supplies that are not provided in accordance with established dental standards, including the standards of the American Dental Association.

11.4 SHORT TERM DISABILITY EXCLUSIONS AND LIMITATIONS

The following exclusions and limitations apply to all short term disability benefits provided by this Plan. Any exclusion listed below shall not apply to the extent the exclusion is prohibited under any applicable law. Short term disability benefits will not be provided under this Plan for a disability related to any of the following:

- A. A condition which is excluded or for which coverage is otherwise not provided under the medical provisions of this Plan.
- B. Any period of disability due to cosmetic surgery.
- C. A self-inflicted Injury or Illness, even if coverage is otherwise provided under the medical provisions of this Plan.
- D. Injuries or Illnesses caused by war or any act of war.
- E. Injuries or Illnesses caused by the Participant's voluntary participation in a riot, act of civil disobedience or criminal act.
- F. Illness or Injuries caused by the Covered Person's use of illegal drugs/alcohol (such as a DUI).

ARTICLE XII

GENERAL INFORMATION

12.1 COORDINATION OF BENEFITS

Coordination of benefits (COB) is a feature that prevents duplicate payment under this Plan and other health insurance or prepayment plans, including Medicare or other types of insurance. A Covered Person may have coverage under this Plan, another health plan of coverage or another kind of insurance policy at the same time. Other health plans of coverage include a group sickness and accident insurance policy or program, a group contract of a health maintenance organization, an individual sickness and accident insurance policy or an individual contract of a health maintenance organization. Other kinds of insurance policies include your automobile insurance policy's medical payments and uninsured motorist's coverage. For example, a person may be covered by an employer's group insurance program and also by the group program provided by a spouse's employer. Or a person may be covered by an employer's group insurance and also have coverage under a parent's group plan.

If a Covered Person files a claim under this Plan for services or supplies that are also covered under another plan or insurance policy, for instance, one of the plans or policies listed in the first paragraph, payments will be "coordinated." This means that this Plan will adjust its benefit payments so that combined payments under this and any other health plan or insurance policy will be no more than the usual, Customary, and Reasonable fee payments.

Once a Covered Person has provided this Plan with information about other health benefit plans and/or health benefits through other insurance policies under which he or she has coverage, the Plan will handle the coordination. This will be done according to the "Order of Benefit Determination." The Order of Benefit Determination will be determined, as described below.

The plan that pays first is called the primary plan. Any other plan that covers the Covered Person is called the secondary plan.

- A. A group or individual plan or policy that does not contain a COB feature is always primary.
- B. A plan that covers a person as the certificate holder or the contract holder is primary. In the two examples given, the coverage the person has through his or her employer would be primary. The coverage through a spouse's or parent's employer would be secondary. The exception to this would be when the laws and regulations governing Medicare require that the plan covering the person as a Dependent pay its benefits as primary to Medicare, but such laws and regulations also provide that the plan covering them as the certificate holder/contract holder should pay its benefits as secondary to Medicare. In such a case, the plan that is required to pay as primary to Medicare shall also pay as primary to the other coverage.
- C. If a person is covered as a Dependent child under more than one (1) plan:
 1. For a Dependent child whose parents are married or are living together, even if they have never been married, the plan of the parent whose birthday falls earlier in the Calendar Year has primary responsibility for paying the claim. The plan of the parent with the later birthday becomes the secondary plan. If both parents have the same birthday, the parent whose coverage has been in effect the longest is primary (such determination is described in Subsection F below). The ages of the respective parents are not relevant. This method of coordinating benefits is commonly referred to as the "birthday rule."
 2. For a Dependent child whose parents are divorced, separated or are not living together (whether or not they have ever been married):

- a. If a court decree states that one (1) of the parents is responsible for the Dependent child's healthcare coverage, and the plan of that parent has actual knowledge of those terms, that plan is primary. If the parent with responsibility has no health care coverage for the dependent child's health care expenses, but that parent's spouse does, such spouse's plan is the primary plan. This item shall not apply with respect to any Plan Year during which benefits are paid or provided before the entity has actual knowledge of the court decree provision.
- b. If a court decree states that both parents are responsible for the Dependent child's health care coverage, the birthday rule, as described in C1 above, shall apply.
- c. If a court decree states that the parents have joint custody without specifying that one (1) parent has responsibility for the health care coverage of the Dependent child, the birthday rule, as described in C1 above, shall apply.
- d. If there is no court decree allocating responsibility for the Dependent child's health care coverage, the order of benefits for the child are as follows:
 - i) the plan of the custodial parent, if any, shall pay its benefits first;
 - ii) the plan of the spouse of the custodial parent, if any, will pay next;
 - iii) the plan of the non-custodial parent, if any, will pay after the prior listed plans; and
 - iv) the plan of the spouse of the non-custodial parent, if any, shall pay it benefits last.
- D. A plan that covers a person as an active employee or as a Dependent of an active employee is primary to a plan that covers a person as an inactive employee, such as a laid-off or retired employee or as a Dependent of a laid-off or retired employee. The exception to this rule is when the laws and regulations governing Medicare require that this Plan pay its benefits as primary to the plan covering the inactive employee.
- E. If a person is covered pursuant to COBRA or under a right of continuation pursuant to state or other federal law is covered under another plan, the plan covering the person as an employee, member, subscriber or retiree or covering the person as a dependent of an employee, member subscriber or retiree is primary to the plan covering that same person pursuant to COBRA or under a right of continuation pursuant to state or other federal law. The exception to this rule is when the laws and regulations governing Medicare require that the plan covering the person pursuant to COBRA, or under a right of continuation pursuant to state or other federal law, pay its benefits as primary to the other plan.
- F. There are some situations in which the above rules do not apply. Here the program that has been in effect longer is primary. To determine the length of time a person has been covered under a plan, two (2) successive plans through the same employer shall be treated as one (1) if the person was eligible under the second plan within twenty-four (24) hours after coverage under the first plan ended. The person's length of time covered under a plan is measured from his or her first date of coverage under that plan. If that date is not readily available, the date the person first became a member of that group shall be used as the date from which to determine the length of time the person's coverage under the present plan has been in force. Some examples of when coordination of benefits is based on coverage in effect longer are:
 - 1. when a person who works two (2) jobs has health coverage through both employers;
or

2. when a person is covered as the dependent of his or her spouse and a parent. If both coverages become effective on the same date, the birthday rule, as described in C1 above, will apply to this situation.
- G. A plan or policy that covers a specific event may be primary to a plan that provides general coverage. For example, if a person is injured in an automobile accident with an uninsured motorist, his or her automobile policy's uninsured motorist's coverage would be primary to a group health plan if both policies had similar provisions regarding other insurance.

If coverage under this Plan is primary, benefits will be paid as if the Covered Person had no other coverage. But if this coverage is secondary, this Plan's payments will be calculated by subtracting the primary plan's benefits for the services and supplies covered under this Plan from the Reasonable and Customary allowance for the services and supplies. Of course, the Plan will not pay more when secondary than it would if primary. By accepting coverage under this Plan, a Covered Person agrees to do two (2) things to enable the Plan to coordinate benefits. First, the Covered Person will supply the Plan with information about other coverage he or she has when asked. Second, if the Plan makes a payment and later finds out that the coverage under this Plan should not have been primary, the Covered Person will return the excess amount to the Plan. The Plan has the right to obtain information needed to coordinate benefits from others as well, i.e., insurance companies and other persons.

In the case of Medicare services that are furnished to End Stage Renal Disease ("ESRD") Participants who are covered under this Plan:

- A. if any Covered Person is eligible for Medicare benefits because of ESRD, the benefits of the Plan will be determined before Medicare benefits for the first thirty (30) months of Medicare entitlement, unless applicable Federal law provides to the contrary, in which event the benefits of the Plan will be determined in accordance with such law; and
- B. in order to coordinate Covered Expenses under this Plan with Medicare coverage, the Covered Person is required to:
 1. notify the Plan Administrator and send a copy of his or her Medicare card when enrolled in Medicare; and
 2. notify the Plan Administrator if or when he or she begins to receive dialysis treatments.

If Medicare reimbursement rates are neither available nor applicable, rates will be set in accordance with this Plan's Customary and Reasonable provision and other provisions.

12.2 THIRD PARTY RECOVERY, SUBROGATION AND REIMBURSEMENT

Payment Condition

The Plan, in its sole discretion, may elect to conditionally advance payment of benefits in those situations where an injury, illness, disease or disability is caused in whole or in part by, or results from the acts or omissions of Participants, and/or their Dependents, beneficiaries, estate, heirs, guardian, personal representative, or assigns (collectively referred to hereinafter in this section as "Covered Person(s)") or a third party, where any party besides the Plan may be responsible for expenses arising from an incident, and/or other funds are available, including but not limited to crime victim restitution funds, civil restitution funds, no-fault restitution funds (including vaccine injury compensation funds), uninsured motorist, underinsured motorist, medical payment provisions, third party assets, third party insurance, and/or guarantor(s) of a third party, any medical, applicable disability, or other benefit payments, and school insurance coverage (collectively "Coverage").

A Covered Person(s), his or her attorney, and/or legal guardian of a minor or incapacitated individual agrees that acceptance of the Plan's conditional payment of medical benefits is constructive notice of these provisions in their entirety and agrees to maintain one hundred percent (100%) of the Plan's conditional payment of benefits or the full extent of payment from

any one (1) or combination of first and third party sources in trust, without disruption except for reimbursement to the Plan or the Plan's assignee. The Plan shall have an equitable lien on any funds received by the Covered Person(s) and/or their attorney from any source and said funds shall be held in trust until such time as the obligations under this provision are fully satisfied. The Covered Person(s) agrees to include the Plan's name as a co-payee on any and all settlement drafts. Further, by accepting benefits the Covered Person(s) understands that any recovery obtained pursuant to this section is an asset of the Plan to the extent of the amount of benefits paid by the Plan and that the Covered Person shall be a trustee over those Plan assets.

In the event a Covered Person(s) settles, recovers, or is reimbursed by any Coverage, the Covered Person(s) agrees to reimburse the Plan for all benefits paid or that will be paid by the Plan on behalf of the Covered Person(s). When such a recovery does not include payment for future treatment, the Plan's right to reimbursement extends to all benefits paid or that will be paid by the Plan on behalf of the Covered Person(s) for charges Incurred up to the date such Coverage or third party is fully released from liability, including any such charges not yet submitted to the Plan. If the Covered Person(s) fails to reimburse the Plan out of any judgment or settlement received, the Covered Person(s) will be responsible for any and all expenses (fees and costs) associated with the Plan's attempt to recover such money. Nothing herein shall be construed as prohibiting the Plan from claiming reimbursement for charges Incurred after the date of settlement if such recovery provides for consideration of future medical expenses.

If there is more than one (1) party responsible for charges paid by the Plan or may be responsible for charges paid by the Plan, the Plan will not be required to select a particular party from whom reimbursement is due. Furthermore, unallocated settlement funds meant to compensate multiple injured parties of which the Covered Person(s) is/are only one (1) or a few, that unallocated settlement fund is considered designated as an "identifiable" fund from which the plan may seek reimbursement.

Subrogation

As a condition to participating in and receiving benefits under this Plan, the Covered Person(s) agrees to assign to the Plan the right to subrogate and pursue any and all claims, causes of action or rights that may arise against any person, corporation and/or entity and to any Coverage to which the Covered Person(s) is entitled, regardless of how classified or characterized, at the Plan's discretion, if the Covered Person(s) fails to so pursue said rights and/or action.

If a Covered Person(s) receives or becomes entitled to receive benefits, an automatic equitable lien attaches in favor of the Plan to any claim, which any Covered Person(s) may have against any Coverage and/or party causing the illness or injury to the extent of such conditional payment by the Plan plus reasonable costs of collection. The Covered Person is obligated to notify the Plan or its authorized representative of any settlement prior to finalization of the settlement, execution of a release, or receipt of applicable funds. The Covered Person is also obligated to hold any and all funds so received in trust on the Plan's behalf and function as a trustee as it applies to those funds until the Plan's rights described herein are honored and the Plan is reimbursed.

The Plan may, at its discretion, in its own name or in the name of the Covered Person(s) commence a proceeding or pursue a claim against any party or Coverage for the recovery of all damages to the full extent of the value of any such benefits or conditional payments advanced by the Plan.

If the Covered Person(s) fails to file a claim or pursue damages against:

- A. the responsible party, its insurer, or any other source on behalf of that party;
- B. any first party insurance through medical payment coverage, personal injury protection, no-fault coverage, uninsured or underinsured motorist coverage, including any similar coverage under a different name in a particular state;
- C. any policy of insurance from any insurance company or guarantor of a third party, including but not limited to an employer's policy;
- D. workers' compensation or other liability insurance company; or

- E. any other source of coverage, including but not limited to, the following:
1. crime victim restitution funds;
 2. civil restitution funds;
 3. no-fault restitution funds such as vaccine injury compensation funds;
 4. any medical, applicable disability or other benefit payments; and
 5. school insurance coverage.

The Covered Person(s) authorizes the Plan to pursue, sue, compromise and/or settle any such claims in the Covered Person(s) and/or the Plan's name and agrees to fully cooperate with the Plan in the prosecution of any such claims. The Covered Person(s) assigns all rights to the Plan or its assignee to pursue a claim and the recovery of all expenses from any and all sources listed above.

Right of Reimbursement

The Plan shall be entitled to recover one hundred percent (100%) of the benefits paid or payable benefits Incurred, that have been paid and/or will be paid by the Plan, or were otherwise Incurred by the Covered Person(s) prior to and until the release from liability of the liable entity, as applicable, without deduction for attorneys' fees and costs or application of the common fund doctrine, make whole doctrine, or any other similar legal or equitable theory, and without regard to whether the Covered Person(s) is fully compensated by his or her recovery from all sources. The Plan shall have an equitable lien which supersedes all common law or statutory rules, doctrines, and laws of any State prohibiting assignment of rights which interferes with or compromises in any way the Plan's equitable lien and right to reimbursement. The obligation to reimburse the Plan in full exists regardless of how the judgment or settlement is classified and whether or not the judgment or settlement specifically designates the recovery or a portion of it as including medical, disability, or other expenses and extends until the date upon which the liable party is released from liability. If the Covered Person's recovery is less than the benefits paid, then the Plan is entitled to be paid all of the recovery achieved. Any funds received by the Covered Person are deemed held in constructive trust and should not be dissipated or disbursed until such time as the Covered Person's obligation to reimburse the Plan has been satisfied in accordance with these provisions. The Covered Person is also obligated to hold any and all funds so received in trust on the Plan's behalf and function as a trustee as it applies to those funds until the Plan's rights described herein are honored and the Plan is reimbursed.

No court costs, experts' fees, attorneys' fees, filing fees, or other costs or expenses of litigation may be deducted from the Plan's recovery without the prior, express written consent of the Plan. Additionally, the Covered Person shall indemnify the Plan against any of the Covered Person's attorney's fees, costs, or other expenses related to the Covered Person's recovery for which the Plan becomes responsible by any means other than the Plan's explicit written consent.

The Plan's right of subrogation and reimbursement will not be reduced or affected as a result of any fault or claim on the part of the Covered Person(s), whether under the doctrines of causation, comparative fault or contributory negligence, or other similar doctrine in law. Accordingly, any lien reduction statutes, which attempt to apply such laws and reduce a subrogating Plan's recovery will not be applicable to the Plan and will not reduce the Plan's reimbursement rights.

These rights of subrogation and reimbursement shall apply without regard to whether any separate written acknowledgment of these rights is required by the Plan and signed by the Covered Person(s).

This provision shall not limit any other remedies of the Plan provided by law. These rights of subrogation and reimbursement shall apply without regard to the location of the event that led to or caused the applicable sickness, injury, disease or disability.

Covered Person is a Trustee Over Plan Assets

Any Covered Person who receives benefits and is therefore subject to the terms of this section is hereby deemed a recipient and holder of Plan assets and is therefore deemed a trustee of the Plan

solely as it relates to possession of any funds which may be owed to the Plan as a result of any settlement, judgment or recovery through any other means arising from any injury or accident. By virtue of this status, the Covered Person understands that he or she is required to:

- A. notify the Plan or its authorized representative of any settlement prior to finalization of the settlement, execution of a release, or receipt of applicable funds;
- B. instruct his or her attorney to ensure that the Plan and/or its authorized representative is included as a payee on all settlement drafts;
- C. in circumstances where the Covered Person is not represented by an attorney, instruct the insurance company or any third party from whom the Covered Person obtains a settlement, judgment or other source of Coverage to include the Plan or its authorized representative as a payee on the settlement draft; and
- D. hold any and all funds so received in trust, on the Plan's behalf, and function as a trustee as it applies to those funds, until the Plan's rights described herein are honored and the Plan is reimbursed.

To the extent the Covered Person disputes this obligation to the Plan under this section, the Covered Person or any of its agents or representatives is also required to hold any/all settlement funds, including the entire settlement if the settlement is less than the Plan's interests, and without reduction in consideration of attorneys' fees, for which he/she exercises control, in an account segregated from their general accounts or general assets until such time as the dispute is resolved.

No Covered Person, beneficiary, or the agents or representatives thereof, exercising control over plan assets and incurring trustee responsibility in accordance with this section will have any authority to accept any reduction of the Plan's interest on the Plan's behalf.

Release of Liability

The Plan's right to reimbursement extends to any incident related care that is received by the Covered Person(s) Incurred prior to the liable party being released from liability. The Covered Person's/Covered Persons' obligation to reimburse the Plan is therefore tethered to the date upon which the claims were Incurred, not the date upon which the payment is made by the Plan. In the case of a settlement, the Covered Person has an obligation to review the "lien" provided by the Plan and reflecting claims paid by the Plan for which it seeks reimbursement, prior to settlement and/or executing a release of any liable or potentially liable third party, and is also obligated to advise the Plan of any incident related care Incurred prior to the proposed date of settlement and/or release, which is not listed but has been or will be Incurred, and for which the Plan will be asked to pay.

Excess Insurance

If at the time of injury, sickness, disease or disability there is available, or potentially available, any Coverage (including but not limited to Coverage resulting from a judgment at law or settlements), the benefits under this Plan shall apply only as an excess over such other sources of Coverage, except as otherwise provided for under the Plan's Coordination of Benefits section.

The Plan's benefits shall be excess to any of the following:

- A. the responsible party, its insurer, or any other source on behalf of that party;
- B. any first party insurance through medical payment coverage, personal injury protection, no-fault coverage, uninsured or underinsured motorist coverage, including any similar coverage under a different name in a particular state;
- C. any policy of insurance from any insurance company or guarantor of a third party including but not limited to an employer's policy;
- D. workers' compensation or other liability insurance company; or
- E. any other source of coverage, including but not limited to the following:
 - 1. crime victim restitution funds;
 - 2. civil restitution funds;

3. no-fault restitution funds, such as vaccine injury compensation funds;
4. any medical, applicable disability or other benefit payments; and
5. school insurance coverage.

Separation of Funds

Benefits paid by the Plan, funds recovered by the Covered Person(s), and funds held in trust over which the Plan has an equitable lien exist separately from the property and estate of the Covered Person(s), such that the death of the Covered Person(s), or filing of bankruptcy by the Covered Person(s), will not affect the Plan's equitable lien, the funds over which the Plan has a lien, or the Plan's right to subrogation and reimbursement.

Wrongful Death

In the event that the Covered Person(s) dies as a result of his or her injuries and a wrongful death or survivor claim is asserted against a third party or any Coverage, the Plan's subrogation and reimbursement rights shall still apply, and the entity pursuing said claim shall honor and enforce these Plan rights and terms by which benefits are paid on behalf of the Covered Person(s) and all others that benefit from such payment.

Obligations

It is the Covered Person's/Covered Persons' obligation at all times, both prior to and after payment of medical benefits by the Plan to:

- A. cooperate with the Plan, or any representatives of the Plan, in protecting its rights, including discovery, attending depositions, and/or cooperating in trial to preserve the Plan's rights;
- B. provide the Plan with pertinent information regarding the sickness, disease, disability, or injury, including accident reports, settlement information and any other requested additional information;
- C. take such action and execute such documents as the Plan may require to facilitate enforcement of its subrogation and reimbursement rights;
- D. do nothing to prejudice the Plan's rights of subrogation and reimbursement;
- E. promptly reimburse the Plan when a recovery through settlement, judgment, award or other payment is received;
- F. notify the Plan or its authorized representative of any incident related claims or care which may be not identified within the lien (but has been Incurred) and/or reimbursement request submitted by or on behalf of the Plan;
- G. notify the Plan or its authorized representative of any settlement prior to finalization of the settlement;
- H. not settle or release, without the prior consent of the Plan, any claim to the extent that the Covered Person may have against any responsible party or coverage;
- I. to instruct his or her attorney to ensure that the Plan and/or its authorized representative is included as a payee on any settlement draft;
- J. in circumstances where the Covered Person is not represented by an attorney, instruct the insurance company or any third party from whom the Covered Person obtains a settlement to include the Plan or its authorized representative as a payee on the settlement draft; and
- K. to make good faith efforts to prevent disbursement of settlement funds until such time as any dispute between the Plan and Covered Person over settlement funds is resolved.

If the Covered Person(s) and/or his or her attorney fails to reimburse the Plan for all benefits paid, to be paid, Incurred, or that will be Incurred, prior to the date of the release of liability from the relevant entity, as a result of said injury or condition, out of any proceeds, judgment or settlement received, the Covered Person(s) will be responsible for

any and all expenses (whether fees or costs) associated with the Plan's attempt to recover such money from the Covered Person(s).

The Plan's rights to reimbursement and/or subrogation are in no way dependent upon the Covered Person's/Covered Persons' cooperation or adherence to these terms.

Offset

If timely repayment is not made, or the Covered Person and/or his or her attorney fails to comply with any of the requirements of the Plan, the Plan has the right, in addition to any other lawful means of recovery, to deduct the value of the Covered Person's amount owed to the Plan. To do this, the Plan may refuse payment of any future medical benefits and any funds or payments due under this Plan on behalf of the Covered Person(s) in an amount equivalent to any outstanding amounts owed by the Covered Person to the Plan. This provision applies even if the Covered Person has disbursed settlement funds.

Minor Status

In the event the Covered Person(s) is a minor as that term is defined by applicable law, the minor's parents or court-appointed guardian shall cooperate in any and all actions by the Plan to seek and obtain requisite court approval to bind the minor and his or her estate insofar as these subrogation and reimbursement provisions are concerned.

If the minor's parents or court-appointed guardian fail to take such action, the Plan shall have no obligation to advance payment of medical benefits on behalf of the minor. Any court costs or legal fees associated with obtaining such approval shall be paid by the minor's parents or court-appointed guardian.

Language Interpretation

The Plan Administrator retains sole, full and final discretionary authority to construe and interpret the language of this provision, to determine all questions of fact and law arising under this provision, and to administer the Plan's subrogation and reimbursement rights with respect to this provision. The Plan Administrator may amend the Plan at any time without notice.

Severability

In the event that any section of this provision is considered invalid or illegal for any reason, said invalidity or illegality shall not affect the remaining sections of this provision and Plan. The section shall be fully severable. The Plan shall be construed and enforced as if such invalid or illegal sections had never been inserted in the Plan.

12.3 MEDICARE BENEFITS

This provision prevents duplication of benefits for Covered Expenses when Medical Care benefits are available from Medicare.

Under the Tax Equity and Fiscal Responsibility Act of 1982, as amended (TEFRA), active employees and/or their spouses who are sixty-five (65) or over may choose to have the Company program as primary coverage, in which case Medicare may pay benefits on a secondary basis. Otherwise, an employee may elect to drop out of the company program and choose Medicare as primary coverage. Employees in this category who are enrolled under this Plan will remain so enrolled with this Plan as primary coverage unless an option form is on file indicating otherwise.

The Plan may also pay its benefits as primary to Medicare's in other situations, as prescribed by applicable laws and regulations.

The Plan intends to comply with the federal Social Security Act, as amended, and other applicable laws, as such apply to Medicare benefits.

12.4 ADDITIONAL RIGHTS OF RECOVERY

If payments are made under the Plan that should not have been made, the Plan may recover that incorrect payment. The Plan may recover this payment from the person to whom it was made or from any other appropriate party. If any such incorrect payment is made to the Participant, the Plan may deduct it when making future payments directly to the Participant. Once the Plan

Administrator determines that a previous benefit payment should be reimbursed, in whole or in part, either due to the provisions described in Section 12.2 or because such benefit payment should not have been made in accordance with the provisions of this Plan, the Participant and/or the applicable provider will be notified of such overpayment, and a request will be made for such Participant/provider to reimburse the Plan. If the reimbursement is not made as requested, such amount will constitute a lien against future claim payments that would otherwise be paid on the Participant or the Covered Person's behalf. The Plan Administrator retains the right to reduce or withhold such future claim payments until the lien is satisfied.

This Plan will comply with Sections 609(b)(1), (2) and (3) of the Employee Retirement Income Security Act with regard to Covered Persons eligible for Medicaid. An Employee's or Dependent's eligibility for, or participation in, Medicaid will not affect determination of whether or not payments should be made. Under state and federal law, should a Covered Person be entitled to payment of a claim under this Plan, and all or part of that claim has been paid by Medicaid, then the state is subrogated to the Covered Person's right to payment under this Plan to the extent of the amount paid by Medicaid, and reimbursement under this Plan will be made in that amount directly to the state.

12.5 FACILITY OF PAYMENT

Whenever a Covered Person or provider to whom payments are directed to be made is mentally, physically, or legally incapable of receiving or acknowledging receipt of such payments, neither the Plan Administrator nor the Benefit Manager shall be under any obligation to see that a legal representative is appointed or to make payments to such legal representative, if appointed. A determination of payment made in good faith shall be conclusive on all persons. The Plan Administrator, Benefit Manager or any fiduciary shall not be liable to any person as a result of a payment made and shall be fully discharged from all future liability with respect to a payment made.

12.6 ADMINISTRATION OF THE PLAN

Except as otherwise specifically provided for in the Plan, the Plan Administrator shall have the exclusive authority to control and manage the operation and administration of the Plan and shall be Named Fiduciary of the Plan for purposes of ERISA. The Plan Administrator shall have all power necessary or convenient to enable it to exercise such authority. In connection therewith, the Plan Administrator may provide rules and regulations, not inconsistent with the provisions thereof, for the operation and management of the Plan, and may from time to time amend or rescind such rules or regulations. The Plan Administrator may accept service of legal process for the Plan and shall have the full discretion, power, and the duty to take all action necessary or proper to carry out the duties required under ERISA and all other applicable law.

The Plan Administrator may delegate duties involved in the administration of this Plan to such person or persons whose services are deemed necessary or convenient; provided however, that both the ultimate responsibility for the administration of this Plan and the authority to interpret this Plan shall remain with the Plan Administrator. The Employer shall indemnify any employee to whom duties are delegated by the Plan Administrator pursuant to this section from and against any liability that such employee may incur in the administration of the Plan, except for liabilities arising from the recklessness or willful misconduct of such employee.

The Plan Administrator shall be responsible for controlling and managing the operation and administration of this Plan, including, but not limited to, the power:

- A. to employ one (1) or more persons or entities to render advice with respect to any responsibility the Plan Administrator has under this Plan;
- B. to construe and interpret this Plan;
- C. to adopt such rules, regulations, forms and procedures as from time to time it deems advisable or appropriate in the proper administration of this Plan;

- D. to decide all questions of eligibility and to determine the amount, manner and time of payment of any benefits hereunder;
- E. to prescribe procedures to be followed by any person in applying for any benefits under this Plan and to designate the forms, documents, evidence or such other information as the Plan Administrator may reasonably deem necessary to support an application for any benefits under this Plan;
- F. to authorize, in its discretion, payments of benefits properly payable pursuant to the provisions of this Plan;
- G. to prepare and to distribute, in such manner as it deems appropriate, information explaining the Plan;
- H. to apply consistently and uniformly to all Covered Persons in similar circumstances its rules, regulations, determinations and decisions;
- I. to prepare and file such reports and to complete and to distribute such other documents as may be required to comply fully with the provisions of ERISA and all other applicable laws, and all regulations promulgated thereunder; and
- J. to retain counsel (who may, but need not, be counsel to the Company), to employ agents and to provide for such clerical, medical, accounting, auditing and other services as it may require in carrying out the provisions of the Plan.

The Plan Administrator shall be the sole judge of the standards of proof required in any case. In the application and interpretation of this Plan document, the decision of the Plan Administrator shall be final and binding on the Participants, Dependents, and all other persons. The Plan Administrator shall have the full power and authority, in its sole discretion, to construe and interpret the provisions and terms of this Plan document and all other written documents. Any such determination and any such construction adopted by the Plan Administrator in good faith shall be binding upon all of the parties hereto and the beneficiaries thereof and may not be reversed by a court of competent jurisdiction unless the court finds the determination to be arbitrary and capricious.

12.7 NON-ALIENATION AND ASSIGNMENT

The Plan shall not be liable for any debt, liability, contract or tort of any employee or Covered Person. The Plan shall pay all benefits due and payable for Covered Expenses directly to the Covered Person who Incurred the Covered Expenses, and no Plan benefits shall be subject to anticipation, sale, assignment, transfer, encumbrance, pledge, charge, attachment, garnishment, execution, alienation or any other voluntary or involuntary alienation or other legal or equitable process not transferable by operation of law; provided however, that a Covered Person to whom benefits are otherwise payable may assign benefits to a Hospital, Physician or other service provider; provided further, that any such assignment of benefits by a Covered Person to a Hospital, Physician or other service provider shall be binding on the Plan only if:

- A. the Plan Administrator or Benefit Manager is notified of such assignment prior to payment of benefits;
- B. the assignment is made on a form provided by, or approved by, the Plan Administrator or the Benefit Manager; and
- C. the assignment contains such additional terms and conditions as may be required from time to time by the Plan Administrator or Benefit Manager.

A Provider that accepts an Assignment of Benefits, in accordance with this Plan, does as consideration in full for services rendered and is bound by the rules and provisions set forth within the terms of this document.

12.8 FAILURE TO ENFORCE

Failure to enforce any provision of this Plan does not constitute a waiver or otherwise affect the Plan Administrator's right to enforce such a provision at another time, nor will such failure affect the right to enforce any other provision.

12.9 FIDUCIARY RESPONSIBILITIES

No fiduciary of the Plan shall be liable for any acts or omission in carrying out his, her or its responsibilities under the Plan, except as may be provided under ERISA and other applicable laws. Each fiduciary under the Plan shall be responsible only for the specific duties assigned to such fiduciary under the Plan and shall not be directly or indirectly responsible for the duties assigned to another fiduciary, except as may be otherwise provided in ERISA and other applicable laws.

12.10 DISCLAIMER OF LIABILITY

The Plan is not responsible for the efficiency or integrity of any health care provider delivering services or supplies utilized by the Participant. The Plan is not liable in any way for the effect of delivery of such services or supplies, the results of actions taken as a result of such services or supplies being limited or not covered by the Plan, nor any limitations imposed on the cost sharing responsibility of the Plan.

Nothing contained herein shall confer upon a Covered Person any claim, right or cause of action, either at law or at equity, against the Plan, Plan Administrator, Benefit Manager, or any Employer for the acts or omissions of any health care provider from whom a Covered Person receives care, or for the acts or omission of any Physician from whom the Covered Person receives care under the Plan, or for any acts or omissions of any provider of services or supplies under this Plan.

12.11 ADMINISTRATIVE AND CLERICAL ERRORS

The benefits payable to or on behalf of a Participant or Dependent under this Plan will not be decreased nor increased due to administrative or clerical errors made by the Employer, the Plan Administrator, the Utilization Review Service or the Benefit Manager. If written application for coverage for an eligible employee or Dependent is submitted by the employee/Participant within the applicable time frame specified in Article V, any subsequent administrative or clerical error made by the Employer, the Plan Administrator or the Benefit Manager shall not act to delay the effective date of such person's coverage beyond the date such coverage would otherwise become effective if such application was processed in a timely manner. In addition, any such error made in claims processing, utilization review or other administrative functions shall not affect the benefits payable to or on behalf of a Covered Person under this Plan. The Plan Administrator may require proof of an error described in this provision. The Plan Administrator shall have the sole responsibility to determine when an error is an "administrative or clerical" error and will be the sole judge of any proof required.

12.12 RESCISSION OF COVERAGE

A rescission of coverage means that the coverage may be legally voided all the way back to the day the Plan began to provide an individual with coverage, just as if he or she never had coverage under the Plan. Such coverage can only be rescinded if the individual (or a person seeking coverage on an individual's behalf) perform an act, practice, or omission that constitutes fraud; or unless the individual (or a person seeking coverage on the individual's behalf) make an intentional misrepresentation of material fact, as prohibited by the terms of this Plan. Coverage can also be rescinded due to such an act, practice, omission or intentional misrepresentation by an employer.

Such individual will be provided with thirty (30) calendar days' advance notice before coverage is rescinded. Such individual has the right to request an internal appeal of a rescission of his or her coverage. Once the internal appeal process is exhausted, Such person has the additional right to request an independent external review.

12.13 NO SURPRISES ACT – EMERGENCY SERVICES AND SURPRISE BILLS

The NSA prohibits providers from pursuing Covered Persons for the difference between the Maximum Allowable Charge and the provider's billed charge for applicable services, with the exception of valid Plan-appointed cost-sharing as outlined in this Plan. Any such cost-sharing amounts will accrue toward Plan cost-sharing maximums.

Benefits for claims subject to the NSA will be denied or paid within thirty (30) days of receipt of an initial claim, and if approved will be paid directly to the Provider.

Claims subject to the NSA are those which are submitted for:

- A. Emergency Services;
- B. non-Emergency services rendered by a provider at a Participating Health Care Facility, provided the Covered Person has not validly waived the applicability of the NSA; and
- C. covered air ambulance services.

The Plan intends to comply with the No Surprise Billing provisions of the No Surprises Act (NSA), including payment calculations and by adding coverage for any amount for which the Plan is required to pay as a result of an Independent Dispute Resolution (IDR).

YOUR RIGHTS AND PROTECTIONS AGAINST SURPRISE MEDICAL BILLS

When you get care for Emergency Services, you are protected from surprise billing or balance billing.

WHAT IS “BALANCE BILLING” (SOMETIMES CALLED “SURPRISE BILLING”)?

When you see a doctor or other health care provider, you may owe certain out-of-pocket costs, such as a copayment, coinsurance, and/or a deductible. You may have other costs or have to pay the entire bill if you see a provider or visit a health care facility that isn't in your health plan's network, if your plan utilizes a network.

“Non-Preferred” describes providers and facilities that haven't signed a contract with your health plan. Non-Preferred providers may be permitted to bill you for the difference between what your plan agreed to pay, and the full amount charged for a service. This is called “**balance billing**.”

“Surprise billing” is an unexpected balance bill. This can happen when you can't control who is involved in your care—like when you have an Emergency or when you schedule a visit at a Participating Health Care Facility, if applicable to your plan, but are unexpectedly treated by a Non-Preferred provider.

YOU ARE PROTECTED FROM BALANCE BILLING FOR:

Emergency Services. If you have an Emergency medical condition and get Emergency Services from a Non-Preferred provider or facility, the most the provider or facility may bill you is your plan's cost-sharing amount (such as copayments and coinsurance). You can't be balance billed for these Emergency Services. This includes services you may get after you're in stable condition unless you give written consent and give up your protections not to be balanced billed for these post-stabilization services.

Please see Article II of the Plan for cost-sharing information for these services.

Certain services at a Participating Health Care Facility, if applicable. When you get services from a Participating Health Care Facility, if your plan contracts with one, certain providers there may still be Non-Preferred. In these cases, the most those providers may bill you is your plan's cost-sharing amount. This applies to emergency medicine, anesthesia, pathology, radiology,

laboratory, neonatology, assistant surgeon, hospitalist, or intensivist services. These providers can't balance bill you and may not ask you to give up your protections not to be balance billed.

If you get other services at a contracted Participating Health Care Facility, Non-Preferred providers **can't** balance bill you, unless you give written consent and give up your protections.

You're never required to give up your protections from balance billing. You can choose a Participating Health Care Facility, if your plan has contracted with one.

Please see Article II of the Plan for cost-sharing information for these services.

WHEN BALANCE BILLING ISN'T ALLOWED, YOU ALSO HAVE THE FOLLOWING PROTECTIONS:

- A. You are only responsible for paying your share of the cost (like the copayments, coinsurance, and deductibles). Your health plan will pay providers and facilities directly.
- B. Your health plan generally must:
 - 1. Cover Emergency Services without requiring you to get approval for services in advance (prior authorization).
 - 2. Cover Emergency Services by Non-Preferred providers.
 - 3. Base what you owe the provider or facility (cost-sharing) on Participating Health Care Facility contracts, if applicable, and show that amount in your explanation of benefits.
 - 4. Count any amount you pay for Emergency Services or Non-Preferred services toward your deductible and out-of-pocket limit.

If you believe you've been wrongly billed, you may contact the Centers for Medicare & Medicaid Services. Visit <https://www.cms.gov/nosurprises/consumers> or call (800) 985-3059 for more information about your rights under federal law.

ARTICLE XIII

PRIVACY

13.1 PRIVACY OF HEALTH INFORMATION

This provision is intended to bring this Plan into compliance with the privacy provisions of the Health Insurance Portability and Accountability Act of 1996, as amended, and the regulations issued thereunder. Health Information transmitted or maintained by the Plan will be subject to the provisions described in this article.

13.2 USE AND DISCLOSURE OF PROTECTED HEALTH INFORMATION

Protected Health Information will only be disclosed or used by the Plan under one (1) of the following conditions:

- A. with the specific consent of the individual who is the subject of the Protected Health Information, provided that the Plan obtains any required authorization;
- B. for payment of claims submitted to the Plan, or for utilization review activities as described in Article VI, including, but not limited to, the review of any grievances or appeals involved in such activities that are generated by the Covered Person or his or her authorized representatives; or
- C. for other reasonable purposes necessary to operate the Plan, to the extent that such Protected Health Information is required for such purposes, including:
 1. quality assessment and improvement activities;
 2. evaluation of Plan performance;
 3. underwriting and premium rating and other activities relating to the procuring, renewal or replacement of stop loss or excess loss insurance;
 4. conducting or arranging for medical review, legal services and auditing functions, including fraud and abuse detection and compliance programs;
 5. business planning and development of the Plan;
 6. business management and general administrative activities of the Plan, including, but not limited to, enrollments, billing, customer service and the resolution of internal grievances; and
 7. other health care operations listed under 45 C.F.R. § 164.501.

No other use or disclosure of Protected Health Information is permitted by this Plan.

13.3 DISCLOSURES OF HEALTH INFORMATION TO THE COMPANY

The Plan Administrator will disclose, or permit the disclosure of, Health Information to the Company only as described below:

- A. for any of the purposes and under the conditions described in Section 13.2;
- B. as Summary Health Information, if requested by the Company for the following purposes:
 1. obtaining premium bids from health plans for providing health insurance coverage under the Plan; or
 2. modifying, amending or terminating the Plan; or
- C. for informational purposes regarding whether an individual is participating in the Plan, provided such information is only used by the Company for the purpose of performing Plan administrative functions.

Prior to any disclosure of Health Information to the Company, such entity must agree:

- A. not to use or further disclose the information other than as permitted or required by this section, or as required by law;

- B. that it will ensure that any agents, including subcontractors, employed by the Company or Plan Administrator for Plan administration or other Plan purposes to whom it provides Protected Health Information, including, but not limited to, the Benefit Manager, any Utilization Review Service or Pharmacy benefit manager, agree to the same restrictions and conditions that apply to the Company with respect to such information;
- C. not to use or disclose the Protected Health Information for employment-related actions and decisions, or in connection with any other benefit or employee benefit plan sponsored by the Company;
- D. that it will report to the Plan Administrator any use or disclosure of the information that is inconsistent with the uses or disclosures provided for in this section of which it becomes aware;
- E. that it will make available Protected Health Information to the subject of such information, and allow amendment to such information as described in Section 13.4 and Section 13.5;
- F. that it will provide an accounting in accordance with 45 C.F.R. § 164.528, upon the request of the subject of Protected Health Information, of the disclosure of such information by the Plan made within six (6) years of the request, except information exempted from such accounting under that section;
- G. that it will make available its internal practices, books, and records relating to the use and disclosure of Protected Health Information received from the Plan to the Secretary of the United States Department of Health and Human Services for the purpose of determining compliance by the Plan with the privacy provisions of HIPAA;
- H. that it will, if feasible, return or destroy all Protected Health Information received from the Plan that the Company still maintains in any form, and that it will not retain any copies of such information when no longer needed for the purpose for which the disclosure was made. If return or destruction is not feasible, that it will limit further uses and disclosures to those purposes that make the return or destruction of the information infeasible; and
- I. that it will provide for adequate separation between the Plan and the Plan Sponsor by implementing the following procedures:
 - 1. access to Protected Health Information will only be provided to the following categories of Company employees:
 - a. the President;
 - b. the Vice President;
 - c. the Controller; and
 - d. employees of the Human Resources Department with responsibilities for Plan administration;
 - 2. that access to and use by such employees or other persons as described above will be limited to the plan administration functions that the Company performs for the Plan; and
 - 3. any non-compliance by such named individuals with the privacy provisions of this Plan will be addressed in accordance with the Company's established employee discipline and termination procedures.

13.4 ACCESS OF COVERED PERSONS TO PROTECTED HEALTH INFORMATION

A Covered Person or other individual has the right of access to inspect and obtain a copy of Protected Health Information about such person as long as such information is maintained by the Plan, except for:

- A. psychotherapy notes;

- B. information compiled in reasonable anticipation, or for use in, a civil, criminal or administrative proceeding or action; or
- C. as such information is otherwise exempted from disclosure under 45 C.F.R. § 164.524.

Any such request must be made to the Plan Administrator a writing signed by the Covered Person whose information is being requested. The Plan Administrator will notify the Covered Person, in writing, as to whether such request is approved or denied, and, if approved, will provide access to the information in accordance with 45 C.F.R. § 164.524(c), including the imposition of reasonable fees for the costs of providing such access.

13.5 AMENDMENT RIGHTS

A Covered Person or other individual has the right to have the Company amend Protected Health Information or other information about such individual as long as such information is maintained by the Plan. The Plan Administrator will deny such a request if:

- A. the information was not created by the Plan, unless the individual provides a reasonable basis to believe that the originator of the Protected Health Information is no longer available to act on the requested amendment;
- B. the information is not currently maintained in any record by the Plan;
- C. the information would not be available for inspection under the reasons cited in Section 13.4; or
- D. the information in the Plan's records is accurate and complete.

Any request for amendment of Protected Health Information must be provided in writing to the Plan Administrator and signed by the Covered Person or individual who is the subject of the information with an explanation as to why such person believes the information is inaccurate, incomplete or incorrect. The Plan Administrator will notify the Covered Person, in writing, as to whether such request is approved or denied, and, if approved, will make the necessary corrections to the information in accordance with 45 C.F.R. § 164.526(c). The Plan Administrator will make reasonable efforts to inform all entities that it has knowledge of such entity's receipt of any information that has been corrected. If the request is denied, the individual may submit a written statement disagreeing with the denial that includes the basis of such disagreement. The Plan Administrator may prepare a written rebuttal of such statement. The statement of disagreement, and the rebuttal, if any, will be included in any future disclosure of the information. Even if no statement of disagreement is submitted, the individual may request that the request for amendment and denial be included with any future disclosures of the information.

13.6 SECURITY OF PROTECTED HEALTH INFORMATION

The Company will implement administrative, physical and technical safeguards that reasonably and appropriately protect the confidentiality, integrity and availability of electronic Protected Health Information (ePHI) that is created, received, maintained or transmitted on behalf of the Plan, including reasonable and appropriate security measures between the Company and the Plan to support the requirements of Section 13.3. The Company will further ensure that any agent, including a subcontractor, to whom it provides access to ePHI agrees to implement reasonable and appropriate security measures to protect the information, and will report any security incident of which it becomes aware to the Plan Administrator.

ARTICLE XIV

STATEMENT OF ERISA RIGHTS

As a Participant in this Plan, you are entitled to certain rights and protections under the Employee Retirement Income Security Act of 1974 (ERISA). ERISA provides that all Plan Participants shall be entitled to:

A. Receive Information About Your Plan and Benefits:

1. Examine, without charge, at the Plan Administrator's office and at other specified locations, such as worksites, all documents governing the Plan, including insurance contracts, and a copy of the latest annual report (Form 5500 Series) filed by the Plan with the U.S. Department of Labor and available at the Public Disclosure Room of the Employee Benefits Security Administration.
2. Obtain, upon written request to the Plan Administrator, copies of documents governing the operation of the Plan, including insurance contracts and copies of the latest annual report (Form 5500 Series) and updated summary plan description. The administrator may make a reasonable charge for the copies.
3. Receive a copy of the Plan's annual financial report. The Plan Administrator is required by law to furnish each Participant with a copy of this summary annual report.

B. Continue Group Health Plan Coverage:

Continue health care coverage for yourself, spouse or Dependents if there is a loss of coverage under the Plan as a result of a Qualifying Event. You or your Dependents may have to pay for such coverage. Review this summary plan description and the documents governing the Plan on the rules governing your COBRA continuation rights.

C. Prudent Actions by Plan Fiduciaries:

In addition to creating rights for Plan Participants, ERISA imposes duties upon the people who are responsible for the operation of the employee benefit plan. The people who operate your Plan, called "fiduciaries" of the Plan, have a duty to do so prudently and in the interest of you and other Plan Participants and beneficiaries. No one, including your Employer or any other person, may fire you or otherwise discriminate against you in any way to prevent you from obtaining a welfare benefit or exercising your rights under ERISA.

D. Enforce Your Rights:

1. If your claim for a welfare benefit is denied or ignored, in whole or in part, you have a right to know why this was done, to obtain copies of documents relating to the decision without charge, and to appeal any denial, all within certain time schedules.
2. Under ERISA, there are steps you can take to enforce the above rights. For instance, if you request a copy of plan documents or the latest annual report from the Plan and do not receive them within thirty (30) days, you may file suit in a Federal court. In such a case, the court may require the Plan Administrator to provide the materials and pay you up to one hundred ten dollars (\$110.00) a day until you receive the materials, unless the materials were not sent because of reasons beyond the control of the administrator. If you have a claim for benefits that is denied or ignored, in whole or in part, you may file suit. If you have a claim for benefits that is denied or ignored, in whole or in part, you may file suit, once the other appeal rights listed in this Plan are exhausted, in a state or Federal court. In addition, if you disagree with the Plan's decision or lack thereof concerning the qualified status of a medical child support order, you may file suit in Federal court. If it should happen that Plan Fiduciaries misuse the Plan's money, or if you are discriminated against for asserting your rights, you may seek assistance from the U.S. Department of Labor, or you may

file suit in a Federal court. The court will decide who should pay court costs and legal fees. If you are successful the court may order the person you have sued to pay these costs and fees. If you lose, the court may order you to pay these costs and fees, for example, if it finds your claim is frivolous.

E. Assistance with Your Questions:

If you have any questions about your Plan, you should contact the Plan Administrator. If you have any questions about this statement or about your rights under ERISA, or if you need assistance in obtaining documents from the Plan Administrator, you should contact the nearest office of the Employee Benefits Security Administration, U.S. Department of Labor, listed in your telephone directory or the Office of Participant Assistance and Communications, Employee Benefits Security Administration, U.S. Department of Labor, 200 Constitution Avenue N.W., Washington, D.C. 20210. You may also obtain certain publications about your rights and responsibilities under ERISA by calling the publications hotline of the Employee Benefits Security Administration.